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Bigger Cycle, Same Compounding DNA

- India's listed hospital sector delivered a 15.5% revenue CAGR and 25% EBITDA CAGR over FY20–FY25 — margins expanded 780 bps even as ~14,000 beds were added, because 70% of capacity came via inorganic and brownfield routes that ramp fast, cost less, and dilute margins minimally
- The next cycle is bigger — 23,039 beds over the next 3–4 years — but greenfield now dominates the pipeline vs. the brownfield and acquisition-led last cycle; operators with a higher brownfield mix or a healthy blend of both will grow faster and hold margins better in the near-to-mid term
- Structural tailwinds are firmly in place — lifestyle disease burden, ageing population, rising insurance penetration, and the CGHS tariff reset together underpin 18–20% sector revenue CAGR over the next 3–4 years; the risk is margin pressure from tightening clinician and nursing supply alongside greenfield ramp-up drag
- Within our covered universe, Apollo and KIMS are our top picks; in the uncovered space, Max and Fortis are set to outperform

Five Years That Rewrote the Sector: India's listed hospital sector (13 companies tracked) posted a compelling 15.5% revenue CAGR over FY20–25 to Rs 492 bn — broad-based, with 12 of 13 players in double digits. Growth was driven by strong IPD momentum (9–29% CAGR across the universe), ARPOB compounding of 7–10% p.a. — itself a function of consistent 3–4% price hikes, ~300–400 bps CONGO revenue share gains, and a structural payor mix shift of ~7–8% from cash toward TPA/insurance — and ~9,400 beds added over FY20–25 (25% of the FY20 base), all while aggregate occupancy held firm at ~60%.

Next Bed Addition Cycle: Bigger but Different: The next cycle is nearly 2.4x larger — 23,039 beds at a 13.1% CAGR through FY29 vs. 5.6% prior — but greenfield now dominates at 51% of additions vs. 30% last cycle, while acquisitions shrink from 38% to just 8%. Brownfield uses existing infrastructure and turns margin accretive within 12 months; greenfield builds from scratch and takes another 12–18 months to breakeven. Operators with higher brownfield mix — Max, KIMS — will hold margins better near term; greenfield-heavy names — Jupiter, Artemis, Medanta — will see drag through FY26–27.

Decade of Tailwinds: Demand Side Getting Better: Ageing population, rising lifestyle diseases, and insurance penetration heading to 45–50% provide a structural demand floor that is only deepening. ARPOB — which compounded at 7–10% p.a. last cycle — has multiple levers still firing: annual price hikes, CONGO mix deepening, payor mix shift toward insurance, and the recent CGHS surgical package revision. On capacity, 74% of the 23,039 beds being added through FY29 are headed to metro markets, with the remaining 26% deepening underpenetrated non-metro catchments. Together, these underpin our 18–20% sector revenue CAGR through FY26–29.

Growth Assured; Margin Journey Diverges: Sector margins will oscillate in the 22–25% band through FY26–27 as greenfield ramp-up and tightening clinician supply weigh on profitability before re-expanding to 25–27% by FY29 as new beds mature. ROCE recovery will be earliest for brownfield and acquisition-heavy operators; greenfield-heavy names trough in FY26–27 before inflecting. Apollo and KIMS are our top picks in the covered universe; Max and Fortis are set to outperform in the uncovered space.

Analysts
Bharat Celly
bharat.celly@equirus.com
+91 079 6901 5036
Vinay Jain
vinay.jain@equirus.com
+91 079 6901 5011

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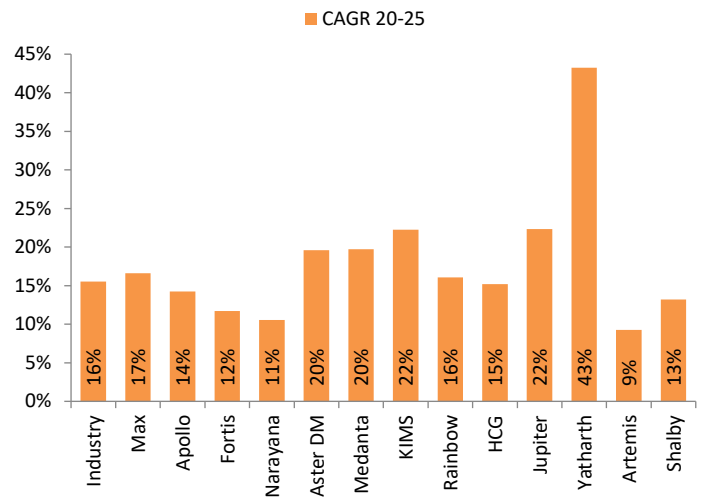
Story in Charts – Last Five years of Compounding

Exhibit 1: Double digit revenue growth across the hospitals



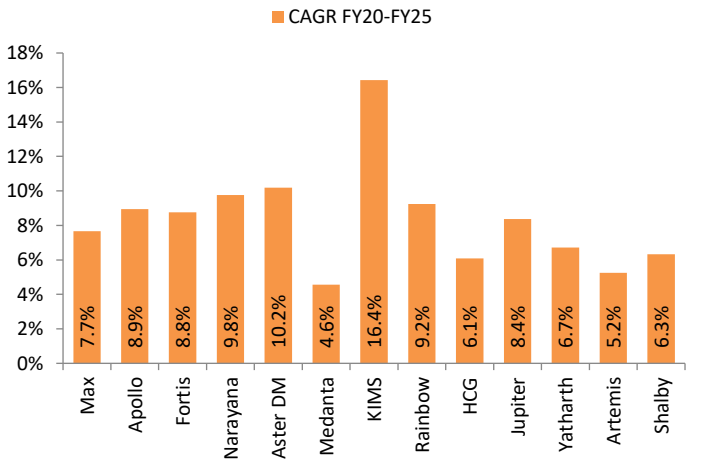
Source: Company Data, Equirus

Exhibit 2: Bed Addition: Expansion with Sharp Post-FY23 Acceleration

Bed Addition	FY20	FY23	FY25	9MFY26
Max	3,371	3,548	4,930	5,267
Apollo	7,491	7,860	8,025	8,072
Fortis	3,522	4,102	4,746	6,056
Narayana	6,487	6,076	5,745	5,750
Aster	3,693	4,317	5,159	5,451
Medanta	2,239	2,697	3,042	3,579
KIMS	3,079	3,940	5,644	6,464
Rainbow	1,371	1,605	1,935	2,285
HCG	2,068	1,871	2,203	2,203
Jupiter	669	950	1,061	1,048
Yatharth	1,100	1,405	1,605	2,305
Artemis	395	541	700	700
Shalby	2,062	2,062	2,152	2,312
Total	**37,547**	**40,974**	**46,947**	**51,492**

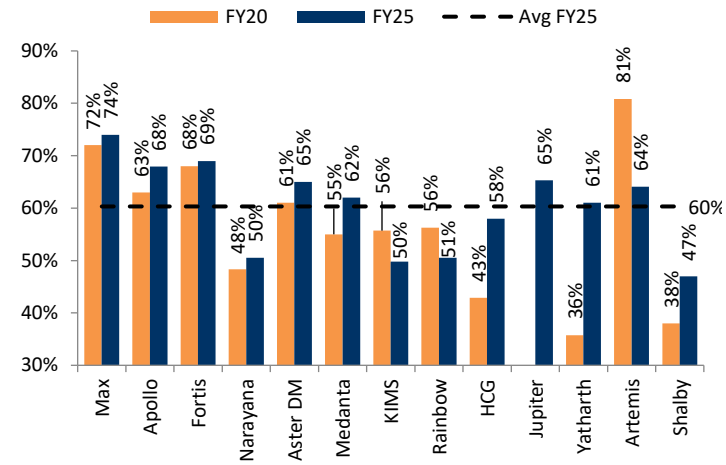
Source: Company Data, Equirus

Exhibit 3: Healthy ARPOB CAGR - Structural Improvement



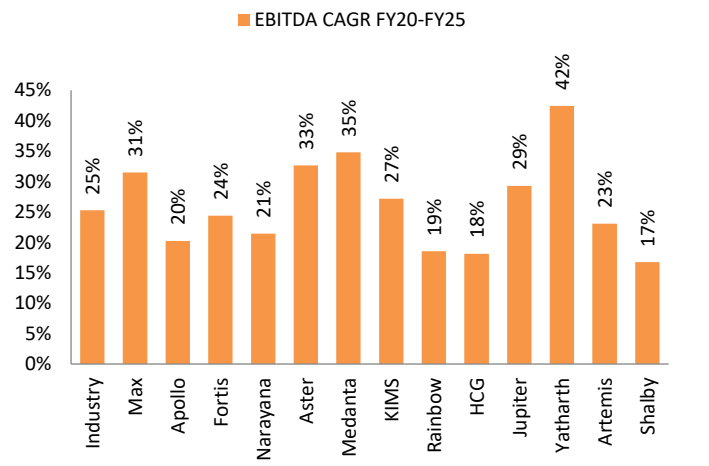
Source: Company Data, Equirus

Exhibit 4: Steady Occupancy Expansion Across Players Over Last 5 yrs



Source: Company Data, Equirus

Exhibit 5: Robust EBITDA CAGR of mid 20s



Source: Company Data, Equirus

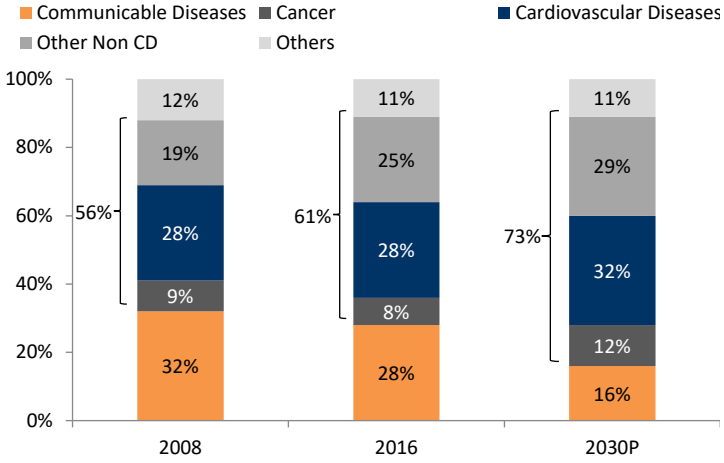
Exhibit 6: Mature Players Sustain High ROCE While Expansion Dilutes

ROCE	FY20	FY21	FY22	FY23	FY24	FY25	9MFY26
Max	11.1%	14.3%	28.8%	33.1%	31.8%	25.9%	21.9%
Apollo	13.2%	5.5%	22.6%	24.2%	24.5%	25.8%	27.2%
Fortis	3.1%	-0.1%	7.6%	11.1%	13.4%	14.4%	15.7%
Narayana	13.7%	1.4%	24.8%	33.5%	33.2%	26.1%	26.1%
Aster	10.3%	9.2%	16.9%	16.6%	22.9%	21.2%	21.0%
Medanta	5.1%	4.3%	14.3%	16.8%	19.3%	19.5%	18.4%
KIMS	20.3%	30.0%	33.0%	23.5%	17.6%	16.2%	11.5%
Rainbow	14.7%	9.6%	20.6%	21.7%	17.3%	16.5%	15.9%
HCG	1.7%	-2.0%	4.6%	7.6%	7.9%	7.1%	7.9%
Jupiter		6.1%	16.1%	20.1%	19.6%	16.7%	15.3%
Yatharth		16.9%	25.3%	25.6%	21.4%	12.5%	11.2%
Artemis	9.8%	3.1%	8.8%	10.0%	12.6%	11.2%	12.0%
Shalby	8.0%	8.0%	14.0%	16.0%	18.0%	14.0%	11.8%

Source: Company Data, Equirus

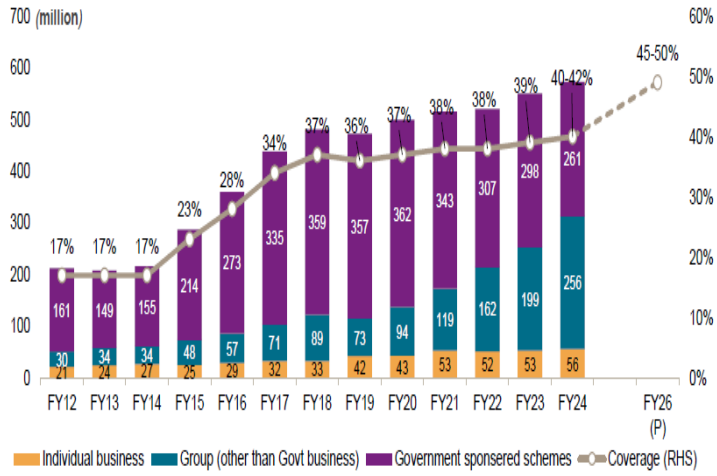
Story in Charts - Future Compounding Levers – all firing

Exhibit 7: Lifestyle disease rising rapidly



Source: Apollo Hospitals AR, Equirus

Exhibit 8: Rise in Insurance Penetration



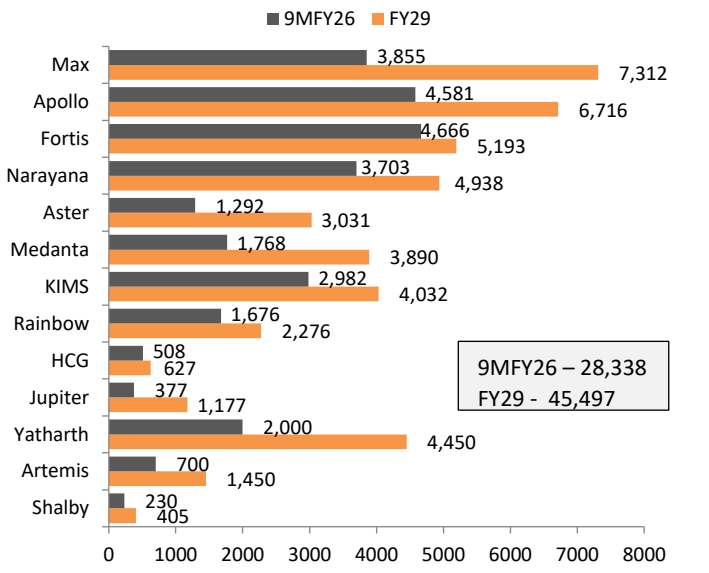
Source: Manipal Hospitals DRHP, Equirus

Exhibit 9: Heavy Greenfield Expansion Ahead

Hospitals	CAGR of Bed Expansion (9MFY26-FY29)				
	Brownfield	Greenfield	Acquisition	O&M/PHF	Total CAGR
Industry	**3.50%**	**6.60%**	**1.90%**	**1.10%**	**13.10%**
Max	6.20%	10.10%	0.00%	6.80%	23.20%
Apollo	2.50%	4.50%	3.50%	0.00%	10.50%
Fortis	2.20%	0.00%	0.70%	0.00%	2.80%
Narayana	1.60%	6.60%	0.00%	0.00%	8.20%
Aster	3.30%	9.10%	0.40%	0.00%	12.80%
Medanta	3.80%	17.50%	0.00%	0.00%	21.20%
KIMS	5.60%	1.40%	0.00%	2.60%	9.60%
Rainbow	0.00%	11.50%	0.00%	0.00%	11.50%
HCG	1.80%	0.00%	0.00%	0.00%	1.80%
Jupiter	3.40%	30.10%	0.00%	0.00%	33.50%
Yatharth	4.90%	0.00%	24.60%	0.00%	29.50%
Artemis	10.20%	25.50%	0.00%	0.00%	35.70%
Shalby	2.50%	0.00%	0.00%	0.00%	2.50%

Source: Company Data, Equirus

Exhibit 10: Metro-Led Expansion Dominates Industry Growth



Source: Company Data, Equirus

Exhibit 11: Resilient capex in future – following greenfield expansion

Amount (Rs in mn)	Capex	Bed Addition (No.)	Capex/Bed
Max	57,570	4,577	12.6
Apollo	54,000	2,815	19.2
Fortis	-	527	-
Narayana	29,740	1,535	19.4
Aster	20,000	2,368	8.4
Medanta	39,013	2,796	14
KIMS	10,950	2,051	5.3
Rainbow	-	880	-
HCG	751	119	6.3
Jupiter	13,250	1,446	9.2
Yatharth	15,260	2,700	5.7
Artemis	7,975	1,050	7.6
Shalby	-	175	-
Total	**2,48,509**	**23,039**	**10.8**

Source: Company Data, Equirus

Exhibit 12: FY25 (Beds per Doctor) – Margin Pressure ahead due to need for Doctors

Hospitals	Doctors	Bed capacity	Beds per Doctor
Max	5,000	4,930	0.99
Apollo	13,000	10,187	0.78
Narayana	4,000	5,745	1.44
Aster	3,302	5,159	1.56
Medanta	1,000	3,042	3.04
KIMS	2,212	5,444	2.46
Rainbow	910	1,935	2.13
Jupiter	1,275	1,061	0.83
Yatharth	750	1,605	2.14
Artemis	500	700	1.40
Total	**31,949**	**39,808**	**1.25**

Source: Company Data, Equirus

Five Years of Compounding That Rewrote the Sector's Identity

Structural Shifts Driving Profitable Compounding

12/13 players in double-digit growth, with ~60% of expansion driven by yield (ARPOB)

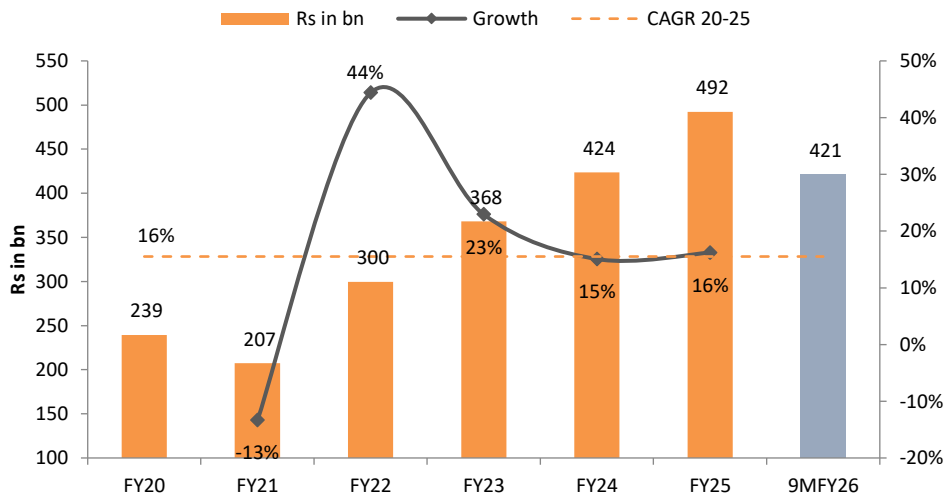
India's listed hospital sector (13 companies tracked) posted a compelling 15.5% revenue CAGR over FY20–25 to Rs 492 bn — broad-based, with 12 of 13 players in double digits. Growth was sacrosanct, driven by: a) strong IPD momentum (9–29% CAGR across the universe), b) ARPOB compounding of 7–10% p.a. — itself a function of consistent 3–4% price hikes, ~300–400 bps CONGO revenue share gains over five years, structural payor mix shift of ~7–8% from cash toward TPA/insurance, and c) ~9,400 beds added over FY20–25 (25% of the FY20 base) while aggregate occupancy held firm at ~60%.

Despite aggressive capacity addition, margin profile saw a sharp improvement — industry EBITDA margins expanded ~780 bps from 15.7% in FY20 to 23.5% in FY25, sustaining at 23% through 9MFY26. Strong margin expansion despite bed addition stems from ~66% of additions being inorganic or brownfield (acquisitions 38%, brownfield 28%), limiting fixed-cost ramp drag, further aided by CONGO mix deepening and payor mix tilting toward higher-yield insurance patients.

It is pertinent to note that brownfield expansion typically turns breakeven within 3 months and margin accretive by end of Year 1 — well evidenced by last 5.5 years of sector data. The sharp ramp is underpinned by inherent infrastructure synergies: senior doctor headcount remains largely intact, with incremental hiring limited to junior and nursing staff — and even that at a count lower than what fresh infrastructure would warrant. This makes brownfield the most capital-efficient and fastest margin-accreting expansion modality in the sector.

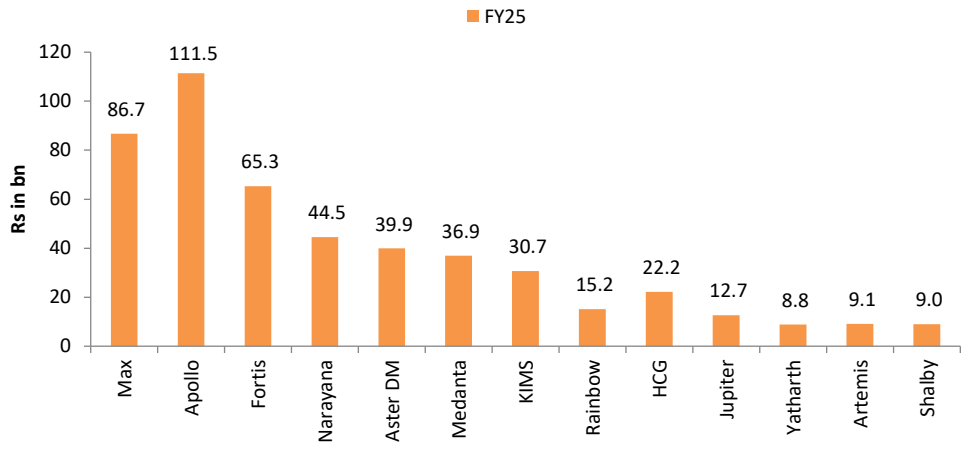
Moreover, the last five years have also seen hospitals advance toward asset-light business models, with O&M operations and infrastructure leasing gaining meaningful share. These initiatives — layered on top of margin expansion and capital-efficient brownfield deployment — have been a key driver of ROCE improvement across the sector, with marked improvement visible across names.

Exhibit 13: Strong 15.5% CAGR over FY20-25



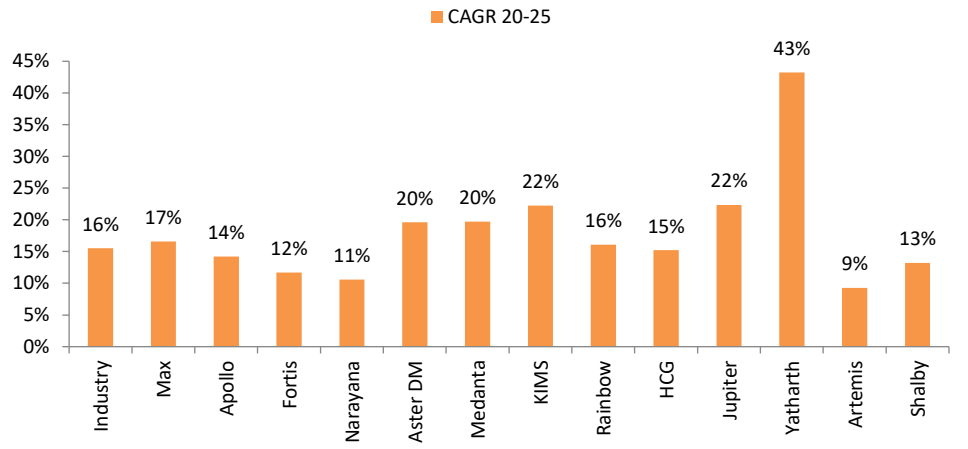
Source: Company Data, Equirus

Exhibit 14: Scale of Leading Hospital Chains in India – FY25 Revenue



Source: Company Data, Equirus

Exhibit 15: Robust double digit revenue growth across the hospitals



Source: Company Data, Equirus

Bed Expansion: Anatomy of ~14,000 Beds Added (FY20–9MFY26)

India's listed hospital universe added ~14,000 beds over FY20–9MFY26, with a clear acceleration post-FY23. Bed additions were measured at 3,427 in FY20–23, picking up sharply to 5,973 in FY23–25, with 9MFY26 alone contributing 4,545 beds — the fastest pace in the sector's history — as strengthening demand visibility and balance sheet confidence incentivised operators to step up capacity deployment. KIMS led absolute additions at 3,385 beds, followed by Fortis (2,534), Max (1,896), Aster (1,758), Medanta (1,340), and Yatharth (1,205) — together accounting for ~85% of total universe bed additions over the period.

Capacity cycle turns post-COVID triggered a step-up in capacity expansion

The expansion cycle was fundamentally **demand-driven**. COVID acted as a structural inflection point – elevating healthcare awareness, accelerating insurance penetration, and increasing elective procedure demand. As patients increasingly prioritized quality care and affordability improved via insurance, hospitals witnessed sustained occupancy and ARPOB strength, creating confidence for capacity expansion.

The nature of bed addition was diversified — only 30% were greenfield, with the balance split across brownfield (28%), acquisitions (38%), and O&M (19%). Financially stressed standalone hospitals increasingly divested or leased out facilities to organised players, creating a steady pipeline of inorganic opportunities that large chains exploited efficiently. This trend is further accelerating, driven by rising acceptance for quality care and deepening insurance penetration — both of which continue to tilt patient preference toward organised hospital networks, strengthening the case for further consolidation.

Exhibit 16: Industry Bed Addition Trend: Phased Expansion with Sharp Post-FY23 Acceleration

Bed Addition	FY20	Addition	FY23	Addition	FY25	Addition	9MFY26
Max	3,371	177	3,548	1,382	4,930	337	5,267
Apollo	7,491	369	7,860	165	8,025	47	8,072
Fortis	3,522	580	4,102	644	4,746	1,310	6,056
Narayana	6,487	-411	6,076	-331	5,745	5	5,750
Aster	3,693	624	4,317	842	5,159	292	5,451
Medanta	2,239	458	2,697	345	3,042	537	3,579
KIMS	3,079	861	3,940	1,704	5,644	820	6,464
Rainbow	1,371	234	1,605	330	1,935	350	2,285
HCG	2,068	-197	1,871	332	2,203	-	2,203
Jupiter	669	281	950	111	1,061	-13	1,048
Yatharth	1,100	305	1,405	200	1,605	700	2,305
Artemis	395	146	541	159	700	-	700
Shalby	2,062	-	2,062	90	2,152	160	2,312
Total	37,547	3,427	40,974	5,973	46,947	4,545	51,492

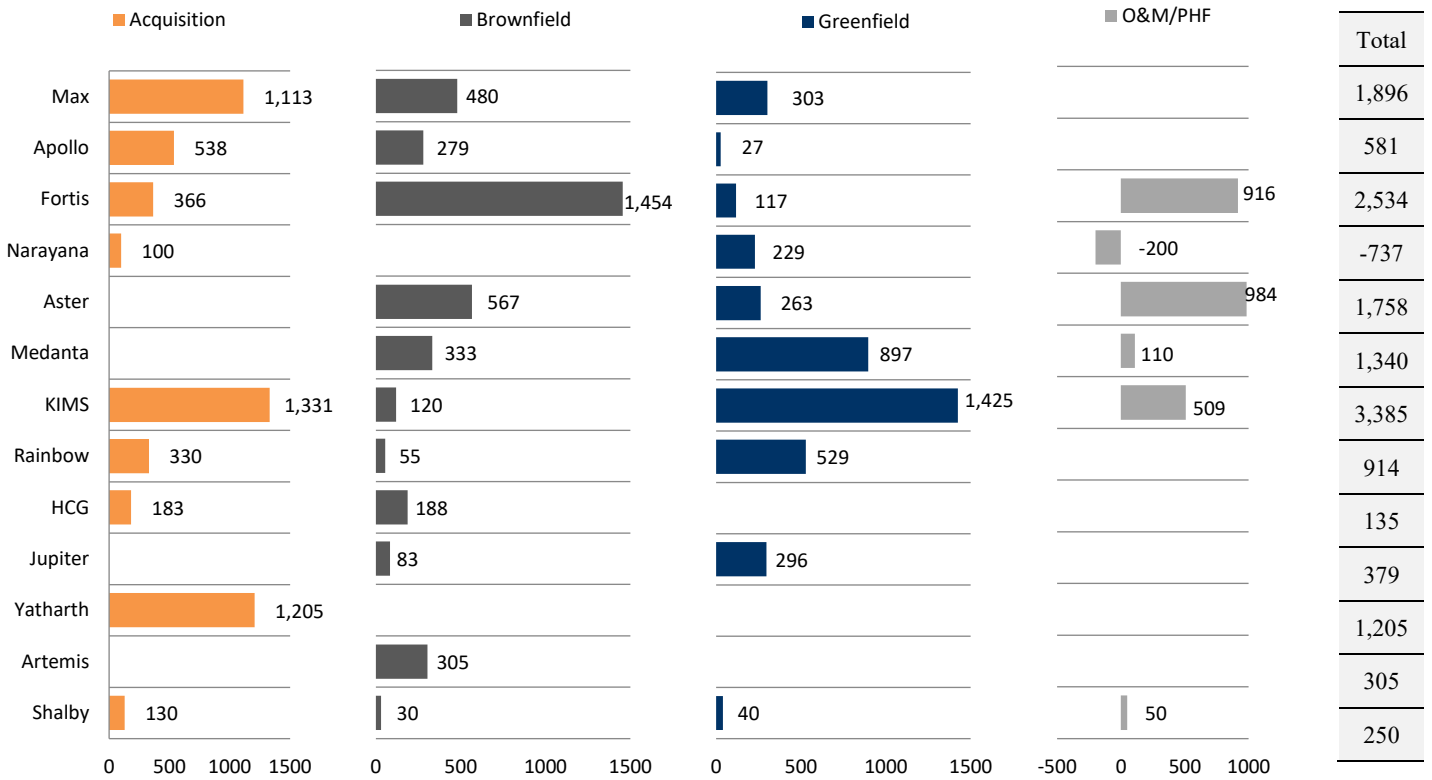
Source: Company Data, Equirus

55% of bed additions driven by inorganic routes—acquisitions and O&M enabling faster, capital-efficient scaling.

Cmpanies complemented inorganic growth with disciplined organic expansion. Greenfield projects, led by Medanta, KIMS, and Rainbow, were used selectively to enter new high-growth micro-markets and build regional clusters. In contrast, brownfield expansion, driven by Fortis, Aster, and Artemis, focused on sweating existing assets-unlocking capacity in high-demand locations with lower capital intensity.

Moreover, a notable shift during this cycle was the parallel focus on yield optimization, with players like Apollo and Narayana prioritizing ARPOB expansion over aggressive bed addition by upgrading infrastructure and converting general wards into higher-yield private formats.

Exhibit 17: Inorganic & Asset-Light Models Drive Majority of Bed Additions

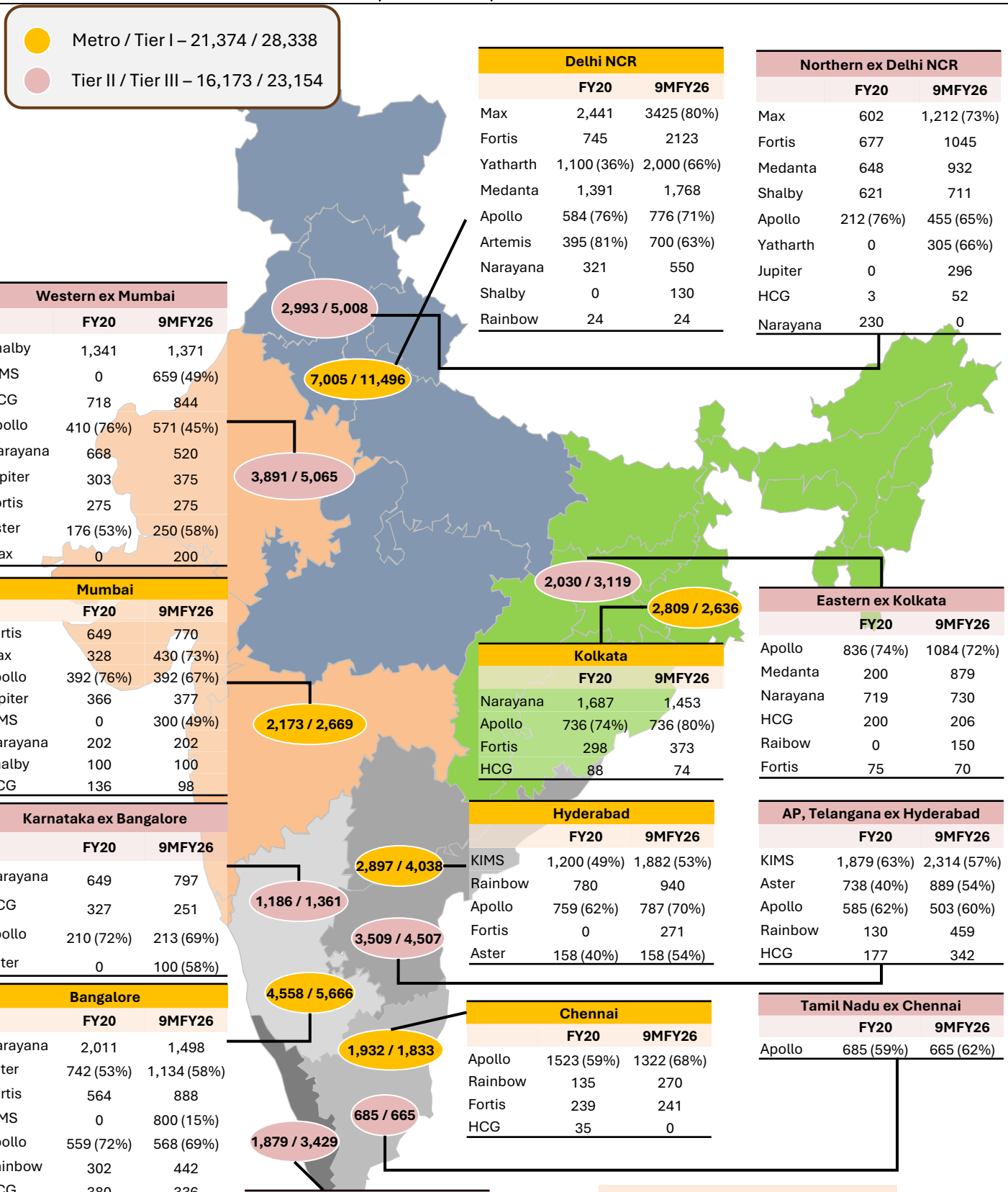


Source: Company Data, Equirus

Geographically, expansion followed a dual-track strategy. Tier-II/III markets added ~7,000 beds (16k → 23k), driven by under penetration and strong demand, while metro/Tier-I markets added ~7,000 beds (21k → 28k), supported by brownfield expansion and sustained premium demand. Regionally, Delhi NCR led with ~4,500 bed additions, followed by strong momentum in North and South India.

Overall, the expansion over FY20–9MFY26 reflects a structurally evolved model-less greenfield-heavy, more inorganic and asset-light, and tightly aligned with demand visibility. Unlike previous cycles, capacity addition has been measured, return-focused, and cluster-driven, reducing the risk of overcapacity while sustaining long-term growth.

Exhibit 18: Bed Mix Evolution: Metro/Tier I vs Tier II/III (FY20–9MFY26)



Delhi NCR	FY20	9MFY26
Max	2,441	3425 (80%)
Fortis	745	2123
Yatharth	1,100 (36%)	2,000 (66%)
Medanta	1,391	1,768
Apollo	584 (76%)	776 (71%)
Artemis	395 (81%)	700 (63%)
Narayana	321	550
Shalby	0	130
Rainbow	24	24

Northern ex Delhi NCR	FY20	9MFY26
Max	602	1,212 (73%)
Fortis	677	1045
Medanta	648	932
Shalby	621	711
Apollo	212 (76%)	455 (65%)
Yatharth	0	305 (66%)
Jupiter	0	296
HCG	3	52
Narayana	230	0

Western ex Mumbai	FY20	9MFY26
Shalby	1,341	1,371
KIMS	0	659 (49%)
HCG	718	844
Apollo	410 (76%)	571 (45%)
Narayana	668	520
Jupiter	303	375
Fortis	275	275
Aster	176 (53%)	250 (58%)
Max	0	200

Mumbai	FY20	9MFY26
Fortis	649	770
Max	328	430 (73%)
Apollo	392 (76%)	392 (67%)
Jupiter	366	377
KIMS	0	300 (49%)
Narayana	202	202
Shalby	100	100
HCG	136	98

Kolkata	FY20	9MFY26
Narayana	1,687	1,453
Apollo	736 (74%)	736 (80%)
Fortis	298	373
HCG	88	74

Eastern ex Kolkata	FY20	9MFY26
Apollo	836 (74%)	1084 (72%)
Medanta	200	879
Narayana	719	730
HCG	200	206
Raibow	0	150
Fortis	75	70

Karnataka ex Bangalore	FY20	9MFY26
Narayana	649	797
HCG	327	251
Apollo	210 (72%)	213 (69%)
Aster	0	100 (58%)

Hyderabad	FY20	9MFY26
KIMS	1,200 (49%)	1,882 (53%)
Rainbow	780	940
Apollo	759 (62%)	787 (70%)
Fortis	0	271
Aster	158 (40%)	158 (54%)

AP, Telangana ex Hyderabad	FY20	9MFY26
KIMS	1,879 (63%)	2,314 (57%)
Aster	738 (40%)	889 (54%)
Apollo	585 (62%)	503 (60%)
Rainbow	130	459
HCG	177	342

Bangalore	FY20	9MFY26
Narayana	2,011	1,498
Aster	742 (53%)	1,134 (58%)
Fortis	564	888
KIMS	0	800 (15%)
Apollo	559 (72%)	568 (69%)
Rainbow	302	442
HCG	380	336

Chennai	FY20	9MFY26
Apollo	1523 (59%)	1322 (68%)
Rainbow	135	270
Fortis	239	241
HCG	35	0

Tamil Nadu ex Chennai	FY20	9MFY26
Apollo	685 (59%)	665 (62%)

Kerala	FY20	9MFY26
Aster	1,879 (65%)	2,920 (66%)
KIMS	0	509 (38%)

Total – 37,547 / 51,492

Source: Company Data, Equirus
Bracket no. stands for occupancy %
Operating Beds – Apollo, Fortis, Medanta, HCG & Jupiter, while others are Bed Capacity

Occupancy, IPD & Patient Volume: The Volume Growth Proof Point

Occupancy: Discipline at Scale

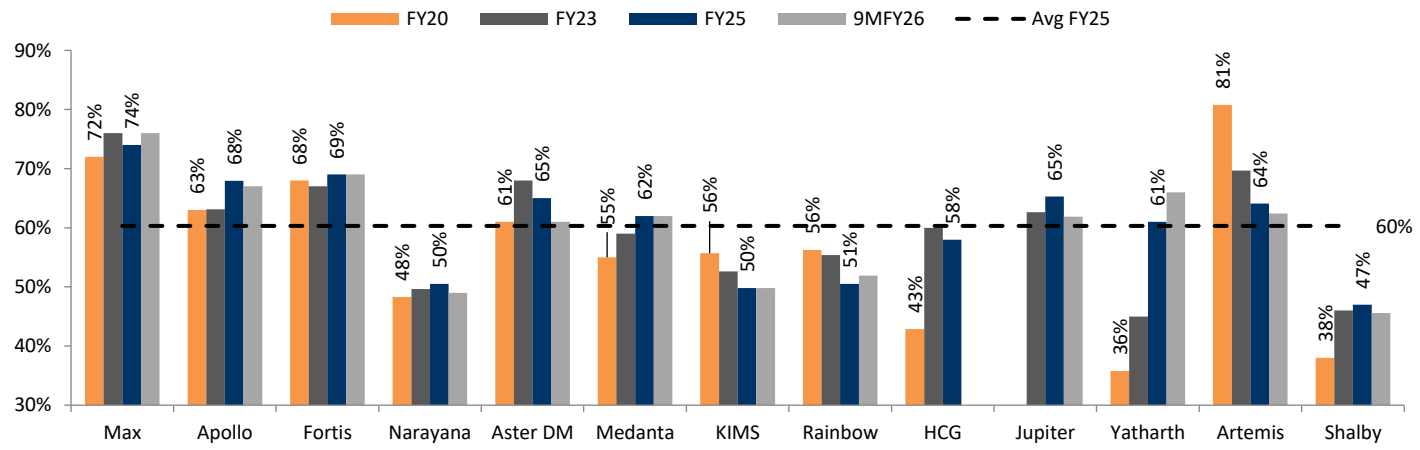
The single most compelling validation of the sector's expansion discipline is not the bed count – it is what happened to occupancy and patient throughput *while* beds were being added. The listed hospital universe collectively added ~14,000 beds over FY20–9MFY26, a 37% expansion in installed capacity, yet aggregate occupancy either held firm or improved versus FY20 levels, and inpatient volumes grew at high-single to mid-double digit CAGRs across virtually every operator.

Beds up, occupancy intact

The average occupancy of ~60% is useful as a reference, but the distribution around it is more revealing. **Five of the six largest players currently operate above this threshold**, a fact that directly contradicts any narrative of oversupply. Capacity addition has been absorbed efficiently, and in several cases has catalysed further occupancy improvement as cluster depth enabled referral network densification and brand recall in new micro-markets.

Even in cases where occupancy has moderated—such as **KIMS and Rainbow (~5% dip)**-this is largely attributable to **recent large-scale capacity additions**, particularly greenfield-led, and should be seen as a **transient ramp-up impact rather than structural weakness**. In contrast, **Yatharth** – the most instructive case study. Near-entirely acquisition-led expansion (beds: 1,100 in FY20 → 2,305 in 9MFY26), yet occupancy *improved* over the period. Yatharth's playbook has been deliberately concentrated in under-penetrated Delhi NCR and adjacent UP catchments – markets where organised hospital supply remains structurally thin relative to population density. In such geographies, an acquired asset's historical occupancy is the floor, not the run-rate, because physical infrastructure, approvals, and staff continuity are inherited at closing. Ramp is faster; dilution is limited.

Exhibit 19: Steady Occupancy Expansion Across Players Over FY20–FY25



Source: Company Data, Equirus

IPD & Total Patient Volume: The Demand Reality

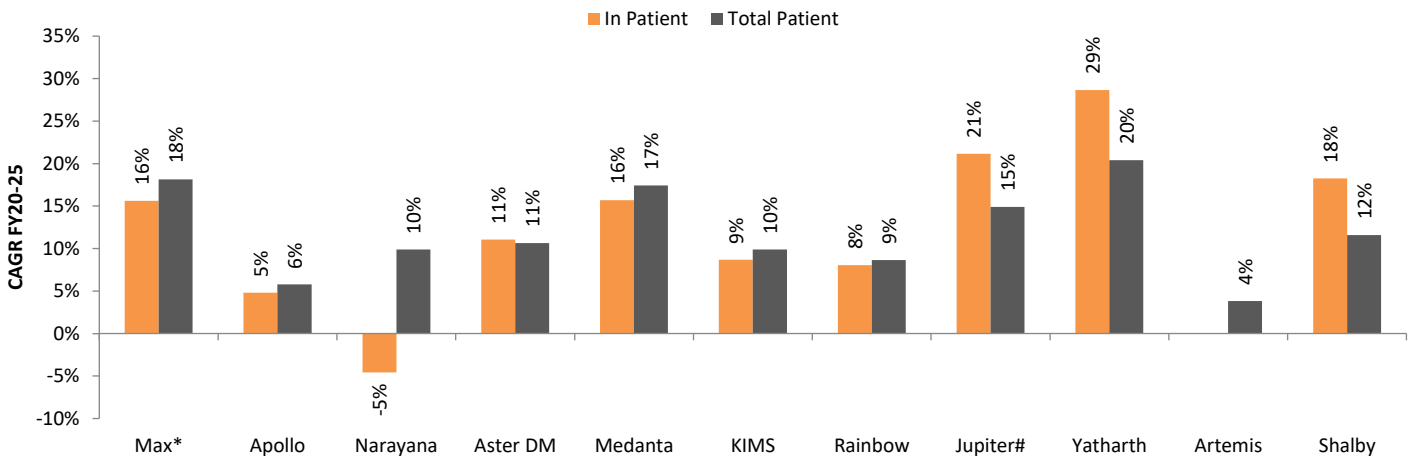
Occupancy is a ratio – it must be read alongside the absolute volume numbers to understand the magnitude of the opportunity being captured. The IPD and total patient data confirms that the occupancy story is not about denominator management; it is about genuine demand absorption.

This is real demand: IPD growth confirms occupancy strength

Across the universe, inpatient volumes grew at a 9–29% CAGR over FY20–FY25, with the pace accelerating post-FY22 as post-COVID elective demand normalised and newly added capacity began contributing. **Yatharth** leads the universe on IPD CAGR at 29%. **Medanta** delivered a 16% IPD CAGR entirely organic, validating its greenfield ramp thesis. **KIMS** grew IPD from 9% CAGR despite the occupancy dilution from its new beds, making clear that absolute volume expansion continued uninterrupted even during the occupancy ramp period.

OPD volumes across the universe have seen a sharp expansion over FY20–25 — Apollo's OPD grew at 6% CAGR to 21.8 lakh visits while KIMS expanded at 10% CAGR to 18.3 lakh visits. As bed capacity scaled and OPD infrastructure deepened in new geographies, this has provided a steady tailwind to overall IPD volumes — with catchment maturity and brand recall driving higher IPD conversion as hospitals establish presence in new markets.

Exhibit 20: Strong IPD Momentum Supporting Total Patient Growth



Source: Company Data, Equirus

*Max – FY23, #Jupter - FY22

Inorganic-Led Expansion: Faster Ramp, More Durable Volume Growth

The sector's focus toward acquisitions, brownfield densification, and asset-light O&M contracts has fundamentally compressed the occupancy ramp timeline and made volume growth more durable. A greenfield hospital requires 36–60 months to reach mature utilisation and absorbs significant cash during ramp. An acquired or O&M asset enters the network with an existing patient base, operational staff, and local brand recall – mature occupancy is typically achievable within 12–24 months. With ~55% of total bed additions over FY20–9MFY26 being inorganic, operators have systematically sidestepped the prolonged utilisation drag that defined greenfield-heavy cycles of the prior decade.

The IPD and total patient data makes clear that volume growth was broad-based, accelerating, and structurally underpinned by real demand. Five of six large operators maintained or improved occupancy through a cycle that added 37% capacity.

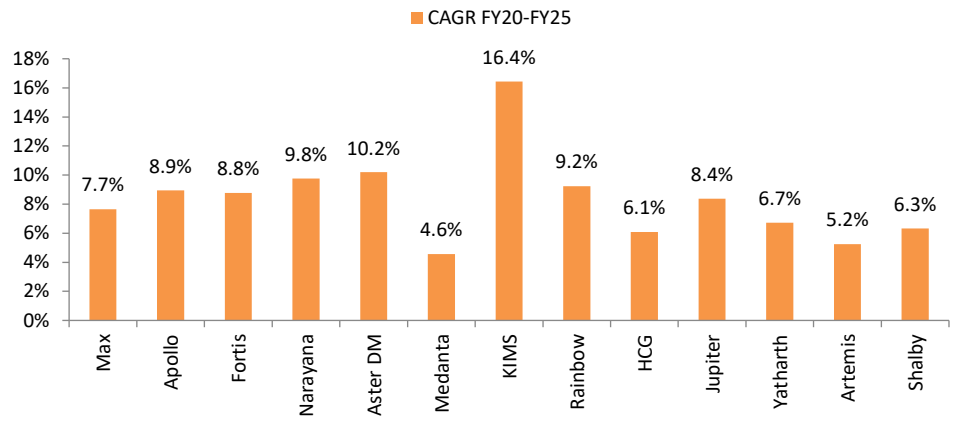
Consistent ARPOB Compounding Across the Universe

Five levers, one outcome—pricing, CONGO mix, payer shift, geography, and ALOS jointly driving ARPOB expansion

Indian Hospital sector saw a consistent ARPOB expansion in last 5 years. Almost all the companies delivered high single-digit ARPOB compounding over FY20–FY25 – with the average settling at a remarkably consistent 7–10% p.a. Growth in ARPOB was led by – 1) **price hikes of 3-4%**, 2) favourable case mix (CONGO gained share almost 3-4% in 5 yrs), 3) structural shift in payor mix from cash to TPA/insurance with continuous check on institutional billings, 4) Geographical expansion to high ARPOB locations (e.g. KIMS) and 5) lowering ALOS (surgical is high margin). All five drivers worked in tandem, each reinforcing the others to produce a cumulative ARPOB expansion of 50–60% across the universe over five years.

Meantime, entry into newer geographies like Bihar or taking up O&M contracts where the ARPOB is lower has also negatively impacted the ARPOB growth profile. It was evident in Medanta which had entered Bihar market through PPP and took over O&M operations in Ranchi.

Exhibit 21: Healthy ARPOB CAGR - Structural Improvement in Realizations

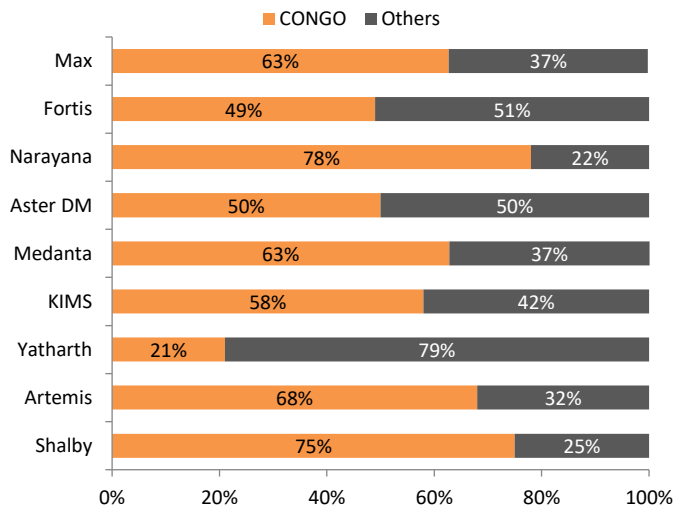


Source: Company Data, Equirus

Case Mix Enrichment — The CONGO Effect

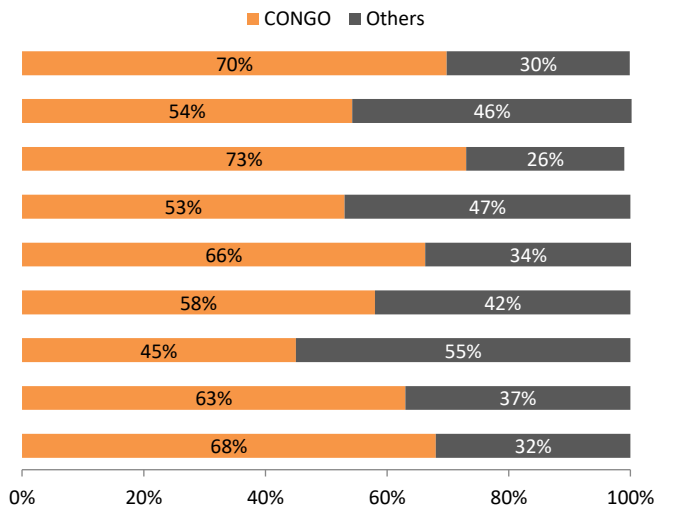
Case mix enrichment was the first structural amplifier. CONGO specialties – collectively gained 300–400 bps of revenue share across the universe over FY20–FY25, with Max, Fortis, and Yatharth each recording shifts of 500 bps or more. CONGO procedures command a structurally higher ARPOB than general medicine not because of pricing discretion, but because of treatment complexity, high margin surgical procedure involvement, and higher adjunct therapy attachment. As CONGO mix rises, blended ARPOB rises mechanically, independent of any tariff action. This is the most durable component of the yield story — case mix is sticky and reversing it would require a deliberate strategic retreat that no operator has any incentive to execute.

Exhibit 22: Rise in CONGO Mix from FY20…



Source: Company Data, Equirus

Exhibit 23: …3–4% Share Increase Across Hospitals by 9MFY26



Source: Company Data, Equirus

Payor Mix Shift: Insurance as an ARPOB Multiplier

Payor mix has seen a structural shift over FY20-25 with ~7-8% of revenues migrating from cash-pay toward insurance and TPA. Contrary to the narrative of hospitals losing pricing power as cash-pay share falls, insured patients typically seek higher-complexity procedures and have lower price sensitivity — all of which are structurally ARPOB accretive.

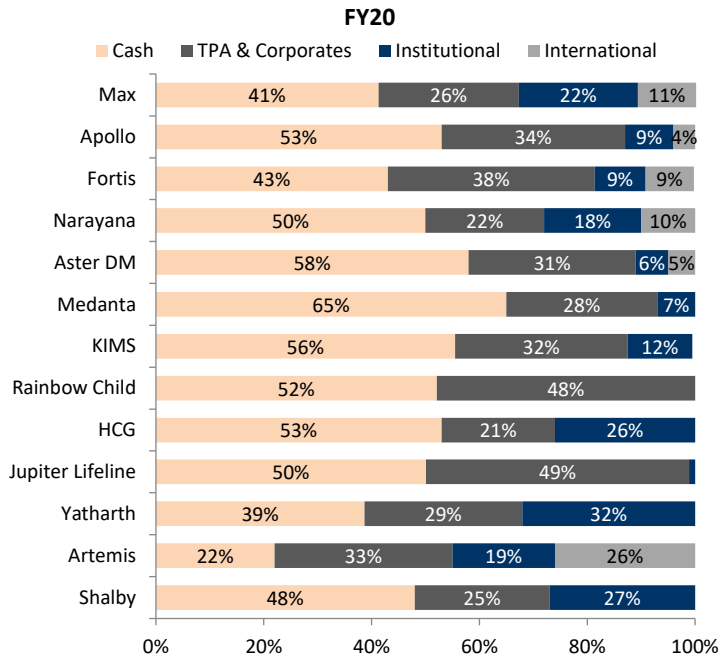
The past five years made this dynamic empirically visible. Across the universe, around 7–8% of revenues structurally migrated from cash-pay toward insurance and TPA – not as a one-quarter event, but as a steady, compounding shift visible in every annual data point from FY20 through FY25.

Large hospital operators (Fortis, Max) have consistently highlighted their intent to reduce institutional billing, given it delivers 30–40% lower realizations versus cash/insurance and dilutes ARPOB. However, execution has been more nuanced. During the ramp-up phase of new capacity (greenfield or acquisitions), institutional payors act as a base-load occupancy driver, helping hospitals stabilize utilization, absorb fixed costs, and build local presence. As facilities mature, players gradually shift toward higher-yield cash and insured patients. This sequencing effect explains why institutional mix has remained broadly stable at a portfolio level over the past five years, despite clear strategic intent to reduce it. KIMS stands out as an exception, having structurally reduced institutional exposure, with the benefit reflected in its industry-leading ARPOB growth, highlighting the monetization potential of payor mix improvement as assets mature.

In **brownfield expansions**, scheme patients play a useful role in the early ramp phase — since incremental fixed costs are already absorbed by the existing base, even low-yielding scheme volumes are immediately margin-accretive. Hospitals deliberately fill new brownfield beds with scheme flow from day one to improve occupancy and contribution, then progressively recalibrate the payer mix towards cash and insurance as organic demand builds.

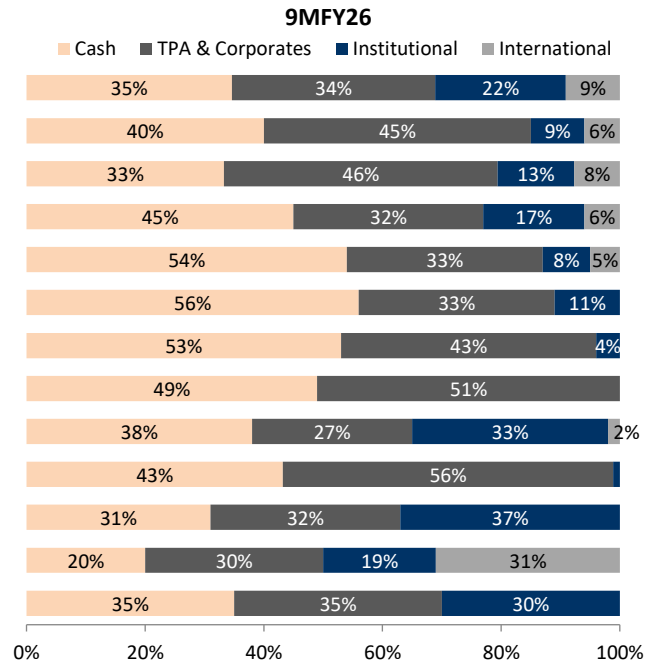
The CGHS tariff reset in FY24 ended a decade-long stagnation, delivering a direct revenue tailwind primarily through an 80–100% surge in Consultation and Room Rent rates. Beyond the immediate P&L impact, this recalibrated the sector's "price floor," providing critical leverage for aggressive TPA renegotiations and reduced discounting across private payer segments. Following mid-cycle adjustments in FY25, the mid FY26 overhaul introduced long-term structural efficiency by revising 2,000+ surgical and diagnostic package rates by an average of 15–20%. This transition to a tiered, accreditation-linked framework—offering premiums for Super Speciality Hospitals (+15%) and NABH accreditation—has effectively de-risked government business. This multi-year cycle has successfully pivoted the sector from a low-yield volume game to a value-driven ARPOB growth story, ensuring sustained margin expansion and a more balanced pricing regime.

Exhibit 24: Structural Shift from FY20…



Source: Company Data, Equirus

Exhibit 25: …7–8% Transition from Cash to Ins./TPA Pay



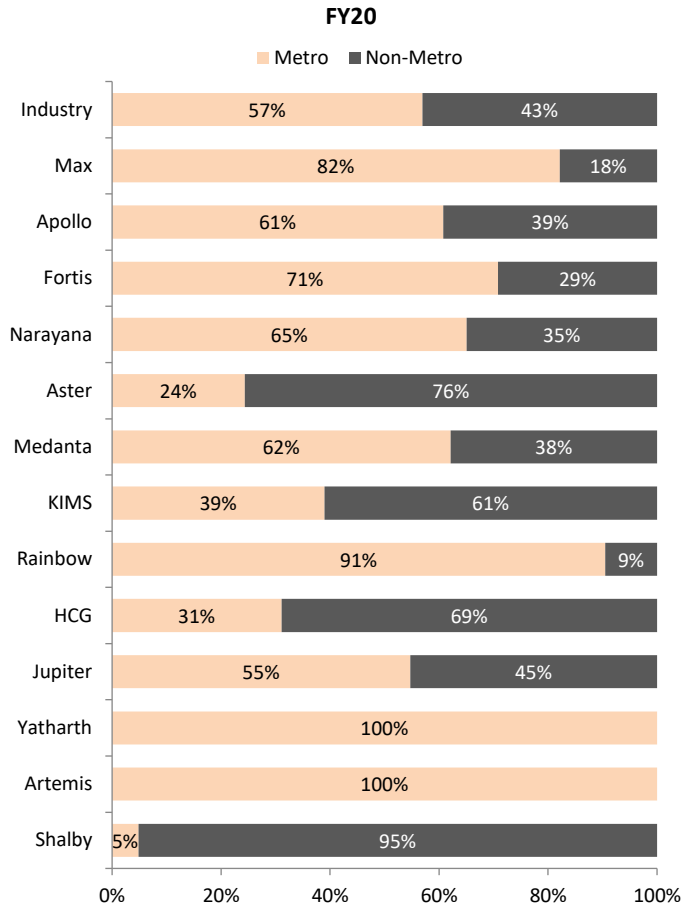
Source: Company Data, Equirus

Geographic Expansion into High-ARPOB Regions: Metro Premium with Selective Tier II/III Penetration

Geographic expansion into higher-ARPOB catchments was the third amplifier. The conventional concern – that Tier-2/3 expansion dilutes blended ARPOB – was empirically disproved over this cycle. The players who expanded most aggressively into new geographies did so by selecting markets with strong insurance penetration, limited organised hospital supply, and rapidly rising complexity demand.

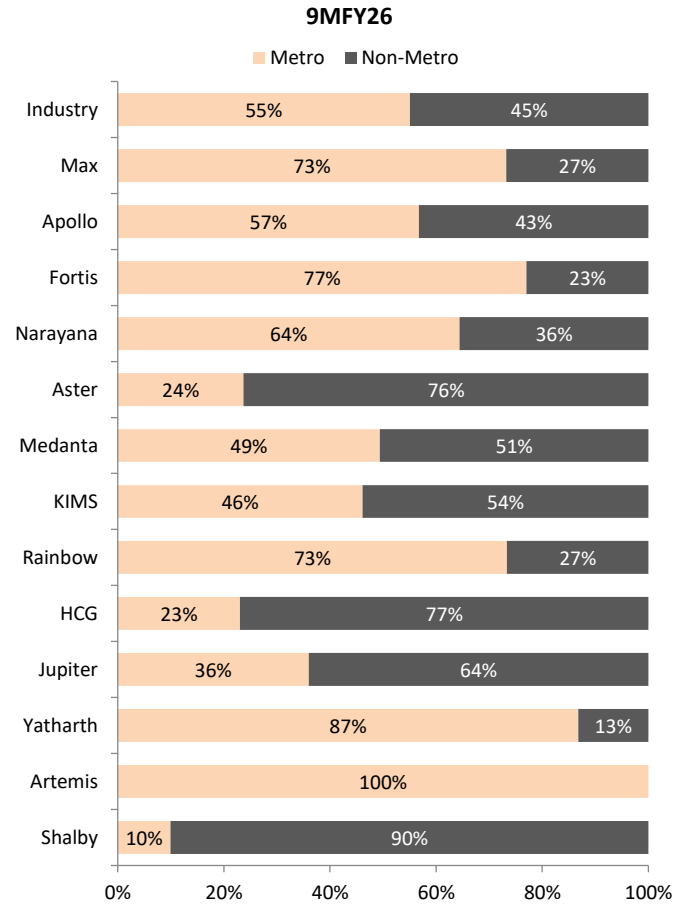
KIMS is the sharpest illustration: starting FY20 as a predominantly Hyderabad/AP chain with the lowest ARPOB in the universe (Rs 18,307), its deliberate expansion into Maharashtra (Nagpur, Nashik, Thane, Sangli) and Kerala (Kannur, Kollam) — both structurally higher-ARPOB markets — produced the highest ARPOB CAGR in the universe at 16.4% p.a., with ARPOB rising to Rs 39,158 by FY25. Aster narrated an equally compelling story: deepening Karnataka alongside its mature Kerala base grew blended ARPOB from Rs 27,700 to Rs 45,000 by FY25 (+10.2% CAGR), demonstrating that geographic diversification and ARPOB accretion were complementary forces, not competing ones.

Exhibit 26: From Metro-Centric Presence…



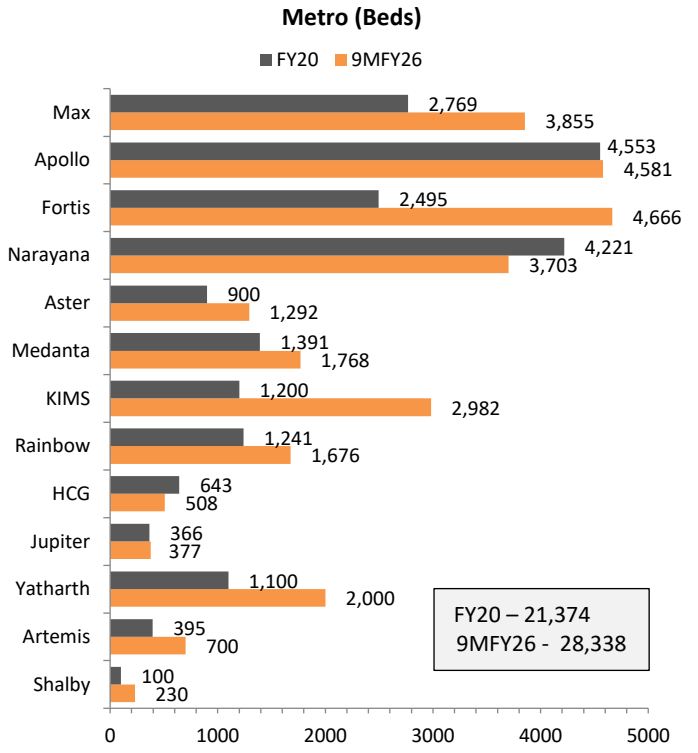
Source: Company Data, Equirus

Exhibit 27: … to Broader Tier II & III Expansion



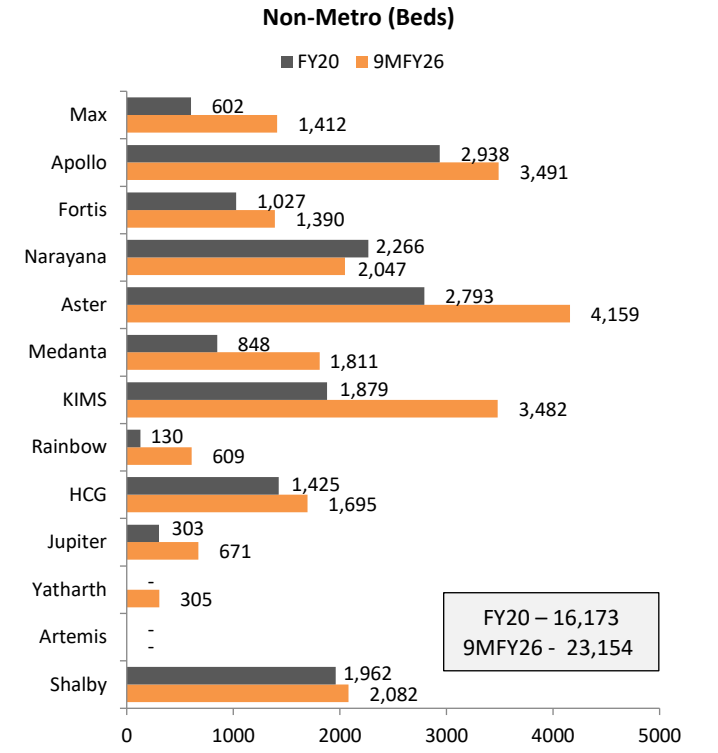
Source: Company Data, Equirus

Exhibit 28: One Foot in Metro Dominance…



Source: Company Data, Equirus

Exhibit 29: … Another in Underpenetrated Non-Metro Regions



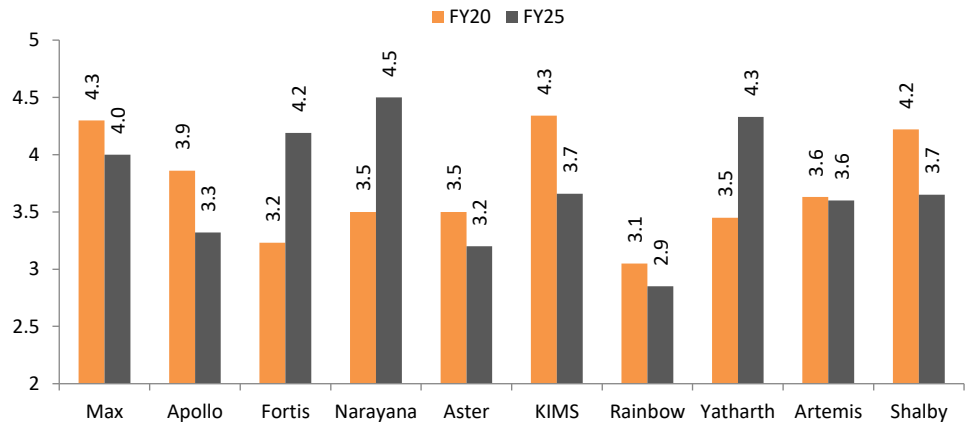
Source: Company Data, Equirus

ALOS Dynamics – Shorter Stays, Higher Daily Intensity

ALOS across the listed universe has largely trended downward over FY20–FY25, reflecting a deliberate shift in case mix toward surgical and procedural work from medical cases. Apollo compressed ALOS from 3.9 to 3.3 days, Aster from 3.5 to 3.2, KIMS from 4.3 to 3.7, Max from 4.3 to 4.0, and Shalby from 4.2 to 3.7. The counterintuitive exceptions — Fortis (3.2→4.2), Narayana (3.5→4.5), and Yatharth (3.5→4.3) — Narayana has seen rise owing to increasing share of revenues from Cayman hospital.

The ALOS compression is a structural driver of ARPOB expansion. As the sector's surgery-to-medicine revenue mix has shifted to approximately 60:40 in favour of surgical and procedural work, shorter stays mean each bed cycles through more patients per year, increasing effective revenue per operational bed without additional infrastructure. Surgical cases command a disproportionately higher daily billing rate. This mechanically inflates ARPOB even without headline price hikes contributes to the sector's 7–9% p.a. ARPOB compounding over FY20–FY25.

Exhibit 30: Structural Decline in ALOS, with Select Players Seeing Uptick on Case Mix Shift



Source: Company Data, Equirus

Aggressive Capex, Smarter Execution — How Acquisition, Brownfield and Asset-Light Models Turned Expansion into a Margin and Return Story

The Capex Surge – Aggressive, Broad-Based, and Accelerating

The Indian listed hospital sector embarked on its most aggressive capital deployment cycle in history over FY20–FY25. Aggregate annual hospital capex across the 13-company universe grew nearly 4x – from Rs 19 bn in FY20 to Rs 80 bn in FY25 – a 33% CAGR, cumulatively amounting to approximately Rs 220 bn deployed over the five-year window. The aggression became more heavier in recent years post FY22.

Capex up 4x in 5 years - from Rs 19 bn in FY20 to Rs 80 bn in FY25

Max Healthcare was the most dramatic: capex grew from just Rs 0.9 bn in FY20 to Rs 15.9 bn in FY25 – a near-18x increase – as it simultaneously executed on Lucknow, Nagpur, and Noida acquisitions while commencing brownfield expansions across Delhi and Mumbai. KIMS was equally aggressive in relative terms: from Rs 0.5 bn in FY20 to Rs 11.4 bn in FY25 – a 21x surge – funding the Maharashtra greenfield cluster (Nagpur, Nashik, Thane), Karnataka entry, and Kerala O&M investments simultaneously. The CWIP data corroborates the pipeline's depth: major hospitals sitting at heavy CWIP compared to FY20 like Max's CWIP surged from Rs 55 mn (FY20) to Rs 12.3 bn (9MFY26); Apollo from Rs 2.4 bn to Rs 10.3 bn; KIMS from Rs 20 mn to Rs 5.7 bn; Jupiter from Rs 100 mn to Rs 3.0 bn – all confirming substantial capacity still in the pipeline yet to convert to operational revenue.

Exhibit 31: Broad based surge in Capex

Capex (Rs in mn)	FY20	FY21	FY22	FY23	FY24	FY25	1HFY26	FY21-25	CWIP FY20	CWIP FY25
Industry	**13,961**	**14,070**	**25,367**	**42,123**	**63,383**	**80,469**	**52,731**	**2,25,412**	**11,755**	**46,309**
Max	901	1,168	6,553	3,322	14,108	15,846	8,354	**40,997**	55	12,259
Apollo	5,106	2,844	4,308	11,435	10,139	14,152	7,132	**42,878**	2,360	10,260
Fortis	1,060	1,767	535	2,843	7,202	7,161	10,650	**19,508**	2,040	3,060
Narayana	1,094	588	1,948	6,280	6,099	7,594	2,909	**22,509**	120	1,350
Aster	239	1,018	2,081	2,800	7,586	3,548	2,339	**17,033**	1,110	4,010
Medanta	1,877	1,419	2,731	2,280	2,772	6,449	4,253	**15,651**	3,820	3,180
KIMS	532	936	1,711	7,881	6,459	11,374	7,407	**28,361**	20	5,730
Rainbow	744	759	622	1,411	3,534	1,456	3,010	**7,782**	430	530
HCG	1,074	354	1,879	1,277	2,324	4,045	1,560	**9,878**	460	960
Jupiter	445	2,416	911	769	852	3,203	1,701	**8,150**	100	2,960
Yatharth	433	213	521	198	1,135	4,034	1,972	**6,100**	620	1,100
Artemis	259	313	1,085	1,449	722	906	635	**4,476**	560	370
Shalby	198	276	483	177	452	701	808	**2,089**	60	540

Source: Company Data, Equirus

EBITDA and Margins – Expanded Simultaneously with Capex

Margins expanded alongside capex – inorganic and brownfield expansion kept the margin up

What makes the capex cycle particularly remarkable is what happened to profitability alongside it – margins did not compress under the weight of new beds; they expanded materially and consistently. Industry EBITDA grew from Rs 37.5 bn in FY20 to Rs 115.7 bn in FY25 – a 25.3% CAGR, outpacing revenue growth of 15.5% by nearly 1,000 bps every year. Blended EBITDA margins expanded by approximately 780 bps – from 15.7% in FY20 to 23.5% in FY25 – a sector-wide margin re-rating. At the individual company level, the expansion was even more pronounced for chains that executed well on ARPOB and occupancy simultaneously. Max Healthcare expanded margins by 1,210 bps (14.7%→26.8%) – the largest absolute expansion in the universe – as the Radiant/BLK hospitals reached maturity and operating leverage kicked in sharply. Medanta expanded by 1,060 bps (13.2%→23.8%), Aster by 880 bps, Fortis by 850 bps, and Narayana by 750 bps. The drivers were consistent across chains: operating leverage, group procurement benefits in pharma and consumables as bed counts grew, fast new hospital ramp up crossed breakeven and matured into profitability and transition of general wards into high yielding private wards (Apollo & Narayana). The one notable outlier is Yatharth, which saw margins barely move (-70 bps, 25.7%→25.0%) – not because of deterioration but because it started from an already-high base and was simultaneously absorbing multiple new hospital ramp-ups (Jhansi, Faridabad, Delhi) that imposed fixed cost drag.

Exhibit 32: Robust EBITDA CAGR of mid 20s

EBITDA (Rs in mn)	FY20	FY21	FY22	FY23	FY24	FY25	9MFY26	CAGR FY20-25
Industry	37,512	28,449	66,063	84,069	97,788	1,15,706	98,769	25%
Max	5,900	6,367	13,900	16,360	19,070	23,190	19,550	31%
Apollo	10,750	6,087	18,031	21,332	23,559	27,005	22,887	20%
Fortis	4,500	2,494	6,555	8,622	10,579	13,390	12,785	24%
Narayana	3,395	199	3,580	6,750	7,772	8,958	7,842	21%
Aster	2,130	2,020	4,070	5,380	6,870	8,740	7,650	33%
Medanta	1,977	1,977	4,539	6,461	7,942	8,805	7,034	35%
KIMS	2,450	3,709	5,158	6,040	6,404	8,146	6,122	27%
Rainbow	1,970	1,628	3,049	3,964	4,578	4,610	3,995	19%
HCG	1,722	1,285	2,408	3,051	3,374	3,964	3,459	18%
Jupiter	826	713	1,574	2,117	2,411	2,981	2,489	29%
Yatharth	376	670	1,108	1,338	1,799	2,202	2,032	42%
Artemis	659	420	665	1,037	1,448	1,859	1,598	23%
Shalby	856	881	1,426	1,616	1,982	1,857	1,326	17%

Source: Company Data, Equirus

Exhibit 33: Major players converged to low to mid 20s EBITDA Margin

EBITDA Margin	FY20	FY21	FY22	FY23	FY24	FY25	9MFY26
Industry	16%	14%	22%	23%	23%	24%	23%
Max	15%	18%	27%	28%	28%	27%	26%
Apollo	19%	14%	27%	25%	24%	24%	25%
Fortis	12%	8%	15%	17%	19%	21%	22%
Narayana	13%	1%	12%	18%	19%	20%	22%
Aster	13%	12%	17%	19%	20%	22%	23%
Medanta	13%	14%	21%	24%	24%	24%	22%
KIMS	22%	28%	31%	27%	26%	27%	22%
Rainbow	27%	25%	31%	34%	35%	30%	32%
HCG	16%	13%	17%	18%	18%	18%	18%
Jupiter	18%	15%	21%	24%	23%	24%	23%
Yatharth	26%	29%	28%	26%	27%	25%	24%
Artemis	11%	10%	12%	14%	17%	20%	21%
Shalby	18%	21%	22%	22%	23%	21%	19%

Source: Company Data, Equirus

Operating Cash Flow and Free Cash Flow — Self-Funding the Cycle

Expansion largely self-funded - strong CFO-to-capex coverage across leading players

The most powerful validation that the capex cycle was absorbed – not merely financed – is the simultaneous and large acceleration in operating cash flows. Industry CFO grew from Rs 30 bn (FY20) to Rs 81.7 bn (FY25) – nearly 2.7x — directly tracking EBITDA expansion. At the individual company level, the CFO acceleration was striking: Fortis went from negative Rs 2 bn CFO in FY20 to Rs 10.9 bn in FY25. Apollo sustained the largest absolute CFO at Rs 23.5 bn in FY25, covering its Rs 14.0 bn hospital capex, having 0.5x capex/CFO over the last 5 years – firmly self-funding. Fortis covered capex at 0.6x, Rainbow at 0.5x, and Narayana at 1.0x over the 5 years. Max, at the peak of its expansion program, ran negligible FCF over 5 years (CFO Rs 46.8 bn vs. capex Rs 41.0 bn, FCF of Rs 5.8 bn) – not a structural concern but a natural peak-capex trough before new beds generate returns. On FCF, Apollo (Rs 40.2 bn), Fortis (Rs 12.3 bn), Medanta (Rs 8.3 bn), and Rainbow (Rs 7.1 bn) were all FCF positive over FY20-FY25 – generating surplus cash even after fully funding their expansion programs from operations. The companies in negative FCF territory over 5 years – KIMS (Rs -7.0 bn), Yatharth (Rs -3.5 bn), and Jupiter (Rs -1.8 bn) – are doing so precisely because they are in the most aggressive phase of their expansion cycles, and in each case the FCF burn is funded by IPO/QIP proceeds or modest debt rather than structural cash shortfalls.

Exhibit 34: Viable self funded growth

CFO (Rs in mn)	FY20	FY21	FY22	FY23	FY24	FY25	Total FY21-25	Capex/CFO	1HFY26
Industry	19,725	26,640	43,233	59,781	63,836	81,697	2,75,187	0.8	51,141
Max	244	288	7,425	13,281	11,217	14,593	46,804	0.9	8,176
Apollo	12,043	11,832	14,102	14,136	19,466	23,512	83,048	0.5	13,253
Fortis	-1,945	2,105	4,613	5,298	8,880	10,893	31,789	0.6	6,760
Narayana	3,178	1,266	3,002	6,406	6,421	5,134	22,229	1.0	6,902
Aster	-1,723	911	2,008	2,716	-506	4,249	9,377	1.8	3,192
Medanta	1,751	2,418	3,113	5,902	6,157	6,373	23,961	0.7	3,285
KIMS	2,015	3,560	3,240	3,944	4,837	5,818	21,400	1.3	2,846
Rainbow	1,736	1,452	2,255	3,420	3,520	4,257	14,904	0.5	2,076
HCG	731	723	1,406	1,626	1,757	1,684	7,195	1.4	1,923
Jupiter	488	847	947	812	1,117	2,655	6,379	1.3	1,301
Yatharth	203	249	386	425	0	1,499	2,559	2.4	485
Artemis	460	182	515	1,127	850	1,345	4,020	1.1	805
Shalby	541	808	221	688	119	-314	1,522	1.4	138

Source: Company Data, Equirus

Exhibit 35: Strong FCF at Mature Players vs Persistent Burn in Expansion Phase

FCF (Rs in mn)	FY20	FY21	FY22	FY23	FY24	FY25	Total FY21-25	1HFY26
Industry	5,764	12,570	17,866	17,658	453	1,228	49,775	-1,589
Max	-657	-880	872	9,959	-2,892	-1,253	5,807	-178
Apollo	6,937	8,988	9,795	2,701	9,327	9,360	40,170	6,121
Fortis	-3,005	338	4,079	2,455	1,679	3,731	12,281	-3,890
Narayana	2,085	678	1,054	126	323	-2,459	-279	3,993
Aster	-1,962	-107	-73	-84	-8,092	700	-7,656	853
Medanta	-127	998	382	3,621	3,385	-77	8,310	-968
KIMS	1,483	2,624	1,529	-3,937	-1,622	-5,556	-6,961	-4,561
Rainbow	992	693	1,633	2,009	-14	2,801	7,122	-934
HCG	-343	369	-473	349	-567	-2,361	-2,683	362
Jupiter	43	-1,569	37	43	266	-548	-1,772	-400
Yatharth	-230	36	-135	227	-1,134	-2,535	-3,541	-1,487
Artemis	202	-131	-570	-322	128	439	-456	170
Shalby	344	532	-262	511	-333	-1,014	-567	-670

Source: Company Data, Equirus

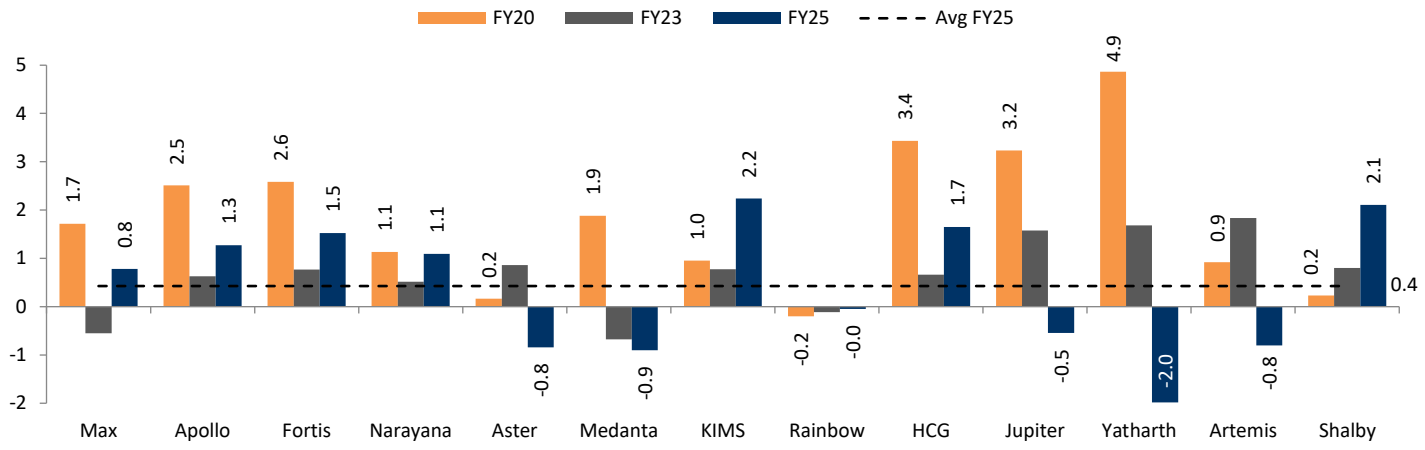
Balance Sheet Discipline – The Capex Cycle Was Self-Funded/IPO, Not Debt-Funded

Growth without leverage - balance sheets strengthened, not stretched

Perhaps the most overlooked dimension of the sector's FY20–FY25 capex cycle is what it did *not* require: meaningful balance sheet leverage. The industry entered the cycle with Net Debt/EBITDA ranging from negative (Rainbow at -0.2x, Shalby at 0.2x) to elevated (Yatharth at 4.9x, HCG at 3.4x, Jupiter at 3.2x, Apollo at 2.5x, Fortis at 2.6x) in FY20 — a wide dispersion reflecting varying stages of prior expansion and governance events. By FY25, that dispersion had compressed dramatically, with the majority of the universe operating at or near 1x Net Debt/EBITDA or below. Yatharth swung from 4.9x to net cash (-2.0x), Medanta from 1.9x to net cash (-0.9x), Aster from 0.2x to net cash (-0.8x), Jupiter from 3.2x to net cash (-0.5x), and Artemis from 0.9x to net cash (-0.8x). Even operators that increased leverage modestly — KIMS from 1.0x to 2.2x, Shalby from 0.2x to 2.1x – did so to fund aggressive expansion programmes, and at levels that remain entirely manageable relative to their EBITDA generation.

The implication is twofold. First, the sector's Rs 225+ billion cumulative capex over FY20–FY25 was funded overwhelmingly by internally generated cash flows and IPOs – validating strong ability to generate cash flows. Second, and equally important from a forward perspective, the balance sheets entering the next expansion cycle are structurally cleaner than they were entering the last one. At a sector-average Net Debt/EBITDA well below 1x, operators have significant incremental debt capacity available should they need to accelerate acquisitions or bridge greenfield programmes – without any meaningful risk of financial stress. This optionality is a strategic asset: in a market where distressed hospital assets continue to surface at attractive valuations, a clean balance sheet is a competitive weapon as much as it is a financial metric.

Exhibit 36: Net Debt/EBITDA



Source: Company Data, Equirus

ROCE – The Ultimate Validation That Capital Is Being Deployed Productively

ROCE is the ultimate test of whether the sector's capex cycle created or destroyed value – and the data delivers a clear verdict: it created value, substantially. Despite deploying Rs 225+ bn of cumulative hospital capex over FY20–FY25, ROCE expanded for the majority of chains.

ROCE expansion confirms the capex cycle was value-accretive, not dilutive

Apollo improved from 13.2% (FY20) to 25.8% (FY25) – the most consistent ROCE improvement in the large-cap universe, by acquiring stake in already proven its high EBITDA JV/associate/Subs and **restructuring of its pharmacy biz**. Narayana, remarkably, expanded ROCE from 13.7% to 26.1% without adding material beds – a pure profitability-per-rupee-of-capital story driven entirely by ARPOB expansion (transition from general wards to premium private wards), margin improvement and Cayman turnaround.

Fortis went from a barely-positive 3.1% (FY20, still recovering from the governance crisis and RHT costs) to 14.4% (FY25) and 15.7% (9MFY26) – one of the most dramatic ROCE recoveries in the sector, with further improvement expected as legacy issues are fully behind it. KIMS saw ROCE decline from 20.3% (FY20) to 16.2% (FY25) – but this is entirely attributable to the entry in new cluster with heavy capex – Maharashtra greenfield cluster (Nagpur, Nashik, Thane), Kerala and Bangalore – that is not yet generating mature returns; the blended decline reflects peak capex dilution, not return deterioration. Max saw ROCE peak at 33.1% in FY23 before moderating to 25.9% in FY25 – again a function of the Lucknow, Nagpur, and Noida acquisitions landing simultaneously and adding capital employed ahead of revenue ramp.

The sector's aggregate ROCE story – from 11–13% in FY20 to 20–26% for the best operators in FY25 – is unambiguous evidence that this was a value-creating, not value-consuming, capex cycle.

Exhibit 37: Mature Players Sustain High ROCE While Expansion Dilutes Returns

ROCE	FY20	FY21	FY22	FY23	FY24	FY25	9MFY26
Max	11.1%	14.3%	28.8%	33.1%	31.8%	25.9%	21.9%
Apollo	13.2%	5.5%	22.6%	24.2%	24.5%	25.8%	27.2%
Fortis	3.1%	-0.1%	7.6%	11.1%	13.4%	14.4%	15.7%
Narayana	13.7%	1.4%	24.8%	33.5%	33.2%	26.1%	26.1%
Aster	10.3%	9.2%	16.9%	16.6%	22.9%	21.2%	21.0%
Medanta	5.1%	4.3%	14.3%	16.8%	19.3%	19.5%	18.4%
KIMS	20.3%	30.0%	33.0%	23.5%	17.6%	16.2%	11.5%
Rainbow	14.7%	9.6%	20.6%	21.7%	17.3%	16.5%	15.9%
HCG	1.7%	-2.0%	4.6%	7.6%	7.9%	7.1%	7.9%
Jupiter		6.1%	16.1%	20.1%	19.6%	16.7%	15.3%
Yatharth		16.9%	25.3%	25.6%	21.4%	12.5%	11.2%
Artemis	9.8%	3.1%	8.8%	10.0%	12.6%	11.2%	12.0%
Shalby	8.0%	8.0%	14.0%	16.0%	18.0%	14.0%	11.8%

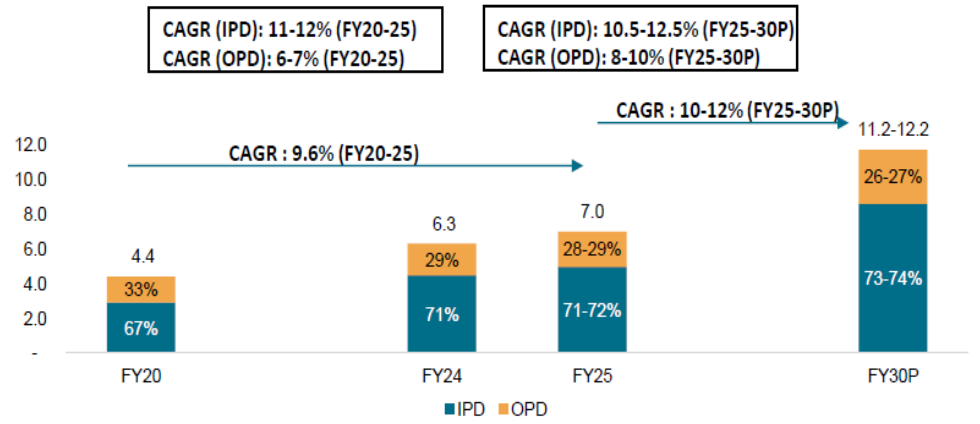
Source: Company Data, Equirus

Built for More — Structural Tailwinds, Disciplined Expansion, and the Case for a Decade of Compounding

India's hospital sector has delivered five years of exceptional compounding — but the more important observation is structural: the conditions that produced the last five years are not just sustaining, they are intensifying. Healthcare in India is transitioning from a cyclical growth story to a secular demand compounding machine. The market, valued at approximately Rs 7.0 trillion in FY25 (*Manipal Hospitals DRHP*), is projected to reach Rs 11.2–12.2 trillion by FY30 at a 10–12%.

> *Key Insight: India's healthcare delivery market is estimated to grow at 10–12% CAGR from FY25 to FY30, reaching Rs 11.2–12.2 trillion — with IPD growing at 10.5–12.5% and OPD at 8–10% over the same period. Private hospitals, commanding a 68% share in FY25, are projected to further expand to 69% by FY30 at a 11.5–13.5% CAGR. (Source: Manipal Hospitals DRHP)*

Exhibit 38: Hospital Growth in Low-Teens, Driven by IPD Momentum



Source: Manipal Hospitals DRHP, Equirus

Four Structural Growth Drivers — And Why They Are Accelerating

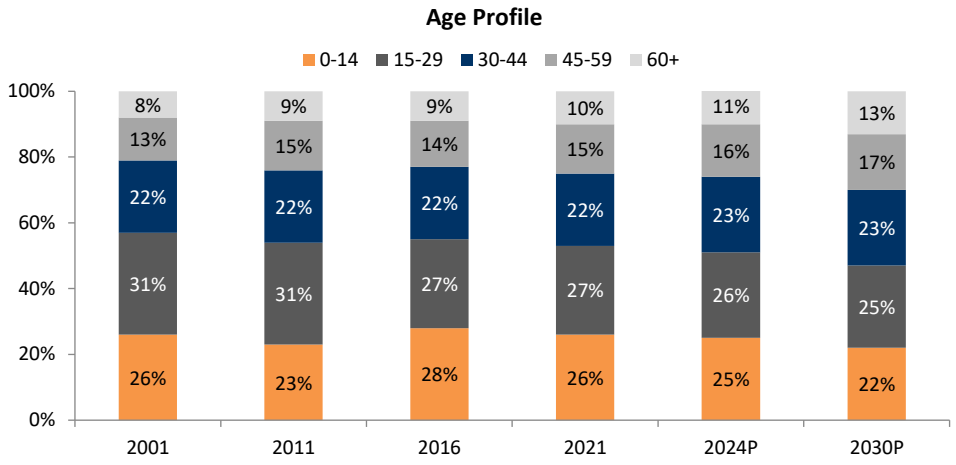
The Indian healthcare sector's demand trajectory is underpinned by six structural drivers, all of which are simultaneously strengthening. Unlike consumer discretionary sectors where one or two catalysts are typically responsible for growth, hospitals benefit from demand that is structurally non-discretionary and driven by irreversible demographic and economic forces.

1. Ageing Demographics and Rising Chronic Disease Burden

India's demographic profile is shifting decisively. The population aged 60+ is projected to rise from 10.5% of the total population in 2023 to 12.6% by 2030, while the 40–59 cohort grows from 22.1% to 24.4% over the same period. Elderly patients disproportionately consume hospitalisation — they are 3–4x more likely to be hospitalised than younger cohorts, and their hospitalisations skew toward complex, multi-day, high-ARPOB procedures. Non-communicable diseases (NCDs) — cardiovascular, oncology, diabetes, neurological — which are directly correlated with age, are already rising sharply. Approximately 66% of elderly Indians reported at least one chronic ailment as per 2011 Census data; by 2030, the sheer size of the ageing cohort will translate this into a step-change in hospitalisation demand.

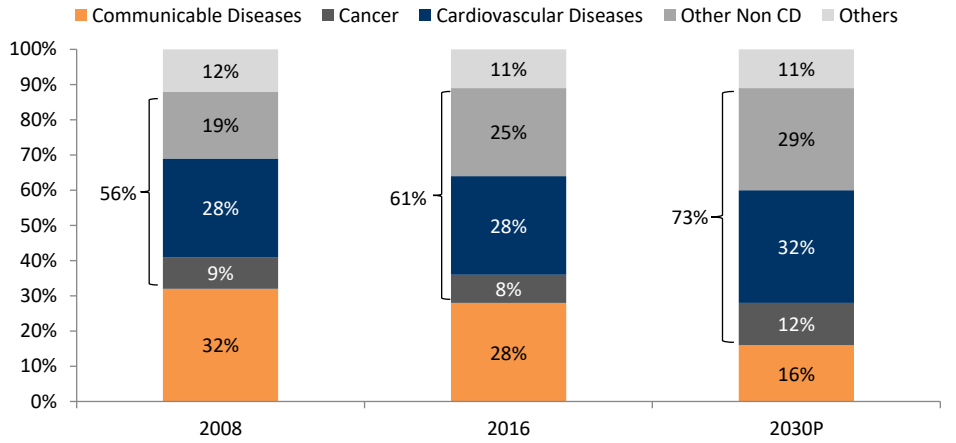
> *~13% of India's population will be over 60 by 2030 — an elderly cohort of approximately 200 million that will require sustained, complex hospitalisation. This alone represents a structural demand tailwind that no amount of capacity addition in the next five years can fully address.*

Exhibit 39: Steady rise in 45+ age profile



Source: Global Health Ltd AR, Equirus

Exhibit 40: Lifestyle disease rising rapidly



Source: Apollo Hospitals AR, Equirus

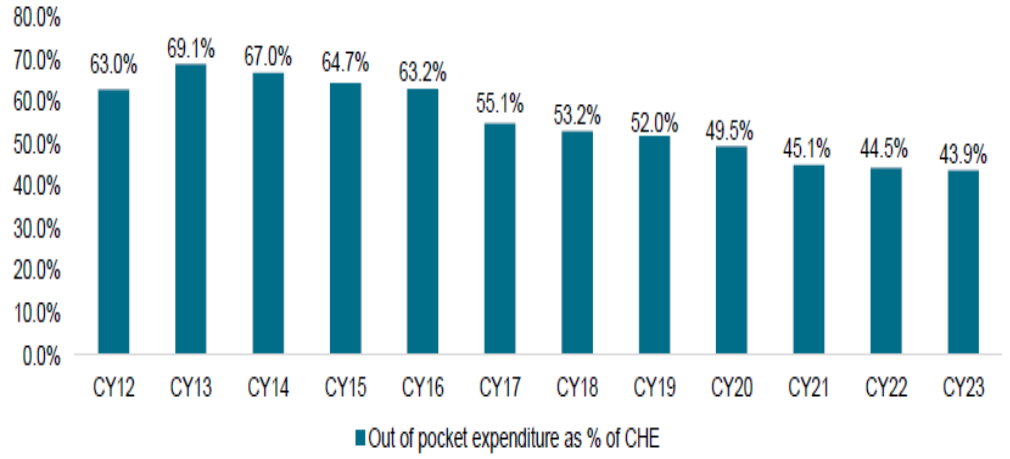
2. Health Insurance Penetration — From 35% to 45–50%

Insurance is the growth engine—more coverage, more hospitalisation

Health insurance penetration in India has risen from 35% in FY19 to 41% in FY24, driven by expanding PMJAY (Pradhan Mantri Jan Arogya Yojana) coverage, growing corporate employer-sponsored schemes, and increasing individual retail policy uptake. CRISIL projects penetration to reach 45–50% by FY26 — covering approximately 600–650 million people. Each percentage point increase in insured population directly catalyses hospitalisation demand: insured patients seek care earlier, undergo elective procedures that cash-pay patients defer, and are 40–60% more likely to choose organised private hospitals over government facilities or standalone clinics.

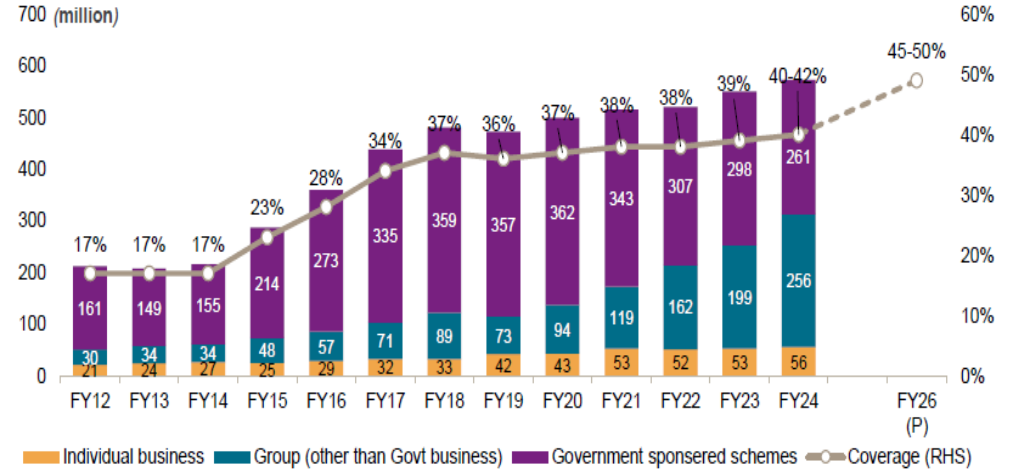
State-level insurance data reveals the concentrated opportunity. Delhi (74%), Haryana (73%), and Karnataka (61%) already have high private insurance penetration — creating the premium demand foundation that drives ARPOB leadership in these markets. Lower-penetration states like West Bengal (34%), Gujarat (35%), and Kerala (34%) present the incremental volume opportunity as coverage expands.

Exhibit 41: Steady decline in out-of-pocket expenditure in health as % of CHE



Source: Manipal Hospitals DRHP, Equirus

Exhibit 42: Rise in Insurance Penetration



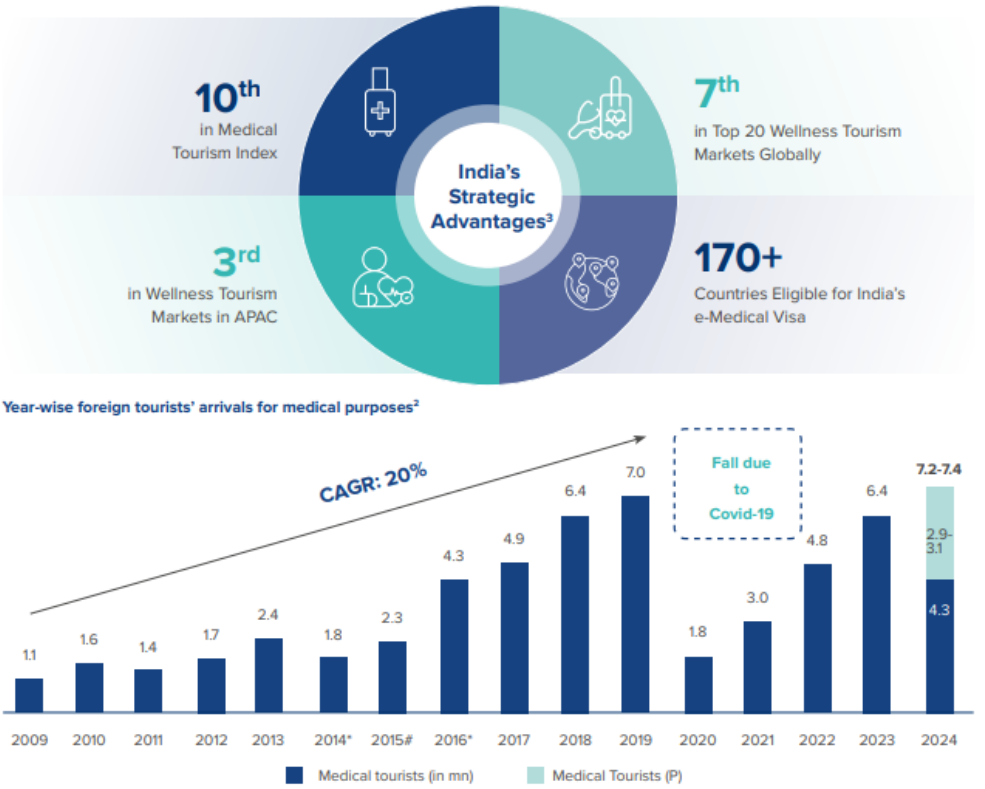
Source: Manipal Hospitals DRHP, Equirus

3. Medical Tourism — India's Comparative Cost Advantage

India's structural cost advantage is driving a steady rise in high-value medical tourism flows.

India's cost advantage in complex procedures is extraordinary and widening relative to Western alternatives. A heart bypass in India costs 1x what it costs in India (baseline), while the same procedure in the US costs 27.7x and in Singapore 3.6x. Medical tourism arrivals recovered to 0.64–0.66 million patients by CY2023–24 from a Covid-induced low of 0.18 million in CY2020, growing at a 20% CAGR over the longer term. More than 95% of medical tourists originate from South Asia, Africa, and the Middle East — regions with structural proximity to India and limited access to affordable complex care at home. Delhi (Max, Apollo, Medanta, Fortis) and Mumbai (Fortis, KIMS) are the primary destinations.

Exhibit 43: India - A key destination for MVT



Source: Max Healthcare AR, Equirus

Exhibit 44: Significant cost advantage – attract India's MVT

Significant cost advantage v/s other countries

Procedure cost (US$)	India	Thailand	Singapore	Korea	US	Avg. global cost[1]	India discount
Hip replacement	7,000	7,879	12,000	14,120	50,000	21,000	-67%
Knee replacement	6,200	12,297	13,000	19,800	50,000	23,774	-74%
Heart bypass	5,200	15,121	18,500	28,900	144,000	51,630	-90%
Angioplasty	3,300	3,788	13,000	15,200	57,000	22,247	-85%
Heart valve replacement	5,500	21,212	12,500	43,500	170,000	61,803	-91%
Dental implant	1,000	3,636	1,500	4,200	2,800	3,034	-67%

Source: Ministry of Tourism, CRISIL research
1. Avg. global cost excl. India

Source: Apollo Hospitals AR, Equirus

4. Favourable Government Policy — PMJAY, CGHS, ABDM

Government policy has turned structurally supportive at a pace not seen in prior decades. The Union Budget FY26 hiked health and wellbeing expenditure by 44.3% versus FY25RE to Rs 998.6 billion — the largest single-year health budget increase. PMJAY coverage expansion, CGHS tariff resets (discussed in the next section), and the ABDM digital infrastructure all collectively create a more favourable operating environment for organised private hospital chains. Budget allocation for centrally-sponsored schemes under health grew to Rs 530.4 bn in FY26BE, up from Rs 475.6 bn in FY25.

The CGHS Tariff Overhaul — From Floor Price to Value-Based Framework

CGHS revision disproportionately benefits hospital chains with high institutional exposure

The CGHS tariff revision cycle of FY24–FY26 has been one of the most consequential policy developments for the Indian hospital sector in a decade. After nearly a decade of stagnation, the FY24 reset delivered a blunt but impactful first move: consultation rates and room rents were revised upward by 80–100%, immediately recalibrating the baseline P&L contribution from the CGHS payroll across affiliated hospitals.

The FY26 package revision touched 2,000+ surgical and diagnostic procedure rates, delivering average increases of 15–20%. More importantly, the framework transitioned from a flat-rate model to a tiered, accreditation-linked structure:

- **Accreditation Tiers:** Super Speciality Hospitals receive a 15% premium over the base CGHS rate
- NABH-accredited hospitals receive an additional uplift, incentivising quality investment
- Procedure rates for complex surgeries (cardiac, neuro, onco) were revised most aggressively, aligning government billing more closely with market rates
- The transition reduces the historical discount between CGHS and cash/TPA rates, making government business a less dilutive payor mix component

Exhibit 45: CGHS Exposure by Hospital Chain — Differential Revenue Impact

Hospital	Key CGHS Catchment	CGHS Revenue Exposure	% of Institutional Biz	Equirus View
Yatharth	Delhi NCR, Greater Noida, Faridabad	High	**37%**	Rising; as CGHS rates improve, payor mix upgrade expected
Max	Delhi NCR (primary), Lucknow	High	**22%**	Highest beneficiary of FY26 CGHS reset
Narayana	PAN-India, ECHS/CGHS mix	High	**17%**	High institutional mix; CGHS reset helps reduce dilution
Fortis	NCR (Gurugram, Noida, Delhi)	Moderate	**13%**	Material CGHS exposure across NCR network
Medanta	Gurugram, Lucknow, Patna, Noida	Moderate	**11%**	Strong beneficiary; Gurugram sees high CGHS inflow
Apollo	Delhi, Hyderabad, Chennai	Moderate	**9%**	Premium-dominated; CGHS a smaller share of revenue
Aster	South India	Low	**8%**	Kerala/Karnataka dominant; minimal CGHS exposure
KIMS	Hyderabad, Maharashtra, Kerala	Low	**4%**	South-focussed; limited CGHS geography overlap
Jupiter	Pune (Wakad)	Low	**1%**	Pune has significant CGHS beneficiary population
Rainbow	Hyderabad, Bengaluru	Low	**-**	Paediatric focus; CGHS covers children, limited direct impact

Source: Company Data, Equirus

The CGHS reset also has a second-order effect that is less visible but equally important: it has recalibrated the sector's effective price floor for TPA negotiations. When hospitals demonstrate that government business is now yielding rates closer to market, it materially strengthens their negotiating position with TPAs who have historically been able to hold hospitals to sub-market rate structures on institutional billing. As CGHS rates normalise, we expect TPA rate renegotiations to follow — a dynamic that compounds the direct CGHS revenue benefit for North India-heavy chains over FY26–FY27.

A Structurally Undersupplied Market

India's national bed density stands at just 16 beds per 10,000 population as of FY25 — less than half the global average of 33, and significantly below even developing-world peers such as Brazil (25), Thailand (24), and Vietnam (25). This is not a temporary gap; it is a structural shortfall that has been widening for decades as population growth, urbanisation, and rising disease burden have outpaced capacity additions.

The regional disparity makes the aggregate number even more alarming. South India leads with 29–30 beds per 10,000 — close to global norms — while North India sits at just 14–15, West India at 13–14, and the eastern states at a critically low 8–9 beds per 10,000.

Exhibit 46: Bed density divergence from World average

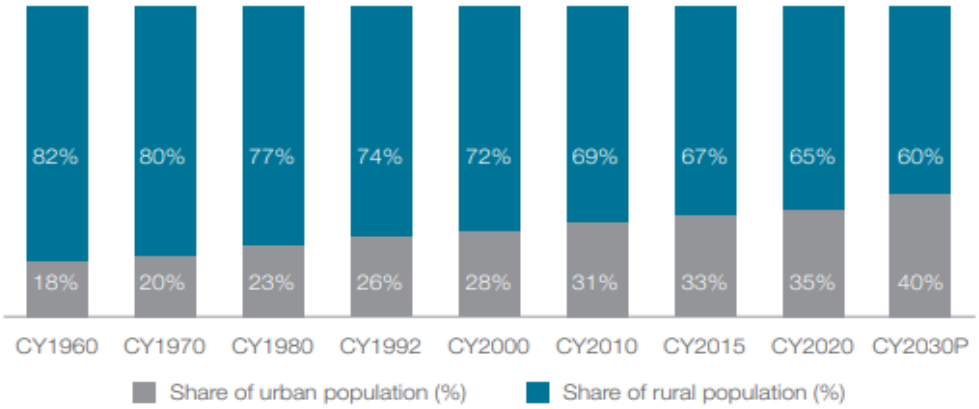
Region	States	Bed Density (per 10,000)
South India	AP, Karnataka, TN, Kerala, Telangana	29–30
North India	Delhi, UP, Punjab, Haryana, Rajasthan, J&K etc.	14–15
West India	Maharashtra, Gujarat, MP, Goa	13–14
East India	Bihar, WB, Odisha, Jharkhand, Chhattisgarh etc.	8–9
India (National Avg.)	–	16
Global Average	–	33

Source: *Manipal Hospitals DRHP, Equirus*

Rapid Urbanisation Driving Healthcare Demand in Metro & Tier I Cities

Rapid urbanisation directly expands the addressable patient pool for listed hospital chains — urban migrants enter geographies where organised private hospitals are the primary access point for quality tertiary care. Urban households carry significantly higher health insurance penetration through employer group policies, driving insured volumes and ARPOB upside. The lifestyle shift accompanying urbanisation — sedentary work, poor diets, pollution — accelerates NCD incidence across cardiovascular, oncology, and neurological specialties, which are precisely the high-yield CONGO procedures that anchor listed chains' case mix. Rising urban incomes simultaneously improve affordability, reducing elective procedure deferral. And urban density makes cluster-based hospital networks economically viable — the compounding model that Max in NCR, KIMS in Hyderabad-Maharashtra, and Medanta across North India have scaled most aggressively.

Exhibit 47: Urbanisation Tailwind Expanding Healthcare Opportunity in Metro & Major Cities



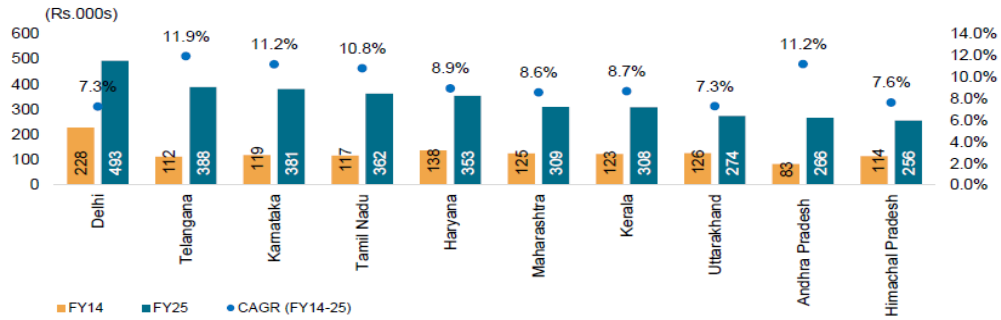
Source: *Apollo Hospitals AR*, *Equirus*

The Key Demand Geographies: Delhi NCR, Mumbai, and Beyond

Delhi NCR, Mumbai and Bangalore – areas where players are expanding beds

Metro cities collectively account for 20–25% of the Rs 7 trillion healthcare delivery market in FY25, disproportionately large relative to their population share. This is because metros host dense insured populations, high-yield tertiary procedures, strong medical tourism inflows, and advanced multi-speciality infrastructure. But the more interesting story is what happens at the micro-market level within these metros — several sub-regions remain acutely undersupplied even within large cities.

Exhibit 48: Delhi, Mumbai and South command highest NSDP (current prices)



Cities	NDDP (RS billion)	Per capita income (Rs)
Bangalore	10,199	7,36,452
Pune	3,992	3,74,257
Delhi	9,972	4,61,910
Mumbai MMR	14,987	4,15,391
Hyderabad	2,380	5,54,105
Chennai	2,578	5,19,941
India	**3,01,226**	**1,88,892**

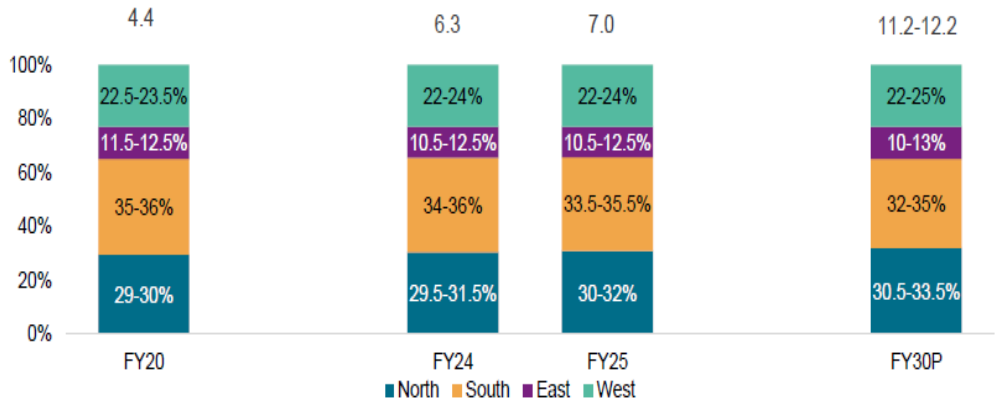
Source: Manipal Hospitals DRHP, Equirus

South India — The Structural Stronghold

South India commands a 33.5–35.5% share of the national healthcare delivery market in FY25, the highest of any region, and is projected to sustain this dominance at 32–35% by FY30. Karnataka (Bengaluru) has the highest bed density in India at 40–42 beds per 10,000, driven by a concentration of private hospital chains and advanced medical infrastructure. Hyderabad (Telangana) and Chennai are the other key markets, with KIMS, Aster, Narayana, and Apollo all having strong anchored positions here.

Bengaluru in FY25 had an estimated 52,000 beds across 1,250–1,300 hospitals, with 75–80% in the private sector — making it the most privatised major hospital market in the country. Hyderabad and Chennai are similarly private-dominated, with CONGO procedure mix particularly high. The south's structural advantages — IT economy, corporate health insurance penetration, a large NRI community funding high-end tertiary care, and an established culture of specialist care-seeking — make it the most mature and highest-yield region in India.

Exhibit 49: Indian Healthcare region wise - North and South continues to dominate



Source: Manipal Hospitals DRHP, Equirus

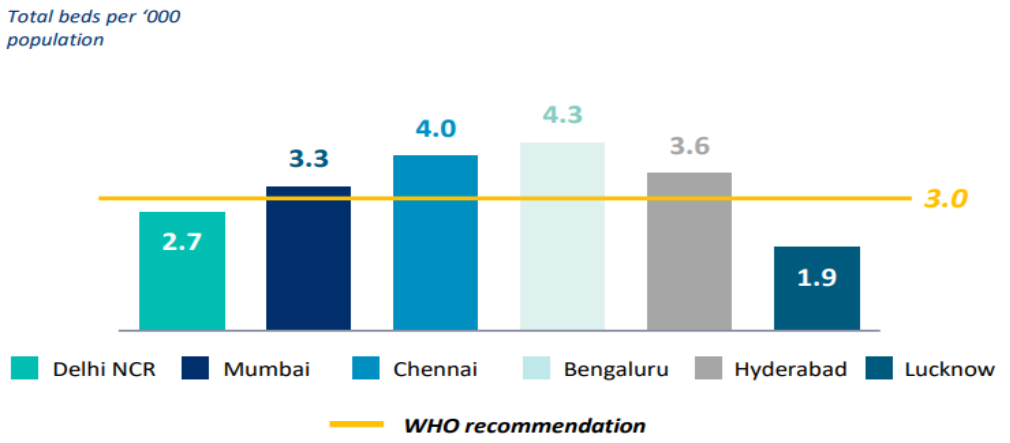
Delhi NCR — The Most Dynamic Hospital Market in India

Delhi NCR has been and will remain the most consequential market for the Indian hospital sector over the next five years. The region combines the highest concentration of government-insured (CGHS) beneficiaries in India, a large and rapidly growing upper-middle income population, and critically — persistent under-supply of quality private hospital beds relative to population density. Delhi's private bed density, at approximately 10–11 per 10,000, remains one of the lowest among major metros despite being the national capital. The structural imbalance between quality healthcare demand and available organised supply is the most acute in any large metro in India.

This is precisely why Delhi NCR led all regions with approximately 4,500 bed additions over FY20–9MFY26 — and yet the supply gap remains materially unaddressed. Max Healthcare, the most concentrated NCR player, grew capex from Rs 0.9 bn in FY20 to Rs 15.9 bn in FY25 and has a CWIP of Rs 12.3 bn — signalling further capacity additions in Noida, East Delhi, and peripheral NCR catchments. Yatharth — the most instructive NCR-concentrated story — grew from 1,100 beds in FY20 to 2,305 beds in 9MFY26, with occupancy improving despite the near-tripling of capacity, validating the depth of unmet demand in the Delhi NCR and adjacent UP belt.

> ***Delhi NCR insight:*** *With private bed density at just 10–11 per 10,000 — versus a national average of 16 and global average of 33 — the NCR remains structurally the most undersupplied premium healthcare market in India. Both Max and Yatharth are adding beds aggressively here, and demand is absorbing every addition without material occupancy dilution.*

Exhibit 50: High demand-supply gap in Delhi NCR & Mumbai



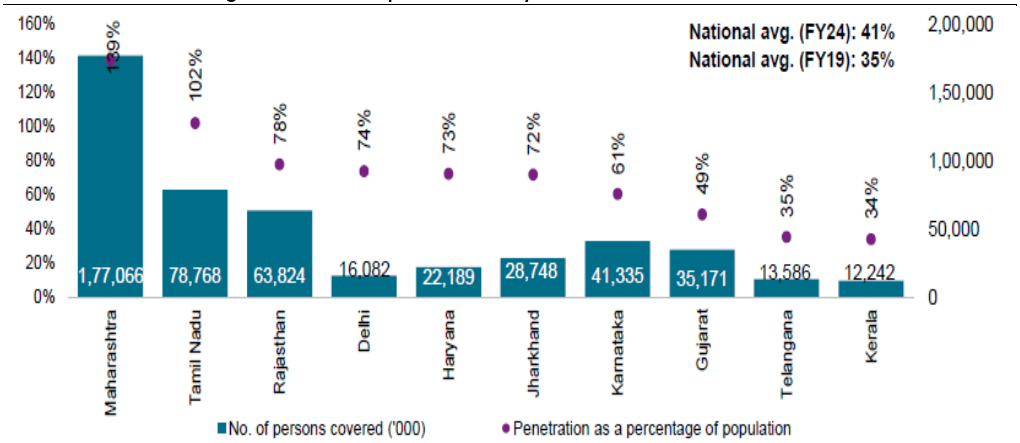
Source: Max Healthcare AR, Equirus

Mumbai & MMR — Insurance Penetration + Premium Demand

Mumbai and its broader metropolitan region represent the highest-income, highest-insurance-penetration healthcare market in India. Maharashtra has the largest number of people covered under health insurance in the country at 1.77 lakh thousand (FY24), and the combined private insurance penetration of Maharashtra and Goa stands at 57% — the highest in India. This structural payor mix advantage makes the Mumbai market particularly attractive for private hospitals: insured patients drive ARPOB upside, longer-duration hospitalisation, and case mix enrichment.

The Mumbai market is also the primary beneficiary of medical tourism from the Middle East, East Africa, and South Asia — sectors that skew toward high-complexity oncology, cardiac, and organ transplant procedures. Fortis, which has significantly increased brownfield capex in Mumbai and Mulund, and KIMS, which entered the market through Thane and Nashik, are both positioning to capture this structural premium demand. Apollo's CWIP has expanded to Rs 10.3 bn, which includes meaningful Mumbai-region commitments.

Exhibit 51: Mumbai highest Insurance penetrated city



Source: Manipal Hospital DRHP, Equirus

Capacity Expansion: Where the Beds Are Going

The listed hospital universe is set to add 23,039 beds from 9MFY26 to FY29, and the distribution of this pipeline carries one clear verdict: **metro is decisively winning the incremental capacity race**. Of the 23,039 beds to be added, approximately **17,159 (74%) are headed to metro markets** versus just 5,880 (26%) to non-metro — a sharp skew toward urban, premium geographies that reflects where demand visibility, insurance penetration, and ARPOB yields are strongest.

Metro wins the bed race - premium markets capture the bulk of expansion

At 9MFY26, the universe already had 28,338 metro beds versus 23,154 non-metro. By FY29, metro beds expand to 45,497 — a 61% jump — while non-metro grows modestly from 23,154 to 29,034, a 25% increase. The metro share of total industry beds rises from 55% to 61% — a structural shift in the character of the portfolio toward higher-yield urban assets.

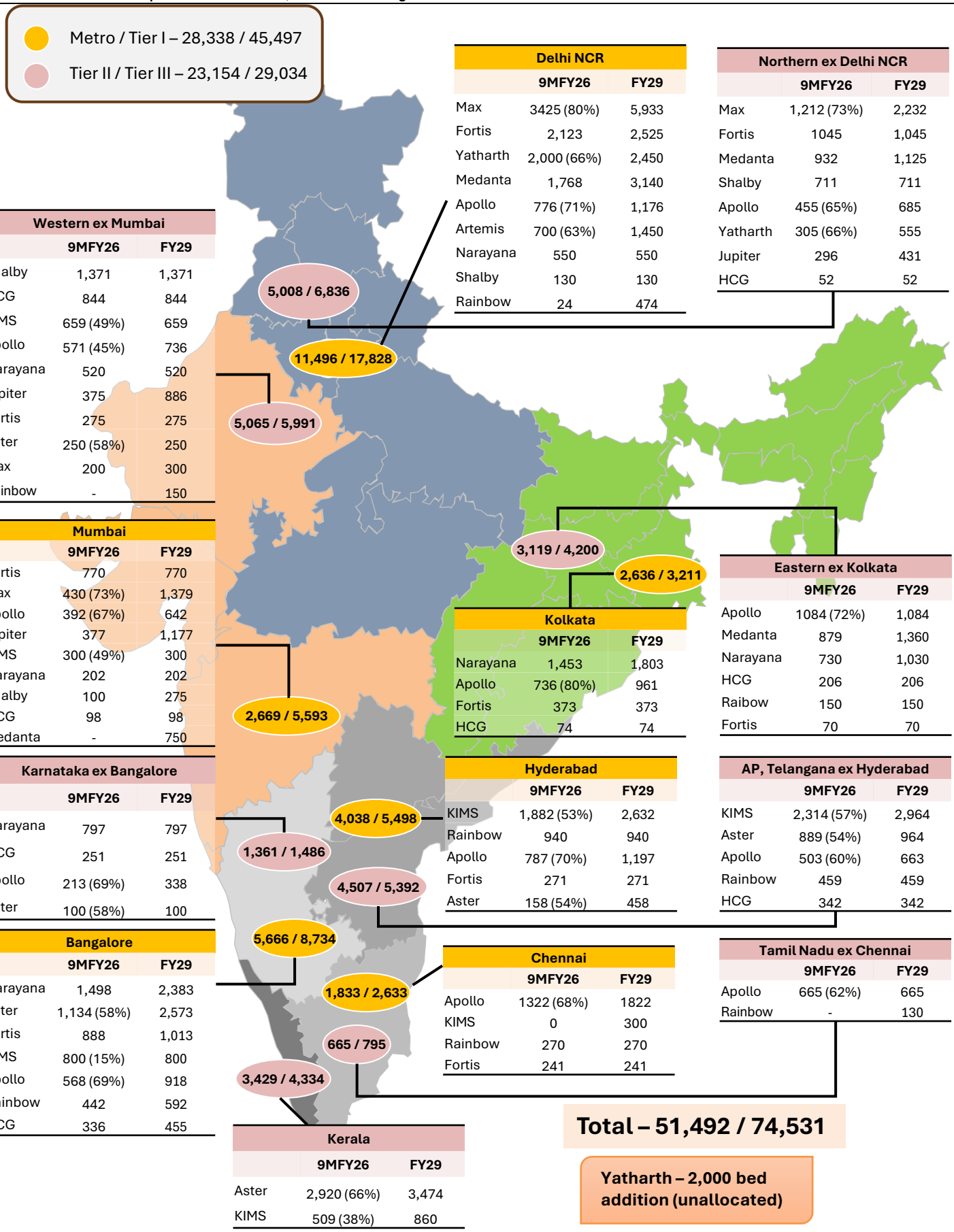
By operator, the metro-led push is coming from:

- **Max** adds 3,457 metro beds (4,581→7,312 — highest absolute metro addition in the universe), cementing its NCR dominance while extending into Mumbai and Lucknow urban corridors
- **Apollo** adds 2,135 metro beds (4,581→6,716), deepening its pan-metro anchor across Delhi, Hyderabad, Chennai, and Bengaluru
- **Yatharth** is the most striking story — 2,450 metro beds added (2,000→4,450) despite being historically an NCR-fringe player, validating its deliberate pivot into the formal Delhi NCR market from its UP-border base
- **Fortis** adds 527 metro beds with meaningful brownfield additions across its existing NCR and Mumbai network
- **KIMS** adds 1,050 metro and 1,001 non-metro beds — the most balanced pipeline in the universe, reflecting its simultaneous Maharashtra metro cluster maturation and South India non-metro deepening
- **Narayana** adds 1,235 metro beds alongside 300 non-metro, with Bangalore and Kolkata driving the metro component
- **Artemis** nearly doubles metro beds (700→1,450), making it one of the most aggressive proportional metro growers from a smaller base

On the non-metro side, **Aster** remains the standout with 629 non-metro additions (4,159→4,788) as it deepens its Kerala cluster, and **Medanta** adds 674 non-metro beds across its Lucknow, Patna, and Ranchi cluster — the most deliberate non-metro strategy among the large-caps.

The forward pipeline is a **metro consolidation play**, with organised chains using their balance sheet strength, brand equity, and specialist depth to capture the premium urban market where returns mature fastest and ARPOB compounds most reliably.

Exhibit 52: Metro-Led Expansion: Delhi NCR, Mumbai & Bangalore Dominate



Delhi NCR	9MFY26	FY29
Max	3425 (80%)	5,933
Fortis	2,123	2,525
Yatharth	2,000 (66%)	2,450
Medanta	1,768	3,140
Apollo	776 (71%)	1,176
Artemis	700 (63%)	1,450
Narayana	550	550
Shalby	130	130
Rainbow	24	474

Northern ex Delhi NCR	9MFY26	FY29
Max	1,212 (73%)	2,232
Fortis	1045	1,045
Medanta	932	1,125
Shalby	711	711
Apollo	455 (65%)	685
Yatharth	305 (66%)	555
Jupiter	296	431
HCG	52	52

Western ex Mumbai	9MFY26	FY29
Shalby	1,371	1,371
HCG	844	844
KIMS	659 (49%)	659
Apollo	571 (45%)	736
Narayana	520	520
Jupiter	375	886
Fortis	275	275
Aster	250 (58%)	250
Max	200	300
Rainbow	-	150

Mumbai	9MFY26	FY29
Fortis	770	770
Max	430 (73%)	1,379
Apollo	392 (67%)	642
Jupiter	377	1,177
KIMS	300 (49%)	300
Narayana	202	202
Shalby	100	275
HCG	98	98
Medanta	-	750

Kolkata	9MFY26	FY29
Narayana	1,453	1,803
Apollo	736 (80%)	961
Fortis	373	373
HCG	74	74

Eastern ex Kolkata	9MFY26	FY29
Apollo	1084 (72%)	1,084
Medanta	879	1,360
Narayana	730	1,030
HCG	206	206
Raibow	150	150
Fortis	70	70

Karnataka ex Bangalore	9MFY26	FY29
Narayana	797	797
HCG	251	251
Apollo	213 (69%)	338
Aster	100 (58%)	100

Hyderabad	9MFY26	FY29
KIMS	1,882 (53%)	2,632
Rainbow	940	940
Apollo	787 (70%)	1,197
Fortis	271	271
Aster	158 (54%)	458

AP, Telangana ex Hyderabad	9MFY26	FY29
KIMS	2,314 (57%)	2,964
Aster	889 (54%)	964
Apollo	503 (60%)	663
Rainbow	459	459
HCG	342	342

Bangalore	9MFY26	FY29
Narayana	1,498	2,383
Aster	1,134 (58%)	2,573
Fortis	888	1,013
KIMS	800 (15%)	800
Apollo	568 (69%)	918
Rainbow	442	592
HCG	336	455

Chennai	9MFY26	FY29
Apollo	1322 (68%)	1822
KIMS	0	300
Rainbow	270	270
Fortis	241	241

Tamil Nadu ex Chennai	9MFY26	FY29
Apollo	665 (62%)	665
Rainbow	-	130

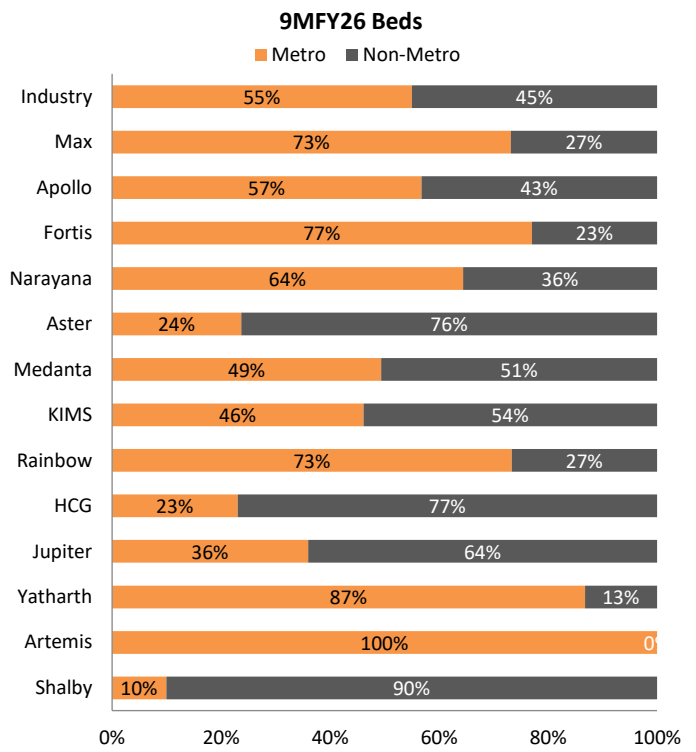
Kerala	9MFY26	FY29
Aster	2,920 (66%)	3,474
KIMS	509 (38%)	860

Total – 51,492 / 74,531

Yatharth – 2,000 bed addition (unallocated)

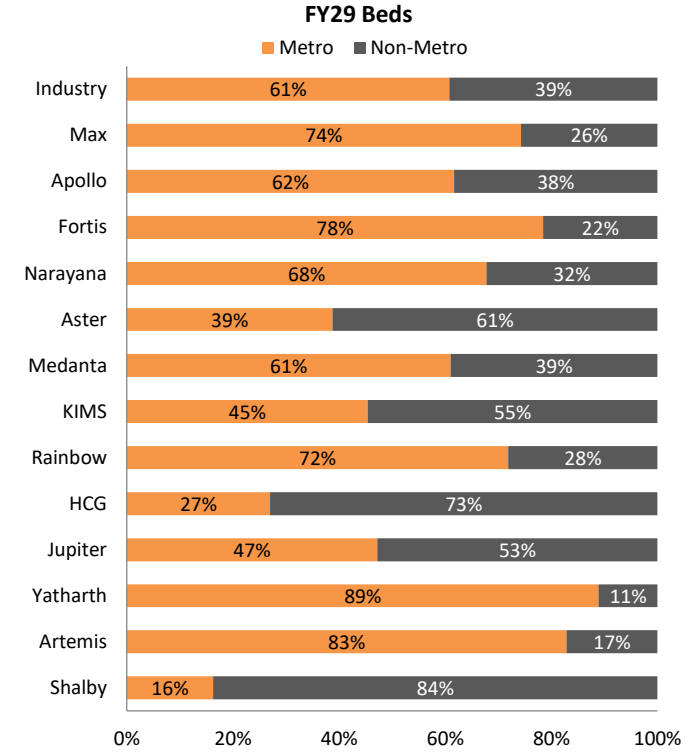
Source: Company Data, Equirus
Bracket no. stands for occupancy %
Operating Beds – Apollo, Fortis, Medanta, HCG & Jupiter, while others are Bed Capacity

Exhibit 53: Metro-Led Expansion to Continue…



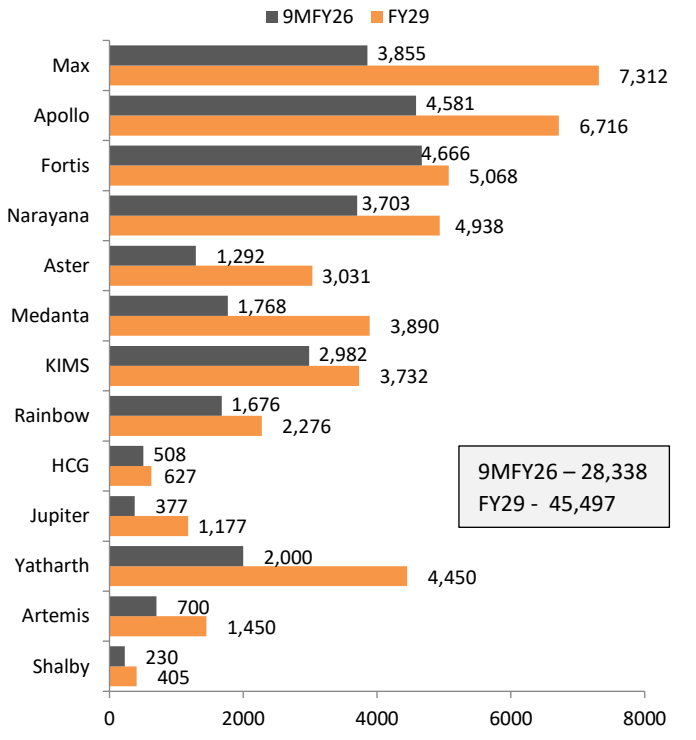
Source: Company Data, Equirus

Exhibit 54: …with Gradual Non-Metro Scale-Up



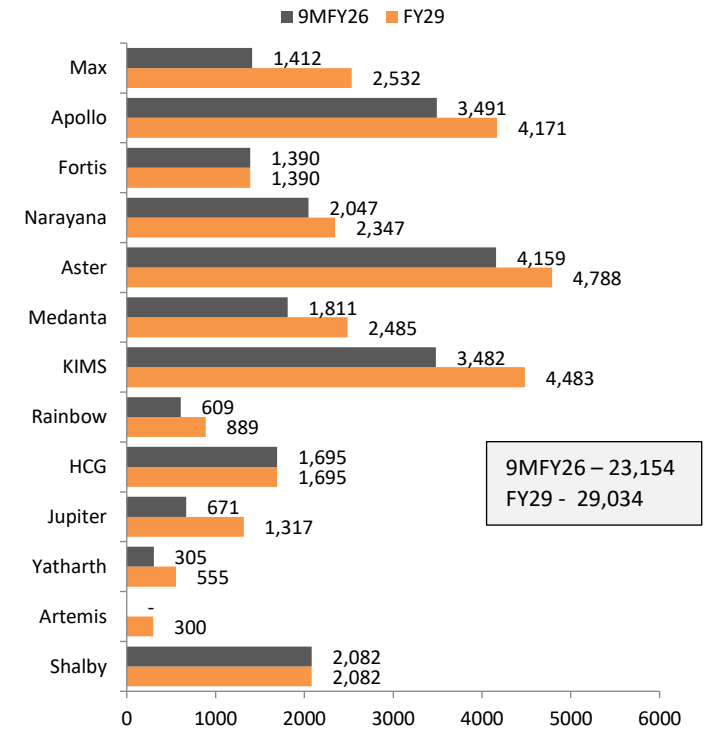
Source: Company Data, Equirus

Exhibit 55: Metro-Led Expansion Dominates Industry Growth …



Source: Company Data, Equirus

Exhibit 56: …with Consistent Build-Out in Non-Metro Markets



Source: Company Data, Equirus

Revenue Accretion: How the Capacity Cycle Translates to the Top Line

The forward capacity pipeline — a 13.1% CAGR of bed additions across the listed universe from 9MFY26 to FY29 — is not just a volume story; it is a revenue accretion story, and the pace at which each operator captures that revenue will be determined entirely by the modality of expansion they are pursuing.

Next growth phase is modality-driven - faster revenue ramp for acquisition/brownfield, gradual for greenfield

Over FY20–FY25, the sector delivered a 15.5% revenue CAGR despite adding beds at only a 5.6% CAGR — proof that yield expansion, not just volume, drives the compounding. What changes ahead is that the bed addition CAGR accelerates sharply to 13.1%, layered on top of structural industry tailwinds — rising insurance penetration, CGHS rate normalisation, ageing demographics, and deepening CONGO case mix — that continue to compound ARPOB independently of capacity. We believe ARPOB CAGR of 6-7% will continue, which will facilitate **industry growth of 18-20% CAGR over FY26-29E.**

The critical differentiator across operators, however, is ramp-up speed — and that is a direct function of how beds are being added.

Acquisitions and O&M — The Revenue-Ready Playbook. The post-COVID environment created a structural window of non-competitive slump sale opportunities as smaller standalone hospitals — unable to sustain through the pandemic, burdened by legacy debt, and lacking capital for infrastructure upgrades — became available at distressed valuations. Organised chains exploited this aggressively: acquisitions represented 38% of FY20–9MFY26 bed additions at the industry level, and for Yatharth — the most instructive case — it was virtually the entire playbook, with a 13.7% acquisition CAGR in the prior cycle and 24.6% guided forward. The revenue case for acquisitions is immediate: an acquired asset enters the network with an existing patient base, operational staff, local referral relationships, and regulatory approvals intact. Revenue contribution begins from day one; the acquiring chain's protocols, ARPOB management, and procurement scale then compound that base over 12–24 months. O&M and asset-light models work similarly — the physical asset and its patient flow are inherited, the listed operator layers on management efficiency and brand. Max's O&M-driven forward CAGR of 6.8% reflects this same thesis.

Brownfield — The Fastest Revenue per Rupee of Capex. Brownfield expansion within or adjacent to mature, high-occupancy hospitals is the most immediate revenue accretion mechanism available to an operator. Incremental beds added to a facility already running at 60–65%+ occupancy typically reach normalised utilisation within one to two quarters. This is why Fortis — the most brownfield-intensive operator in the prior cycle at a 5.7% CAGR — delivered one of the sharpest margin expansions in the universe. Looking ahead, KIMS (5.6% brownfield forward CAGR), Max (6.2%), and Medanta (3.8%) are the names where brownfield additions will begin reflecting in revenue relatively quickly, likely within the same financial year of commissioning.

Greenfield — Revenue Accretive, But tricky on margins. Greenfield hospitals are a different proposition entirely. Land acquisition, construction, fit-out, regulatory approvals, and specialist recruitment stretch the timeline from capex commitment to first revenue to 24–36 months, and from first revenue to normalised occupancy to a further 12–18 months. The operators with the heaviest greenfield exposure in their forward pipeline — Jupiter at a 30.1% greenfield CAGR, Artemis at 25.5%, Medanta at 17.5%, Rainbow at 11.5%, and Narayana at 6.6% — will therefore see their capacity additions flow into the revenue line later and more gradually than acquisition or brownfield peers.

Nevertheless, we have seen greenfield expansion in the core markets and underpenetrated markets have surprised positively on ramp-up.

Our Expectation: Mid-Teen to High-Teen Revenue Growth, Differentiated by Modality. The combination of a 13.1% forward bed CAGR — nearly 2.4x the historical 5.6% — and sustained ARPOB compounding of 5–7% p.a. creates a sector-level revenue growth setup that we expect to sustain 18-20% growth through FY27–FY28. Operators with acquisition and brownfield-heavy pipelines — Max, Yatharth, KIMS, and Fortis — should reflect the tailwind most rapidly in their reported numbers. Greenfield-heavy names — Jupiter, Artemis, Medanta, Rainbow — will show the revenue benefit with a lag, but the incremental contribution will compound more cleanly once those assets reach maturity.

Exhibit 57: Heavy Greenfield Expansion Ahead

Hospitals	CAGR of Bed Expansion (9MFY26-FY29)					CAGR of Bed Expansion (FY20-9MFY26)*					CAGR FY20-FY25
	Brownfield	Greenfield	Acquisition	O&M/PHF	Total CAGR	Brownfield	Greenfield	Acquisition	O&M/PHF	Total CAGR	Revenue
Industry	**3.5%**	**6.6%**	**1.9%**	**1.1%**	**13.1%**	**1.6%**	**1.7%**	**2.1%**	**1.0%**	**5.6%**	**15.5%**
Max	**6.2%**	**10.1%**	**0.0%**	**6.8%**	**23.2%**	2.0%	1.3%	4.7%	0.0%	8.1%	16.6%
Apollo	**2.5%**	**4.5%**	**3.5%**	**0.0%**	**10.5%**	0.6%	0.1%	1.2%	0.0%	1.3%	14.2%
Fortis	**2.2%**	**0.0%**	**0.7%**	**0.0%**	**2.8%**	5.7%	0.5%	1.4%	3.6%	9.9%	11.7%
Narayana	1.6%	6.6%	0.0%	0.0%	8.2%	0.0%	0.6%	0.3%	-0.6%	-2.1%	10.6%
Aster	3.3%	9.1%	0.4%	0.0%	12.8%	2.3%	1.0%	0.0%	3.9%	7.0%	19.6%
Medanta	3.8%	17.5%	0.0%	0.0%	21.2%	2.1%	5.7%	0.0%	0.7%	8.5%	19.7%
KIMS	**5.6%**	**1.4%**	**0.0%**	**2.6%**	**9.6%**	0.5%	5.8%	5.4%	2.1%	13.8%	22.3%
Rainbow	0.0%	11.5%	0.0%	0.0%	11.5%	0.6%	5.4%	3.4%	0.0%	9.3%	16.1%
HCG	1.8%	0.0%	0.0%	0.0%	1.8%	1.5%	0.0%	1.5%	0.0%	1.1%	15.2%
Jupiter	3.4%	30.1%	0.0%	0.0%	33.5%	1.8%	6.3%	0.0%	0.0%	8.1%	22.3%
Yatharth	4.9%	0.0%	24.6%	0.0%	29.5%	0.0%	0.0%	13.7%	0.0%	13.7%	43.2%
Artemis	10.2%	25.5%	0.0%	0.0%	35.7%	10.5%	0.0%	0.0%	0.0%	10.5%	9.3%
Shalby	2.5%	0.0%	0.0%	0.0%	2.5%	0.2%	0.3%	1.0%	0.4%	2.0%	13.2%

Source: Company Data, Equirus

* *Divested Beds included in total CAGR but not in acquisition, brownfield, greenfield & O&M*

Capex: How the Expansion Modality Determines the Financial Outcome

The Rs. 248.5 billion capex pipeline across the listed hospital universe is not homogeneous — and the financial outcome for each operator will be determined less by how much capital they deploy and more by how they deploy it. Brownfield, acquisition, greenfield, and O&M each carry a fundamentally different cost-per-bed, ramp-up timeline, and margin and ROCE profile. Understanding this modality mix — not just the total bed count — is the key to identifying which operators will deliver margin expansion and ROCE recovery earliest.

Capital efficiency and ramp-up timelines will dictate ROCE trajectories across operators.

Capex Efficiency: The First Differentiator

At the portfolio level, the sector is deploying Rs 10.9 mn per bed on average — a figure that masks enormous dispersion. Yatharth at Rs 5.7 mn/bed and KIMS at Rs 6.3 mn/bed represent the most capital-efficient expansions in the universe, driven by their heavy reliance on lease, O&M structures and locations. Aster at Rs 8.4 mn and Jupiter at Rs 9.2 mn sit in the middle tier. At the expensive end, Apollo at Rs 19.2 mn/bed and Narayana at Rs 19.4 mn/bed reflect greenfield-heavy programmes where ground-up construction, land acquisition, and fit-out inflate the per-bed cost significantly. The implication is direct: greenfield-heavy operators are committing substantially more capital per unit of future revenue — which means the ROCE dilution during the ramp phase is deeper, and the recovery timeline longer.

Exhibit 58: Significant capex commitment ahead

Capex (Rs in mn)	4QFY26E	FY27E	FY28E	FY29E	Total	Bed Addition	Capex/Bed
Max	6,000	23,170	16,000	12,400	57,570	4,577	12.6
Apollo				54,000	54,000	2,815	19.2
Fortis		Meaningful capex already done			-	527	-
Narayana	-	840	20,900	8,000	29,740	1,535	19.4
Aster				20,000	20,000	2,368	8.4
Medanta	-	3,038	17,988	17,988	39,013	2,796	14.0
KIMS	-	7,950	1,500	1,500	10,950	2,051	5.3
Rainbow				Not Guided	-	880	-
HCG		751			751	119	6.3
Jupiter	-	4,250	5,000	4,000	13,250	1,446	9.2
Yatharth				15,260	15,260	2,700	5.7
Artemis	-	2,050	1,050	4,875	7,975	1,050	7.6
Shalby		Not meaning full capex in coming year			-	175	-
Total					**2,48,509**	**23,039**	**10.8**

Source: Company Data, Equirus

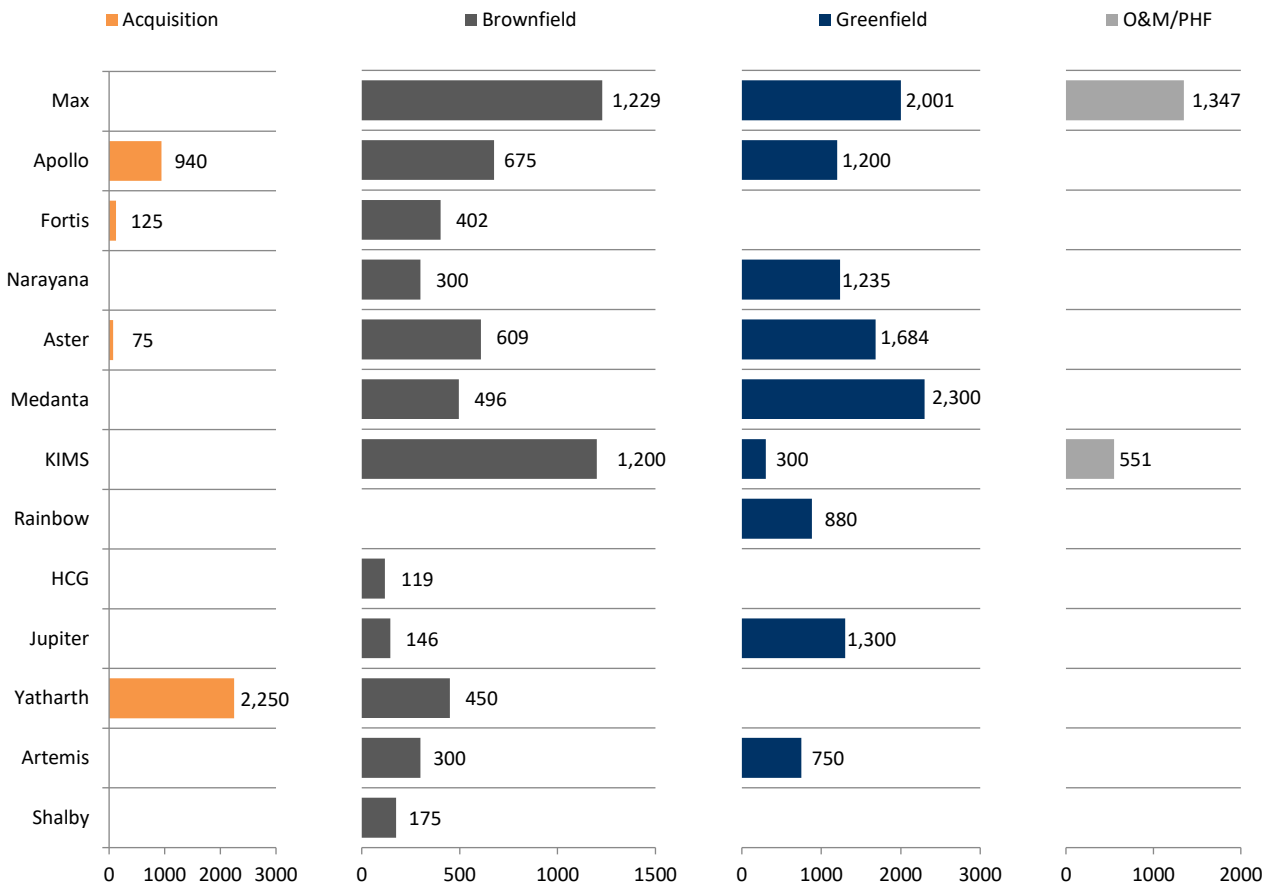
Brownfield — Margin Accretive Almost Immediately

Brownfield additions are the most powerful near-term margin lever in the pipeline. They are added to facilities already operating at mature occupancy — Max at 76%, Fortis at 69%, Apollo at 67%, Medanta at 62% — where the fixed cost base is fully absorbed and the incremental bed operates at near-zero marginal infrastructure cost. Ramp-up to normalised utilisation takes just one to two quarters because the catchment, specialist team, referral network, and brand are all in place. Every additional percentage point of occupancy on an already-mature facility drops through to EBITDA at a structurally higher conversion rate than a new asset ramping from zero. Max (1,229 brownfield beds), KIMS (1,200), Apollo (675), Medanta (496), Yatharth (450), Fortis (402), and Narayana (300) are the key beneficiaries here — and among these, Max and KIMS carry the most material brownfield bed additions in absolute terms, meaning the margin accretion from brownfield will be visible in their P&Ls within two to three quarters of commissioning.

Greenfield — The Long Game, and Where ROCE Pressure Will Be Felt

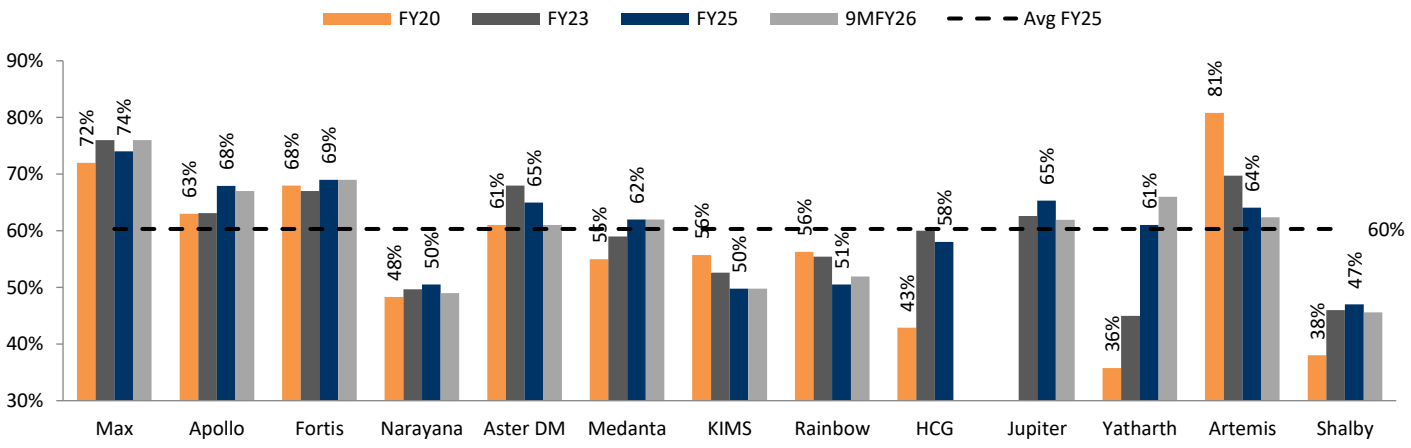
Greenfield is the most complex variable in the financial model. Construction timelines of 24–36 months, followed by 12–18 months of ramp-up to normalised occupancy, mean that greenfield beds committed today will not make meaningful EBITDA contributions until FY28 at the earliest — and full ROCE maturity will not arrive until FY29–FY30 for the larger programmes. The operators with the heaviest greenfield exposure are carrying the most pronounced ROCE pressure in the near-to-medium term. Medanta's 2,300 greenfield beds — the largest absolute greenfield programme in the universe — alongside a relatively high Rs 14 mn/bed capex, will weigh on its ROCE through FY27 before the Lucknow, Patna, and Ranchi assets begin to normalise. Apollo's 1,200 greenfield beds at Rs 19.2 mn/bed carry the highest per-bed cost in the universe, reflecting premium metro location construction. Aster's 1,684 greenfield beds, Narayana's 1,235, Jupiter's 1,300, Rainbow's 880, and Artemis's 750 all carry this same delayed-return dynamic. The ROCE trough for these operators will likely deepen through FY26–FY27 before the ramp inflection begins.

Exhibit 59: Future bed expansion skews heavily towards greenfield



Source: Company Data, Equirus

Exhibit 60: Steady Occupancy Expansion Across Players Over FY20–FY25



Source: Company Data, Equirus

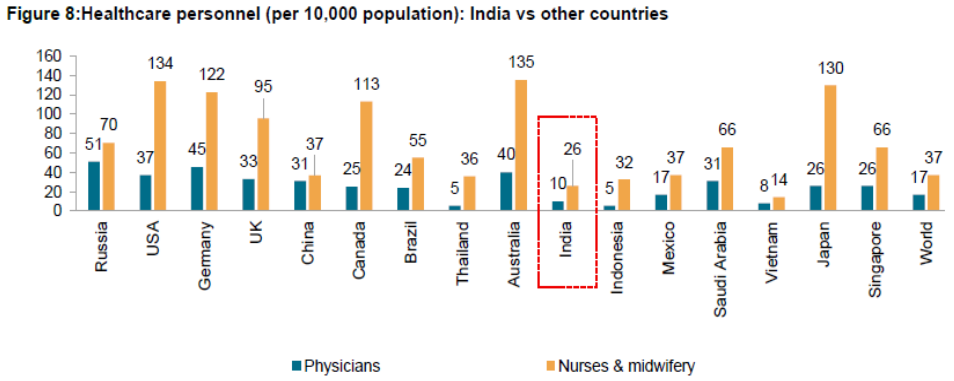
The Doctor Deficit: A Structural Cost Pressure Hidden Within the Demand Story

Running parallel to the demand acceleration is a supply-side constraint that will define the hospital sector's cost structure over the next five years: the critical shortage of qualified physicians and nursing professionals. India had just 10 physicians and 26 nursing and midwifery personnel per 10,000 population as of CY22 — against a global average of 17 physicians and 37 nursing personnel. Every key comparable — Brazil (24 physicians), Thailand (24), Mexico (17) — is better supplied than India on a per-capita basis.

Beds are rising faster than doctors - talent becomes the bottleneck.

The implications for the hospital sector are significant and multidimensional. First, as hospitals add beds aggressively – our universe will add 23,039 beds by FY29 – the demand for specialist physicians will intensify far faster than supply can respond. Medical colleges in India produce approximately 100,000 MBBS graduates annually, but the pipeline for super-specialists (cardio, onco, neuro, hepatobiliary surgeons) takes 10–12 years from graduation and is structurally inadequate relative to the installed bed capacity being built.

Exhibit 61: Healthcare personnel structural shortage in India



Source: Manipal Hospitals DRHP, Equirus

Exhibit 62: FY25 – Beds per Doctor Data – Hospital wise

Hospitals	Doctors	Bed capacity	Beds per Doctor
Max	5,000	4,930	0.99
Apollo	13,000	10,187	0.78
Narayana	4,000	5,745	1.44
Aster	3,302	5,159	1.56
Medanta	1,000	3,042	3.04
KIMS	2,212	5,444	2.46
Rainbow	910	1,935	2.13
Jupiter	1,275	1,061	0.83
Yatharth	750	1,605	2.14
Artemis	500	700	1.40
Total	**31,949**	**39,808**	**1.25**

Source: Company Data, Equirus

Second, specialist doctors are not uniformly distributed – they are concentrated in the same metro geographies where hospitals are competing most aggressively for capacity share. Delhi NCR, Mumbai, Bengaluru, Hyderabad, and Chennai collectively host a disproportionate share of India's super-specialists, creating a talent market that will only tighten as bed capacity expands. The consequence: hospitals in these markets should expect structural escalation in specialist compensation, both on fixed pay and variable/consulting fee arrangements.

Margin and Return Outlook: Operating Leverage Meets Expansion Dilution

The sector delivered a 25% EBITDA CAGR over FY20–FY25 – outpacing revenue growth of 15.5% by nearly 1,000 bps every year – and expanded blended margins by ~780 bps from 15.7% to 23.5%. The forward question is not whether EBITDA will grow, but at what pace, and which operators will see margin expand versus compress as the Rs 248.5 billion capex cycle plays out.

Strong EBITDA compounding continues, with margins temporarily moderated by expansion before re-accelerating as new capacity matures.

The Sector-Level Setup: Mid-20s Margins Are Achievable, High-20s Require Maturation

The listed universe as a whole converged to a 23–24% EBITDA margin band by FY25–9MFY26 – a structural re-rating from the fragmented 15–18% range of FY20. This convergence was driven by operating leverage, group procurement benefits, ARPOB expansion, and fast new-bed ramp-ups crossing breakeven. Looking ahead, the same forces remain active, but they interact with a more complex capital cycle. The sector-level margin is likely to oscillate between 22–25% through FY26–FY27 – temporarily capped by greenfield ramp-up fixed cost drag across several operators – before expanding toward 25–27% for the best operators by FY28–FY29 as newly added beds reach mature occupancy and the operating leverage flywheel re-engages. The EBITDA absolute quantum will compound meaningfully – we expect industry EBITDA growth to moderate to 16-17% CAGR through FY27, as greenfield expansion dilutes the margins.

The Structural Margin Narrative for FY26–FY29: At the sector level, the margin story has three overlapping phases. Through FY26–FY27, greenfield ramp-up drag across Jupiter, Medanta, Artemis, Narayana, and Rainbow will create a modest sector-wide margin overhang — blended margins likely range in the 20–23% band. From FY27 onward, as the greenfield cohort crosses breakeven occupancy, the operating leverage re-engages and margin accretion resumes — we expect the sector to exit FY29 with blended margins in the 24–26% range for the universe, with leading brownfield and acquisition-focused operators (Max, Fortis, Apollo, KIMS post-recovery) comfortably at 26–28%.

ROCE: Trough Timing Mapped to Modality

The ROCE outlook maps directly onto the modality mix. Operators with acquisition and brownfield-heavy forward pipelines – Yatharth, Max, KIMS, Fortis – are positioned to see ROCE stabilise or begin recovering from 9MFY26 levels as new beds reach normalised occupancy within one to two quarters of commissioning. Operators with greenfield-heavy programmes – Jupiter, Medanta, Artemis, Rainbow, Narayana – should be expected to trough ROCE in FY26–FY27 before the recovery begins in FY28E/FY29E as beds commission and ramp. This is not structural deterioration – it is the textbook ROCE arc that Max and Medanta themselves demonstrated in FY21–FY23 – but it does mean the re-rating timing will differ meaningfully across the universe.

Conclusion: Growth is sector-wide; the return journey is phased

India's listed hospital sector enters its next phase of growth with the most favourable combination of structural demand tailwinds, disciplined capacity strategy, and financial maturity in its history. The past five years proved that the sector can compound revenue at 15%+, expand EBITDA margins by 780 bps, and improve ROCE — simultaneously, through a capital cycle that would have broken a less structurally sound business model. The next five years are set up to deliver a continuation of this trajectory, with the added benefit that several high-capex players are now approaching the inflection point of their expansion cycle and will begin to demonstrate the same operating leverage and FCF improvement that mature players showed in FY23–FY25.

Equirus Conviction: India's hospital sector is not at the end of a multi-year compounding cycle — it is at the beginning of its most mature, best-capitalised, and most structurally supported phase of growth. The demand tailwinds are irreversible, the expansion is disciplined, the financial model is self-funding, and the policy environment is the most supportive in a decade. We maintain our constructive sector stance and view any near-term ROCE trough or FCF burn at expansion-phase players as setup, not structural risk.

Top Picks in the Sector

Within this constructive sector backdrop, we find Apollo Hospitals and KIMS as our top picks in the covered universe, with Max Healthcare and Fortis Healthcare as our preferred names in the non-covered space.

Apollo Hospitals:

Apollo's new bed addition is a well-blended mix of acquisitions, brownfield expansions, and greenfield developments, offering scale while providing a natural floor to margins. Management has guided 8-10% revenue growth from incremental beds, with existing capacity delivering 11-12% — pointing to a 20-22% CAGR over the next three years, a target we view as achievable with limited margin contraction. Over FY26-28, acquisition-led additions in Bengaluru, Pune, and Kolkata — with established catchments and brand recall — support faster ramp-ups and sub-12-month breakeven, while Gurgaon's and Hyderabad losses, given its greenfield-like startup profile, are likely offset by sharp turnarounds in these acquisition-led beds. Net margin impact is expected to remain contained within 100 bps, with drag concentrated in FY27 and recovery visible from FY28.

KIMS:

KIMS has undergone a massive capex cycle over FY25-FY26 with commercialisation of greenfield hospitals across Bengaluru (two units), Thane, Nashik and Kerala (two leased hospitals). All these hospitals combined have dented its ROIC profile, considering they were predominantly greenfield expansions requiring 12-18 months to breakeven. However, KIMS now offers one of the most visible ROCE recovery setups in the listed hospital universe — with recently commissioned beds progressively breaking even over the next 2-3 quarters, the drag on consolidated margins is structurally behind us.

Moreover, the future expansion is more skewed towards brownfield expansion with the biggest being commercialisation of Kondapur beds in early FY27 and gradual ramp-up of recently commercialised greenfield beds as occupancy picks up across the new sites.

On numbers, we forecast 25% revenue CAGR with 41% EBITDA CAGR over FY26-FY28E, as margins recover sharply to ~26% in FY28E from ~20% in FY26 — a 600-bps expansion driven entirely by operating leverage on a largely fixed-cost base. We reiterate **LONG** with a Mar'27 TP of ₹863.

Uncovered

Max healthcare

Max Healthcare is set for structural long-term growth, with near-term performance anchored in new bed ramp-ups at Saket, Nanavati, Gurugram (H1 FY27E) and Mohali O&M. Recent acquisitions — Noida (Jaypee), Lucknow and Nagpur — are turning around and will underpin EBITDA growth. We expect margins to hold through this expansion phase given the brownfield-heavy mix; Gurugram Phase 1 should drive a meaningful earnings step-up. Beyond FY28, a healthy mix of greenfield and brownfield additions across Delhi NCR, Thane, Mohali and Pune provides a well-distributed long-term pipeline.

We believe Max is set to deliver healthy double-digit revenue growth of 18-20% over the next five years, while maintaining or improving margins and return metrics. This will be further aided by the recent CGHS price revision, which could add ~Rs. 1.2 bn to annual EBITDA

Fortis- a near term play:

Fortis has recently commissioned new brownfield capacity at Faridabad and Kolkata, with FMRI and Manesar brownfield beds set to follow. The ramp-up of recently acquired People Tree in Bangalore, combined with fee income from O&M contracts, supports near-term (12-18 month) growth visibility. However, beyond this, the absence of a sizeable greenfield or acquisition pipeline leaves long-term growth triggers relatively limited

Valuation Table

Exhibit 63: Valuations have moderated following recent correction

Company Name	CMP	Mkt Cap (Bn)	FY20-25 CAGR			P/E			EV/EBITDA			P/B			ROE		
			Return	Sales*	EBITDA*	FY26	FY27	FY28	FY26	FY27	FY28	FY26	FY27	FY28	FY26	FY27	FY28
Max	975	949	59%	17%	31%	57	46	37	37	30	25	8.0	6.9	5.9	16%	17%	17%
Apollo	7,549	1,085	42%	14%	20%	57	45	36	31	25	21	11.1	9.2	7.4	21%	22%	22%
Fortis	814	615	41%	12%	24%	57	43	34	30	25	21	6.2	5.4	4.7	11%	13%	15%
Narayana	1,636	332	47%	11%	21%	38	30	25	22	17	15	7.6	6.2	5.0	22%	22%	22%
Aster DM	668	345	38%	20%	33%	87	60	44	39	24	19	9.0	7.4	6.5	11%	13%	18%
Medanta	985	265	NA	20%	35%	48	38	31	29	23	19	6.8	5.8	4.9	15%	16%	17%
KIMS	636	255	NA	22%	27%	91	50	34	35	25	19	10.4	8.7	7.0	12%	18%	22%
Rainbow	1,164	118	NA	16%	19%	44	36	30	23	19	17	7.0	6.2	5.3	17%	18%	18%
HCG	532	79	49%	15%	18%	141	54	34	20	16	13	7.5	6.7	5.7	5%	12%	16%
Jupiter	1,280	84	NA	22%	29%	45	40	31	25	22	18	5.4	4.8	4.2	13%	13%	14%
Yatharth	642	62	NA	43%	42%	35	25	19	21	15	11	3.4	3.1	2.6	10%	12%	14%
Artemis	216	34	15%	9%	23%	31	23	16	17	15	10	3.4	3.0	2.5	12%	13%	16%
Shalby	131	14	34%	13%	17%	131	60	34	13	13	5	1.4	1.3	0.6	2%	4%	13%

*Source: Bloomberg; Note: *Sales and EBITDA pertain to Hospital segment*

equirus SECURITIES

Apollo Hospitals Enterprise Limited

India's Most Complete Healthcare Compounder

CMP	Target Price
Rs 7,549	Rs 9,395 Mar 2027
Rating	Upside
LONG	24% (↑)

- Apollo Hospitals — deepest specialist clinical brand; consistent 9% ARPOB CAGR, 14% hospital revenue CAGR, and 500+ bps EBITDA margin expansion to 24% over FY20-25.
- 2,815 beds through FY29 — 75% in highest-ARPOB metros — driving 17% consolidated revenue CAGR to Rs. 344.8bn and 21% EBITDA CAGR to Rs. 54.4bn by FY28E.
- Pharmacy at 18% CAGR to Rs. 150.5bn on ~600 store additions/yr and mid-teen SSSG; digital approaching breakeven — margin expands to 15.8% and ROCE +600 bps to 25.4% by FY28E.
- Retain LONG; Mar'27 SOTP TP Rs. 9,395 — hospitals/pharmacy/AHLL at 26x/25x/15x EV/EBITDA.

Consistent mid-teen growth in Hospital Business: Apollo's hospital business has delivered a compelling combination of volume, yield, and capital efficiency over FY20-25. ARPOB compounded at 9% p.a., driven by CONGO-T mix deepening and conversion of general wards to premium private formats, while occupancy improved from 63% to 68% and ALOS compressed. This operating leverage translated into strong financials — hospital revenue grew at 14% CAGR to Rs. 111.5bn, EBITDA at 20% CAGR with 500+ bps margin expansion to 24%, and ROCE expanded from 13% to 26% through the cycle. Capital discipline remained intact, with Capex/CFO at 0.5x — best among large-cap peers — and cumulative FCF of Rs. 40.2bn generated despite deploying Rs. 42.9bn in capex.

Expansion-led margin dip in FY27; recovery from FY28: Out of ~2,815 beds addition across FY27-FY29, ~1,000 are slated for FY27 commissioning across Gurgaon, Hyderabad, Bengaluru, and Kolkata. This expansion implies ~Rs. 10bn FY27 revenue but ~Rs. 1.5bn losses from new hospitals, driving an estimated 150bps FY27E margin decline vs FY26, partially offset by revision in CGHS prices and improving profitability in existing beds. From FY28, we expect new beds to turn accretive as occupancy ramps, scaling to ~Rs. 22bn revenue and ~Rs. 3.5bn EBITDA, alongside growth in CONGO specialties and better institutional tariffs.

Pharmacy steady; digital breakeven ahead: Apollo pharmacy retained healthy 18% growth in 9MFY26 with margins steady at 8% as generics/private labels reached 16% of mix. With ~600 annual store additions, mid-teen SSSG, and high-teen online GMV growth, we model ~18% medium-term pharmacy growth. Continued opex rationalisation and lower ESOP costs should enable digital breakeven by FY28, supporting a ~220bps HealthCo EBITDA margin expansion to 6.6% over FY26E-FY28E.

Ramp-up and digital turnaround to lift margins: While FY27 may see a modest margin dip from capacity additions, we model 100bps combined margin expansion to 16% over FY26-FY28E. Drivers include (a) EBITDA accretion as new hospitals ramp up, (b) scaling of high-value CONGO-T specialties, and (c) improving pharmacy profitability as digital turns EBITDA positive. This supports a 600bps ROCE improvement to 25.4% over the same period.

Financial Summary

YE Mar Rs mn	Sales	EBITDA	Recurring PAT	EPS (Rs)	P/E (x)	P/B (x)	EV/ EBITDA (x)	ROE (%)	Core ROIC (%)	EBITDA Margin (%)
FY25A	2,17,937	30,215	13,796	95.9	78.7	13.2	37.7	18.2	7.2	13.9
FY26E	2,49,499	37,073	18,826	130.9	57.7	11.1	30.6	20.9	8.4	14.9
FY27E	2,96,311	44,003	25,161	175.0	43.1	9.2	25.6	23.3	10.1	14.9
FY28E	3,44,765	54,408	33,001	229.5	32.9	7.5	20.6	25.2	12.0	15.8

Source: Company, Equirus Securities

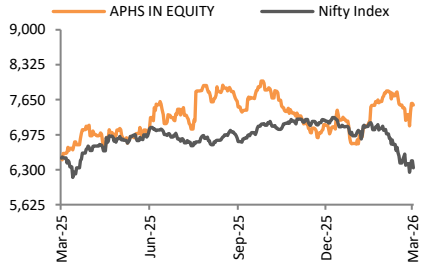
Estimate unchanged

	Forecasts		% Change	
(Rs mn)	FY26E	FY27E	FY26E	FY27E
Sales	2,49,499	2,96,311	-	-
EBITDA	37,073	44,003	-	-
PAT	18,826	25,161	-	-
EPS	130.9	175.0	-	-

Stock Information	
Market Cap (Rs Mn)	10,85,430
52 Wk H/L (Rs)	8,100/6,430
Avg Daily Volume (1yr)	3,96,006
Avg Daily Value (Rs Mn)	30.9
Equity Cap (Rs Mn)	82,123
Face Value (Rs)	5
Share Outstanding (Mn)	143.8
Bloomberg Code	APHS IN
Ind Benchmark	

Ownership (%)	Recent	3M	12M
Promoters	28.0	0.0	(1.3)
DII	21.5	0.4	1.5
FII	43.5	(0.7)	(1.8)
Public	6.9	0.3	1.5

Relative price chart



Source: Bloomberg

Analysts
Bharat Celly
bharat.celly@equirus.com
+91-079 6901 5036
Vinay Jain
vinay.jain@equirus.com
+91-079 6901 5011

Broad Based but Metro centric Capacity Addition

Hospital Operating Matrix: Yield Over Volume

Apollo's FY20–FY25 operating matrix reflects a deliberate yield-first strategy. ARPOB compounded at 9% p.a. (Rs 39,744→Rs 60,979) – the most consistent large-cap yield trajectory in the sector – driven by CONGO-T case mix deepening to 67% revenue share, insurance mix expansion, and conversion of general wards into premium private formats. This ward repositioning restrained net bed additions to a 1% CAGR (7,491→8,025 beds) but structurally repriced the portfolio. Occupancy improved from 63% to 68%, ALOS compressed from 3.86 to 3.32 days as surgical mix rose, and inpatient volumes grew at a 5% CAGR to 6.04 lakh – confirming that the ARPOB story was layered on genuine volume growth, not achieved at its expense.

Exhibit 64: ARPOB led growth with continuous occupancy improvement

Operating Matrix	FY20	FY21	FY22	FY23	FY24	FY25	9MFY26	CAGR FY20-FY25
ARPOB	39,744	34,747	52,523	52,403	57,172	60,979	67,805	9%
Operating Beds	7,491	7,409	7,875	7,860	7,945	8,025	8,072	1%
Occupancy	63%	63%	58%	63%	72%	68%	67%	
ALOS	3.86	4.19	3.96	3.4	3.3	3.32		
In-Patient	4,78,012	3,52,624	4,60,152	5,40,881	5,64,046	6,04,250	4,72,263	5%
Out-Patient	16,27,514	11,60,247	25,14,365	18,79,171	21,36,793	21,84,357	17,47,298	6%

Source: Company Data, Equirus

Exhibit 65: Bed movement across Metro and Non Metro over FY20-9MFY26

Region	FY20	Acquisition	Brownfield	Greenfield	Restructure	Addition	9MFY26
Chennai	1523				-201	-201	1322
Hyderabad	759				28	28	787
Bangalore	559				9	9	568
Delhi NCR	584		165	27		192	776
Mumbai	392				0	0	392
Kolkata	736				0	0	736
Metro	4553	0	165	27	-164	28	4581
Tamil Nadu ex Chennai	685				-20	-20	665
AP, Telangana ex Hyderabad	585				-82	-82	503
Karnataka ex Bangalore	210				3	3	213
Norther Region ex Delhi NCR	212	243				243	455
Western Region ex Mumbai	410	75	86			161	571
Eastern Region ex Kolkata	836	220	28			248	1084
Non Metro	2938	538	114	0	-99	553	3491
Total	7491	538	279	27	-263	581	8072
% rise in Bed		7%	4%	0%	-4%	8%	

Source: Company Data, Equirus

Financial Matrix and Returns: Self-Funding Through the Cycle

The operating matrix translated cleanly into financials. Hospital revenue grew 14% CAGR to Rs 111.5 bn (FY25), with EBITDA compounding at 20% to Rs 27.0 bn – a 500+ bps margin expansion to 24% alongside a near-3x capex surge. Capex to CFO of 0.5 over FY20–25 is the strongest among large-cap peers, with cumulative FCF of Rs 40.2 bn generated while deploying Rs 42.9 bn of capex. ROCE expanded from 13% to 26% through the cycle – validating the quality of capital deployment. Net Debt/EBITDA, which stood at 2.5x in FY20, compressed to 0.6x by FY23 and remains manageable at 1.3x in FY25 despite the accelerated capex programme.

Exhibit 66: Strong financials with mid teen growth and highest free cash flow maker

Financial Matrix	FY20	FY21	FY22	FY23	FY24	FY25	9MFY26	CAGR FY20-FY25
Hospital revenue	57,297	42,681	65,823	86,769	98,674	1,11,474	92,873	14%
EBITDA	10,750	6,087	18,031	21,332	23,559	27,005	22,887	20%
EBITDA Margin	19%	14%	27%	25%	24%	24%	25%	
CFO	12,043	11,832	14,102	14,136	19,466	23,512	13,253	
Capex	-5,106	-2,844	-4,308	-11,435	-10,139	-14,152	-7,132	
FCF	6,937	8,988	9,795	2,701	9,327	9,360	6,121	
ROCE	13%	5%	23%	24%	24%	26%	27%	
Net Debt/EBITDA	2.5			0.6		1.3		

Source: Company Data, Equirus

Metro-Centric Expansion – Right Geography, Right Yield

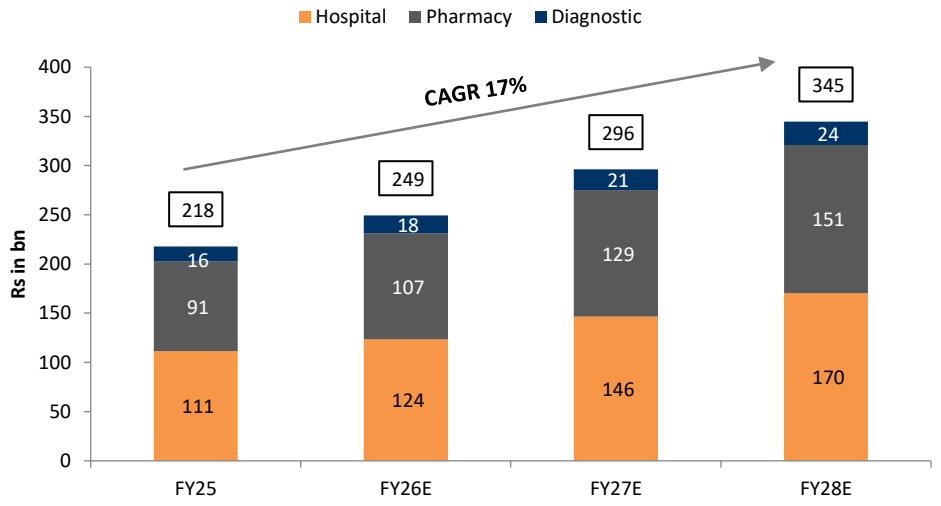
Apollo's bed expansion pipeline is the most metro-concentrated of any large operator – 75% of the 2,815 beds being added through FY29 are in metro markets. These are precisely the markets with the highest ARPOB, the deepest corporate and private insurance penetration, and the highest medical tourism inflows. The greenfield-heavy nature of this expansion (~1,200 of 2,815 beds) means the FY27 ramp-up phase will see approximately Rs 1.5 bn of new hospital losses against approximately Rs 10 bn of new revenue — a well-flagged ~150 bps EBITDA margin dilution in FY27. However, by FY28, as the ~1,000 FY27-commissioned beds across Gurugram, Hyderabad, Bengaluru, and Kolkata reach normalised occupancy, these assets are expected to contribute ~Rs 22 bn of revenue and ~Rs 3.5 bn of EBITDA — transforming the near-term drag into a structural earnings step-up.

Exhibit 67: Strong metro centric – greenfield + acquisition driven expansion

Region	9MFY26	Acquisition	Brownfield	Greenfield	Addition	FY29
Chennai	1,322			500	500	1,822
Hyderabad	787		110	300	410	1,197
Bangalore	568	150	200		350	918
Delhi NCR	776	400			400	1,176
Mumbai	392			250	250	642
Kolkata	736	225			225	961
Metro	**4,581**	**775**	**310**	**1,050**	**2,135**	**6,716**
Tamil Nadu ex Chennai	665					665
AP, Telangana ex Hyderabad	503		160		160	663
Karnataka ex Bangalore	213		125		125	338
Norther Region ex Delhi NCR	455		80	150	230	685
Western Region ex Mumbai	571	165			165	736
Eastern Region ex Kolkata	1,084					1,084
Non Metro	**3,491**	**165**	**365**	**150**	**680**	**4,171**
Total	**8,072**	**940**	**675**	**1,200**	**2,815**	**10,887**
% rise in Bed		**12%**	**8%**	**15%**	**35%**	

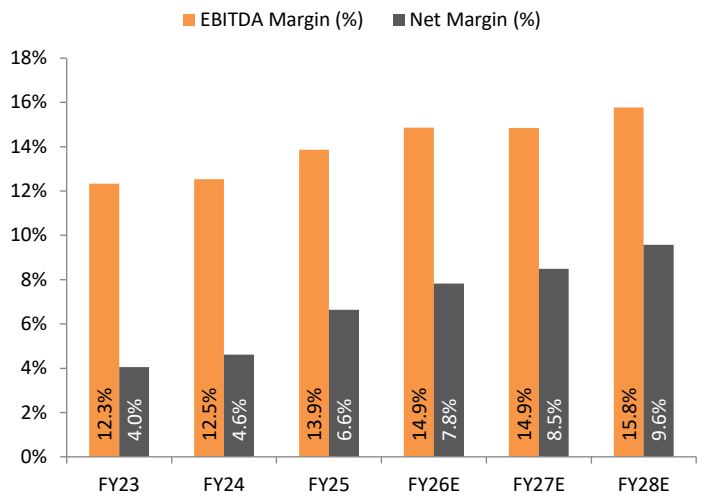
Source: Company Data, Equirus

Exhibit 68: Mid to High teen Revenue CAGR across the segment



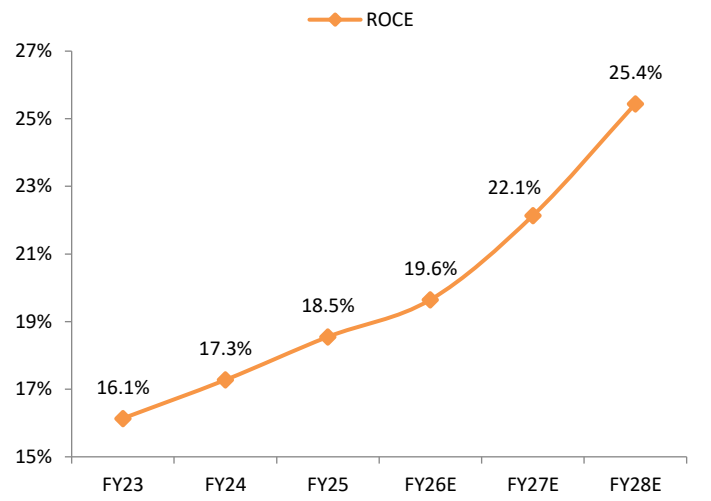
Source: Company Data, Equirus

Exhibit 69: A story of consol margin expansion…



Source: Company Data, Equirus

Exhibit 70: …and improving return profile



Source: Company Data, Equirus

Quarterly performance

Y/E Mar (Rs mn)	1QFY25A	2QFY25A	3QFY25A	4QFY25A	1QFY26A	2QFY26A	3QFY26A	4QFY26E
Revenue	**55,893**	**55,893**	**55,269**	**58,421**	**63,035**	**63,035**	**64,774**	**63,269**
COGS	28,577	28,577	29,007	30,476	32,436	32,436	33,824	33,038
Employee Cost	7,001	7,001	6,864	7,126	7,667	7,667	7,490	8,000
Other Expenses	12,160	12,160	11,783	12,300	13,521	13,521	13,807	12,741
EBITDA	**8,155**	**8,155**	**7,615**	**8,519**	**9,411**	**9,411**	**9,653**	**9,490**
Depreciation	1,845	1,845	1,846	2,147	2,178	2,178	2,192	2,148
EBIT	**6,310**	**6,310**	**5,769**	**6,372**	**7,233**	**7,233**	**7,461**	**7,342**
Interest Exp.	1,175	1,175	1,098	1,083	1,096	1,096	1,126	1,433
Other Income	382	382	638	402	547	547	528	956
Profit before Tax	**5,517**	**5,517**	**5,309**	**5,691**	**6,684**	**6,684**	**6,863**	**6,865**
Tax Expenses	1,617	1,617	1,568	1,417	1,807	1,807	1,657	1,657
Profit After Tax	**3,900**	**3,900**	**3,741**	**4,274**	**4,877**	**4,877**	**5,206**	**5,207**
Minority Interest	169	169	71	82	168	168	140	0
Profit/(Loss) from Associates	57	57	53	136	63	63	149	0
Recurring PAT	**3,674**	**3,674**	**3,617**	**4,056**	**4,646**	**4,646**	**4,917**	**5,207**
Exceptional Items	0	0	0	0	0	0	192	0
Reported PAT	**3,674**	**3,674**	**3,617**	**4,056**	**4,646**	**4,646**	**5,109**	**5,207**
Other comprehensive income.	0	0	0	0	0	0	0	0
PAT after comp. income.	**3,674**	**3,674**	**3,617**	**4,056**	**4,646**	**4,646**	**5,109**	**5,207**
FDEPS	**25.6**	**25.6**	**25.2**	**28.2**	**32.3**	**32.3**	**35.5**	**36.2**
Cost items as % of sales								
RM expenses	51.1	51.1	52.5	52.2	51.5	51.5	52.2	52.2
Employee expenses	12.5	12.5	12.4	12.2	12.2	12.2	11.6	12.6
Other expenses	21.8	21.8	21.3	21.1	21.4	21.4	21.3	20.1
Margin (%)								
Gross Margin	48.9	48.9	47.5	47.8	48.5	48.5	47.8	47.8
EBITDA Margin	14.6	14.6	13.8	14.6	14.9	14.9	14.9	15.0
PAT Margin	6.6	6.6	6.5	6.9	7.4	7.4	7.9	8.2
YoY Growth (%)								
Sales	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
EBITDA	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
EBIT	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0
PAT	0.0	0.0	0.0	0.0	0.0	0.0	0.0	0.0

Key Financials (Consolidated)

Income Statement

Y/E Mar (Rs mn)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Revenue	**1,46,626**	**1,66,125**	**1,90,592**	**2,17,937**	**2,49,499**	**2,96,311**	**3,44,765**
COGS	75,735	85,742	98,055	1,13,100	1,29,774	1,53,193	1,78,244
Employee Cost	17,865	21,438	24,937	27,692	30,283	33,917	37,987
Other Expenses	31,175	38,448	43,693	46,930	52,369	65,199	74,127
EBITDA	**21,851**	**20,496**	**23,907**	**30,215**	**37,073**	**44,003**	**54,408**
Depreciation	6,007	6,152	6,870	7,575	8,665	9,146	9,699
EBIT	**15,844**	**14,344**	**17,037**	**22,640**	**28,408**	**34,857**	**44,709**
Interest Exp.	3,786	3,808	4,494	4,585	4,738	5,130	4,530
Other Income	781	903	1,063	2,003	2,433	3,822	3,822
Profit before Tax	**12,839**	**11,439**	**13,606**	**20,058**	**26,103**	**33,548**	**44,001**
Tax Expenses	3,888	4,028	4,455	5,340	6,538	8,387	11,000
Profit After Tax	**8,951**	**7,411**	**9,151**	**14,718**	**19,564**	**25,161**	**33,001**
Minority Interest	527	255	364	592	390	0	0
Profit/(Loss) from Associates	74	(432)	0	330	348	0	0
Recurring PAT	**8,350**	**7,588**	**8,787**	**13,796**	**18,826**	**25,161**	**33,001**
Exceptional Items	(2,059)	(1,466)	0	0	0	0	0
Reported PAT	**6,291**	**6,122**	**8,787**	**13,796**	**18,826**	**25,161**	**33,001**
Other comprehensive income.	0	0	0	0	0	0	0
PAT after comp. income.	**6,291**	**6,122**	**8,787**	**13,796**	**18,826**	**25,161**	**33,001**
FDEPS	43.8	42.6	61.1	95.9	130.9	175.0	229.5
DPS	12	15	10	10	27	35	46
BVPS	393	431	482	571	680	820	1,003

YoY Growth (%)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Sales	38.9	13.3	14.7	14.3	14.5	18.8	16.4
EBITDA	92.1	(6.2)	16.6	26.4	22.7	18.7	23.6
EBIT	180.8	(9.5)	18.8	32.9	25.5	22.7	28.3
PAT	2,179.2	(2.7)	43.5	57.0	36.5	33.7	31.2

Key Ratios

Profitability (%)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Gross Margin	48.3	48.4	48.6	48.1	48.0	48.3	48.3
EBITDA Margin	14.9	12.3	12.5	13.9	14.9	14.9	15.8
PAT Margin	4.3	3.7	4.6	6.3	7.5	8.5	9.6
ROE	16.3	12.8	13.4	18.2	20.9	23.3	25.2
ROIC	10.8	8.8	9.5	11.5	12.8	15.5	17.7
Core ROIC	6.1	4.2	4.9	7.2	8.4	10.1	12.0
Dividend Payout	26.9	35.2	16.4	10.4	20.7	20.0	20.0

CAGR (%)	1 year	2 years	3 years	5 years	7 years	10 years
Revenue	14.3	14.5	14.1	14.1	14.9	15.5
EBITDA	26.4	21.4	11.4	13.7	21.1	15.2
PAT	57.0	50.1	29.9	84.5	42.3	16.7

Valuation (x)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
P/E	172.5	177.3	123.5	78.7	57.7	43.1	32.9
P/B	19.2	17.5	15.7	13.2	11.1	9.2	7.5
P/FCFF	0.0	0.0	0.0	0.0	0.0	0.0	0.0
EV/EBITDA	50.9	54.3	46.7	37.7	30.6	25.6	20.6
EV/Sales	7.6	6.7	5.9	5.2	4.5	3.8	3.3
Dividend Yield (%)	0.2	0.2	0.1	0.1	0.4	0.5	0.6

Balance Sheet

Y/E Mar (Rs mn)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Equity Capital	719	719	719	719	719	719	719
Reserves	55,733	61,253	68,635	81,404	97,022	1,17,151	1,43,551
Net Worth	**56,452**	**61,971**	**69,354**	**82,123**	**97,741**	**1,17,870**	**1,44,270**
Total Debt	26,357	27,103	31,619	52,752	47,752	42,752	37,752
Other long term liabilities	25,043	26,341	25,325	30,086	30,086	30,086	30,086
Minority Interest	2,543	3,341	3,851	4,406	4,406	4,406	4,406
Account Payables	16,318	19,157	23,686	22,405	27,342	32,472	37,783
Other Current Liabilities	5,429	6,364	13,696	14,802	16,946	20,125	23,416
Total Liabilities	**1,32,142**	**1,44,277**	**1,67,531**	**2,06,574**	**2,24,273**	**2,47,711**	**2,77,713**
Gross Fixed Assets	99,654	1,06,720	1,17,607	1,33,515	1,42,515	1,51,015	1,59,515
Acc. Depreciation	27,688	33,841	40,711	48,286	56,951	66,097	75,796
Net Fixed Assets	71,966	72,880	76,896	85,229	85,564	84,917	83,719
Capital WIP	455	6,098	8,728	9,210	9,210	9,210	9,210
long term investments	3,003	2,815	3,021	10,545	10,545	10,545	10,545
Others	15,505	19,113	26,089	31,301	31,301	31,301	31,301
Inventory	4,318	3,902	4,598	4,808	6,836	8,118	9,446
Receivables	17,676	22,342	25,149	30,161	30,760	36,531	42,505
Loans and advances	41	56	49	51	51	51	51
Other current assets	8,821	9,313	13,663	21,667	22,731	24,308	25,941
Cash & Cash Equivalents.	10,359	7,758	9,338	13,602	27,276	42,729	64,995
Total Assets	**1,32,142**	**1,44,277**	**1,67,531**	**2,06,574**	**2,24,273**	**2,47,711**	**2,77,713**
Non-Cash WC	9,109	10,092	6,077	19,480	16,089	16,411	16,744
Cash Conv. Cycle	(12.3)	(14.0)	(18.4)	(12.9)	(9.0)	(12.0)	(12.1)
WC Turnover	16.1	16.5	31.4	11.2	15.5	18.1	20.6
Gross Asset Turnover	1.5	1.6	1.6	1.6	1.8	2.0	2.2
Net Asset Turnover	2.0	2.1	2.2	2.3	2.6	3.1	3.7
Net D/E	0.3	0.3	0.3	0.5	0.2	0.0	(0.2)

Days (x)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Receivable Days	39	44	45	46	45	41	42
Inventory Days	16	17	16	15	16	18	18
Payable Days	67	76	80	74	70	71	72
Non-cash WC days	23	22	12	33	24	20	18

Cash Flow

Y/E Mar (Rs mn)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Profit Before Tax	12,839	11,439	13,606	20,058	26,103	33,548	44,001
Depreciation	6,007	6,152	6,870	7,575	8,665	9,146	9,699
Others	22,262	(1,466)	0	5,016	0	0	0
Tax paid	(3,888)	(4,028)	(4,455)	(5,340)	(6,538)	(8,387)	(11,000)
Change in WC	(21,064)	(984)	4,015	(5,945)	3,391	(322)	(333)
Operating Cashflow	**16,156**	**11,114**	**20,036**	**21,364**	**31,620**	**33,986**	**42,366**
Capex	(6,518)	(15,696)	(20,227)	(16,978)	(9,000)	(8,500)	(8,500)
Change in Invest.	(1,749)	188	(206)	(17,563)	0	0	0
Others	359	(623)	(265)	735	0	0	0
Investing Cashflow	**(7,907)**	**(16,130)**	**(20,698)**	**(33,806)**	**(9,000)**	**(8,500)**	**(8,500)**
Change in Debt	(2,866)	746	4,516	21,557	(5,000)	(5,000)	(5,000)
Change in Equity	0	(514)	33	(24)	0	0	0
Others	(4,811)	887	(1,292)	(8,365)	(3,946)	(5,032)	(6,600)
Financing Cashflow	**(7,677)**	**1,119**	**3,257**	**13,168**	**(8,946)**	**(10,032)**	**(11,600)**
Net Change in Cash	**572**	**(3,898)**	**2,595**	**726**	**13,674**	**15,453**	**22,266**

Source: Company, Equirus Research

equirus SECURITIES

Krishna Institute of Medical Sciences Ltd

Turnaround in sight- reiterate Long

CMP	Target Price
Rs 636	Rs 863 Mar 2027
Rating	Upside
LONG	36% (↑)

- KIMS has followed a cluster-based approach to enter new geographies through doctor partnerships, with operations now spanning 5 states.
- In a backdrop where the industry has largely relied on brownfield expansions and acquisitions, KIMS has undertaken a significant greenfield push — commissioning ~1,950 beds across Thane, Bengaluru (two hospitals), Kerala (two hospitals) and Nashik.
- New site losses have weighed on profitability, with margins contracting ~600 bps and ROIC declining to 9%. With new sites breaking even over the next 2-3 quarters and contributing positively from the tail-end of FY27E, we believe the worst is behind us — Revenue/EBITDA CAGR of 25%/41% over the next two years.
- Street concerns around oversupply and structural margin dilution are overdone, in our view — strong occupancy across relevant micro-markets suggests incremental capacity will be absorbed well. We reiterate Long with a Mar'28 TP of Rs. 863 at 24x EV/EBITDA.

New site accretion to drive EBITDA step-up: With new sites approaching breakeven, the earnings drag is structurally reversing. We estimate new hospitals to swing from a combined loss of Rs. 700mn in FY26E to an EBITDA contribution of Rs. 1.7bn by FY28E — a Rs. 2.4bn swing that alone accounts for a meaningful portion of our 570bps margin recovery forecast. Management expects all new units except Bengaluru Electronic City to break even by 1QFY27, with Electronic City following by 3QFY27. Notably, new site losses have already improved sequentially, validating the ramp-up trajectory ahead of schedule.

New geographies commanding premium ARPOB: New regions — Thane, Bengaluru and Kerala — command materially higher ARPOB than KIMS's legacy AP/Telangana base, reflecting better payor mix and higher case complexity. As occupancy ramps in these units, incremental revenue will accrue at higher realisations, supporting both revenue growth and margin expansion simultaneously. Strong occupancy trends across these micro-markets provide high confidence on absorption of incremental capacity with minimal ramp-up risk.

Next capex phase: mature market brownfield: Over the past 18 months, KIMS expanded bed capacity by 39% to 6,464 beds. The next phase through FY27-FY28 is focused on mature markets — Telangana and AP — with ~1,150 beds planned across Kondapur (550 beds, early FY27), Rajahmundry (350 beds) and Anantapur (250 beds). These brownfield additions target markets where KIMS already holds dominant share, implying faster ramp-up and better capital efficiency versus the greenfield cycle just concluded.

ROCE recovery the core thesis: We expect EBITDA to nearly double over FY26E-FY28E, margins to recover to 26.2% and ROCE to improve ~1,000bps to 22%. We reiterate Long with a Mar'28 TP of Rs. 863 at 24x EV/EBITDA.

Financial Summary

YE Mar Rs mn	Sales	EBITDA	Recurring PAT	EPS (Rs)	P/E (x)	P/B (x)	EV/ EBITDA (x)	ROE (%)	Core ROIC (%)	EBITDA Margin (%)
FY25A	30,350	7,827	3,737	9.6	66.2	11.9	35.0	18.8	12.2	25.8
FY26E	39,045	7,985	2,682	6.7	95.0	10.6	34.7	11.8	8.7	20.5
FY27E	50,906	12,283	5,756	14.4	44.2	8.5	22.4	21.4	13.7	24.1
FY28E	60,806	15,938	8,520	21.3	29.9	6.6	17.2	25.0	18.7	26.2

Source: Company, Equirus Securities

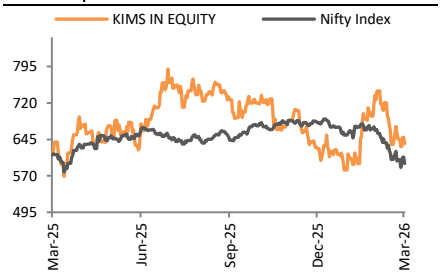
Estimate unchanged

	Forecasts		% Change	
(Rs mn)	FY26E	FY27E	FY26E	FY27E
Sales	39,045	50,906	-	-
EBITDA	7,985	12,283	-	-
PAT	2,682	5,756	-	-
EPS	6.7	14.4	-	-

Stock Information	
Market Cap (Rs Mn)	2,54,648
52 Wk H/L (Rs)	798/528
Avg Daily Volume (1yr)	4,26,640
Avg Daily Value (Rs Mn)	3.0
Equity Cap (Rs Mn)	21,378
Face Value (Rs)	2
Share Outstanding (Mn)	400.1
Bloomberg Code	KIMS IN
Ind Benchmark	

Ownership (%)	Recent	3M	12M
Promoters	34.1	0.0	(4.7)
DII	32.6	0.5	0.6
FII	14.3	(0.7)	(1.4)
Public	19.0	0.2	5.5

Relative price chart



Source: Bloomberg

Analysts
Bharat Celly
bharat.celly@equirus.com
+91-079 6901 5036
Vinay Jain
vinay.jain@equirus.com
+91-079 69 01 5011

Exhibit 71: Key Metrics performance

Operating Matrix	FY20	FY21	FY22	FY23	FY24	FY25	9MFY26	CAGR FY20-FY25
ARPOB	18,307	20,609	25,323	29,946	31,916	39,158	43,767	16%
Bed Capacity	3,079	3,079	3,940	3,940	3,975	5,444	6,464	12%
Occupancy	56%	58%	59%	53%	54%	50%	50%	
ALOS	4.34	5.53	4.79	4.1	4.11	3.66		
In-Patient	1,40,676	1,16,590	1,36,731	1,77,181	1,91,167	2,13,346	1,82,542	9%
Out-Patient	11,37,560	8,30,211	10,13,759	14,62,439	16,07,563	18,34,312	16,60,690	10%

Source: Company Data, Equirus

Exhibit 72: Bed expansion break-up over FY20-9MFY26

Region	FY20	Acquisition	Brownfield	Greenfield	O&M/PHF	Total Addition	9MFY26
Hyderabad	1200	662	20			682	1882
AP, Telangana ex Hyderabad	1879	335	100			435	2314
Mumbai	0			300		300	300
Western Region ex Mumbai	0	334		325		659	659
Kerala	0				509	509	509
Bangalore	0			800		800	800
Total	3,079	1,331	120	1,425	509	3,385	6,464

Source: Company Data, Equirus

Exhibit 73: Key historical financial metrics

Financial Matrix	FY20	FY21	FY22	FY23	FY24	FY25	9MFY26	CAGR FY20-FY25
Revenue	11,226	13,299	16,508	21,977	24,981	30,669	28,466	22%
EBITDA	2,450	3,709	5,158	6,040	6,404	8,146	6,122	27%
EBITDA Margin	22%	28%	31%	27%	26%	27%	22%	
CFO	2,015	3,560	3,240	3,944	4,837	5,818	2,846	
Capex	-532	-936	-1,711	-7,881	-6,459	-11,374	-7,407	
FCF	1,483	2,624	1,529	-3,937	-1,622	-5,556	-4,561	
ROCE	20%	30%	33%	24%	18%	16%	12%	
Net Debt/EBITDA	1.0			0.8		2.2		

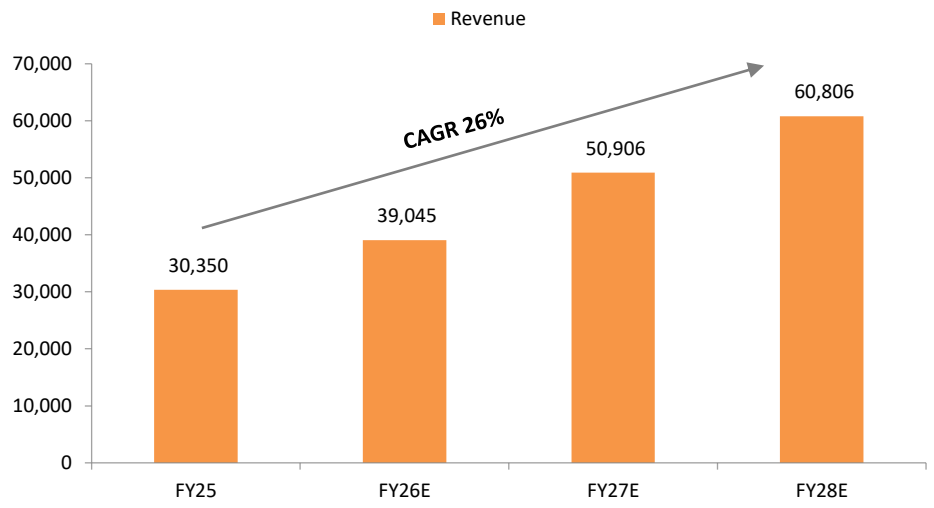
Source: Company Data, Equirus

Exhibit 74: Planned bed expansion over 9MFY26-FY29E

Region	9MFY26	Brownfield	Greenfield	O&M/PHF	Total	FY29
Hyderabad	1,882	550		200	750	2,632
Bangalore	800					800
Mumbai	300					300
Chennai	-		300		300	300
Metro	2,982	550	300	200	1,050	4,032
AP, Telangana ex Hyderabad	2,314	650			650	2,964
Kerala	509			351	351	860
Western Region ex Mumbai	659					659
Non Metro	3,482	650	-	351	1,001	4,483
Total	6,464	1,200	300	551	2,051	8,515
% of bed addition		19%	5%	9%	32%	

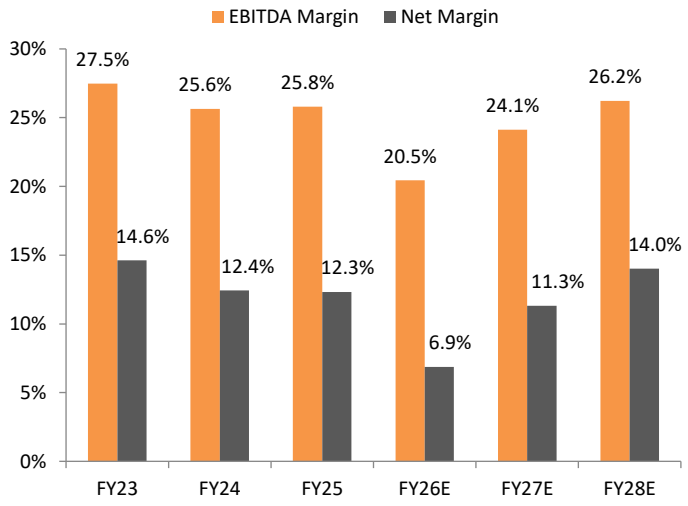
Source: Company Data, Equirus

Exhibit 75: On a strong growth trajectory of 26% CAGR over FY25-28E



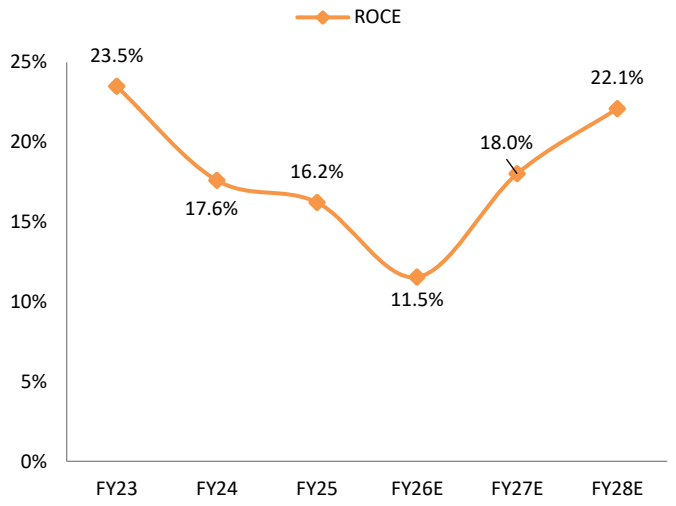
Source: Company Data, Equirus

Exhibit 76: Margins to revive with new capacity turning profitable



Source: Company Data, Equirus

Exhibit 77: Return to strengthen as new capacities pays off



Source: Company Data, Equirus

Quarterly performance

Y/E Mar (Rs mn)	1QFY25A	2QFY25A	3QFY25A	4QFY25A	1QFY26A	2QFY26A	3QFY26A	4QFY26E
Revenue	**6,884**	**7,773**	**7,724**	**7,969**	**8,716**	**9,607**	**9,977**	**10,745**
COGS	1,426	1,555	1,618	1,671	1,840	1,952	1,969	2,256
Employee Cost	1,186	1,192	1,275	1,341	1,510	1,745	1,763	1,800
Other Expenses	2,478	2,845	2,959	2,977	3,440	3,870	4,256	4,659
EBITDA	**1,794**	**2,181**	**1,872**	**1,980**	**1,926**	**2,040**	**1,989**	**2,030**
Depreciation	390	410	447	526	534	661	787	673
EBIT	**1,404**	**1,771**	**1,425**	**1,454**	**1,392**	**1,379**	**1,202**	**1,357**
Interest Exp.	179	199	257	267	326	450	568	331
Other Income	46	50	178	45	71	42	52	20
Profit before Tax	**1,271**	**1,622**	**1,346**	**1,232**	**1,137**	**971**	**686**	**1,046**
Tax Expenses	319	415	421	279	287	248	167	272
Profit After Tax	**952**	**1,207**	**925**	**953**	**850**	**723**	**519**	**774**
Minority Interest	(86)	(133)	(38)	(43)	(64)	(51)	15	(84)
Profit/(Loss) from Associates	0	0	0	0	0	0	0	0
Recurring PAT	**866**	**1,074**	**887**	**910**	**786**	**672**	**534**	**690**
Exceptional Items	0	0	0	(108)	0	3	0	0
Reported PAT	**866**	**1,074**	**887**	**802**	**786**	**675**	**534**	**690**
Other comprehensive income.	0	0	0	0	0	0	0	0
PAT after comp. income.	**866**	**1,074**	**887**	**802**	**786**	**675**	**534**	**690**
FDEPS	**2.2**	**2.7**	**2.2**	**2.0**	**2.0**	**1.7**	**1.3**	**1.7**
Cost items as % of sales								
RM expenses	20.7	20.0	20.9	21.0	21.1	20.3	19.7	21.0
Employee expenses	17.2	15.3	16.5	16.8	17.3	18.2	17.7	16.8
Other expenses	36.0	36.6	38.3	37.4	39.5	40.3	42.7	43.4
Margin (%)								
Gross Margin	79.3	80.0	79.1	79.0	78.9	79.7	80.3	79.0
EBITDA Margin	26.1	28.1	24.2	24.8	22.1	21.2	19.9	18.9
PAT Margin	12.6	13.8	11.5	10.1	9.0	7.0	5.4	6.4
YoY Growth (%)								
Sales	13.6	19.1	27.5	25.7	26.6	23.6	29.2	34.8
EBITDA	14.2	23.0	27.2	24.6	7.4	(6.5)	6.3	2.5
EBIT	12.5	22.4	27.5	29.0	(0.9)	(22.1)	(15.6)	(6.7)
PAT	7.2	16.7	23.5	22.5	(9.2)	(37.2)	(39.8)	(14.0)

Key Financials (Consolidated)

Income Statement

Y/E Mar (Rs mn)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Revenue	**16,508**	**21,977**	**24,981**	**30,350**	**39,045**	**50,906**	**60,806**
COGS	3,552	4,806	5,303	6,270	8,017	10,690	12,769
Employee Cost	2,619	3,464	4,224	4,994	6,818	7,841	9,330
Other Expenses	5,180	7,666	9,051	11,259	16,225	20,091	22,768
EBITDA	**5,158**	**6,040**	**6,404**	**7,827**	**7,985**	**12,283**	**15,938**
Depreciation	727	1,293	1,465	1,773	2,655	3,021	3,261
EBIT	**4,431**	**4,747**	**4,939**	**6,054**	**5,330**	**9,262**	**12,677**
Interest Exp.	160	305	470	902	1,675	1,234	1,144
Other Income	203	259	131	319	185	206	436
Profit before Tax	**4,473**	**4,701**	**4,599**	**5,471**	**3,840**	**8,234**	**11,970**
Tax Expenses	1,131	1,191	1,236	1,434	974	2,141	3,112
Profit After Tax	**3,343**	**3,510**	**3,363**	**4,037**	**2,866**	**6,094**	**8,858**
Minority Interest	(111)	(295)	(259)	(300)	(184)	(337)	(337)
Profit/(Loss) from Associates	95	0	0	0	0	0	0
Recurring PAT	**3,327**	**3,215**	**3,104**	**3,737**	**2,682**	**5,756**	**8,520**
Exceptional Items	0	(148)	0	(108)	3	0	0
Reported PAT	**3,327**	**3,363**	**3,104**	**3,845**	**2,679**	**5,756**	**8,520**
Other comprehensive income.	0	0	0	0	0	0	0
PAT after comp. income.	**3,327**	**3,363**	**3,104**	**3,845**	**2,679**	**5,756**	**8,520**
FDEPS	8.3	8.4	7.8	9.6	6.7	14.4	21.3
DPS	0	0	0	0	0	0	0
BVPS	35	42	46	53	60	75	96

YoY Growth (%)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Sales	24.1	33.1	13.7	21.5	28.6	30.4	19.4
EBITDA	39.1	17.1	6.0	22.2	2.0	53.8	29.8
EBIT	47.0	7.1	4.0	22.6	(12.0)	73.8	36.9
PAT	65.3	1.1	(7.7)	23.9	(30.3)	114.9	48.0

Key Ratios

Profitability (%)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Gross Margin	78.5	78.1	78.8	79.3	79.5	79.0	79.0
EBITDA Margin	31.2	27.5	25.6	25.8	20.5	24.1	26.2
PAT Margin	20.2	15.3	12.4	12.7	6.9	11.3	14.0
ROE	29.6	21.0	17.7	18.8	11.8	21.4	25.0
ROIC	23.5	15.7	11.1	10.2	7.2	11.2	14.3
Core ROIC	31.1	18.5	12.9	12.2	8.7	13.7	18.7
Dividend Payout	0.0	0.0	0.0	0.0	0.0	0.0	0.0

CAGR (%)	1 year	2 years	3 years	5 years	7 years	10 years
Revenue	21.5	17.5	22.5	22.0	24.3	21.9
EBITDA	22.2	13.8	14.9	26.2	33.2	26.1
PAT	23.9	6.9	4.9	26.4	(235.0)	52.6

Valuation (x)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
P/E	76.5	75.7	82.0	66.2	95.0	44.2	29.9
P/B	18.3	15.2	13.9	11.9	10.6	8.5	6.6
P/FCFF	0.0	0.0	0.0	0.0	0.0	0.0	0.0
EV/EBITDA	49.7	43.0	41.4	35.0	34.7	22.4	17.2
EV/Sales	15.5	11.8	10.6	9.0	7.1	5.4	4.5
Dividend Yield (%)	0.0	0.0	0.0	0.0	0.0	0.0	0.0

Balance Sheet

Y/E Mar (Rs mn)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Equity Capital	800	800	800	800	800	800	800
Reserves	13,073	15,895	17,483	20,578	23,260	29,016	37,536
Net Worth	**13,873**	**16,695**	**18,284**	**21,378**	**24,060**	**29,816**	**38,336**
Total Debt	1,610	5,332	10,462	19,060	22,060	20,560	19,060
Other long term liabilities	1,440	2,094	3,789	8,269	8,269	8,269	8,269
Minority Interest	233	2,684	2,649	2,810	2,994	3,332	3,669
Account Payables	1,295	1,743	1,972	2,691	3,209	4,184	4,998
Other Current Liabilities	621	864	1,365	3,152	1,562	2,036	2,432
Total Liabilities	**19,073**	**29,413**	**38,520**	**57,360**	**62,154**	**68,197**	**76,765**
Gross Fixed Assets	13,388	22,292	30,433	41,352	48,352	52,352	56,352
Acc. Depreciation	3,336	4,629	6,094	7,867	10,522	13,543	16,804
Net Fixed Assets	10,052	17,663	24,339	33,485	37,830	38,809	39,548
Capital WIP	208	4,769	6,001	12,138	12,138	12,138	12,138
long term investments	3,325	0	744	771	771	771	771
Others	1,555	2,204	1,961	3,993	3,993	3,993	3,993
Inventory	364	429	494	640	856	1,116	1,333
Receivables	1,286	2,527	2,944	3,954	4,279	5,579	6,664
Loans and advances	0	0	0	477	477	477	477
Other current assets	383	479	716	885	868	1,131	1,351
Cash & Cash Equivalents.	1,901	1,343	1,320	1,017	943	4,183	10,490
Total Assets	**19,073**	**29,413**	**38,520**	**57,360**	**62,154**	**68,197**	**76,764**
Non-Cash WC	117	827	818	113	1,708	2,082	2,395
Cash Conv. Cycle	(76.9)	(53.6)	(56.1)	(61.2)	(61.8)	(57.2)	(59.5)
WC Turnover	141.2	26.6	30.6	268.6	22.9	24.4	25.4
Gross Asset Turnover	1.2	1.0	0.8	0.7	0.8	1.0	1.1
Net Asset Turnover	1.6	1.0	0.8	0.7	0.8	1.0	1.2
Net D/E	0.0	0.2	0.4	0.7	0.8	0.5	0.2

Days (x)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Receivable Days	26	32	40	41	38	35	37
Inventory Days	31	30	32	33	34	34	35
Payable Days	134	115	128	136	134	126	131
Non-cash WC days	3	14	12	1	16	15	14

Cash Flow

Y/E Mar (Rs mn)	FY22A	FY23A	FY24A	FY25A	FY26E	FY27E	FY28E
Profit Before Tax	4,473	4,701	4,599	5,471	3,840	8,234	11,970
Depreciation	727	1,293	1,465	1,773	2,655	3,021	3,261
Others	(85)	(148)	0	766	3	0	0
Tax paid	(1,131)	(1,191)	(1,236)	(1,434)	(974)	(2,141)	(3,112)
Change in WC	(745)	(710)	9	(758)	(1,595)	(374)	(312)
Operating Cashflow	**3,240**	**3,944**	**4,837**	**5,818**	**3,929**	**8,741**	**11,807**
Capex	(1,700)	(14,115)	(9,130)	(9,617)	(7,000)	(4,000)	(4,000)
Change in Invest.	(2,549)	3,325	(744)	(5,284)	0	0	0
Others	134	0	0	3,739	0	0	0
Investing Cashflow	**(4,115)**	**(10,790)**	**(9,875)**	**(11,162)**	**(7,000)**	**(4,000)**	**(4,000)**
Change in Debt	(1,094)	3,722	5,130	7,938	3,000	(1,500)	(1,500)
Change in Equity	1,917	(541)	(1,516)	(942)	3	0	0
Others	(212)	2,453	(294)	(1,565)	(6)	0	0
Financing Cashflow	**610**	**5,634**	**3,320**	**5,431**	**2,997**	**(1,500)**	**(1,500)**
Net Change in Cash	**(265)**	**(1,212)**	**(1,717)**	**87**	**(74)**	**3,241**	**6,307**

Source: Company, Equirus Research

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